Filling Reference Number
202607221897

(1)

BEFORE THE HON'BLE DISTRICT CONSUMER DISPUTES REDRESSAL COMMISSION, GHAZIABAD (U.P.)

CONSUMER COMPLAINT NO. _____ OF 2026

(Under Section 35 of the Consumer Protection Act, 2019)

IN THE MATTER OF:

Vikas Gandhi ...Complainant

VERSUS

ACKO General Insurance Limited ...Opposite Party

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Place: Ghaziabad
Date: 12.07.2026

COMPLAINANT
(Vikas Gandhi)

BEFORE THE HON'BLE DISTRICT CONSUMER DISPUTES REDRESSAL COMMISSION, GHAZIABAD (U.P.)

CONSUMER COMPLAINT NO. _____ OF 2026

(Complaint under Section 35 of the Consumer Protection Act, 2019)

IN THE MATTER OF:

Vikas Gandhi ...Complainant

VERSUS

ACKO General Insurance Limited ...Opposite Party

PROFORMA FOR FILING CONSUMER COMPLAINT

1. Commission before which the complaint is filed	District Consumer Disputes Redressal Commission, Ghaziabad, Uttar Pradesh
2. Case type	Consumer Complaint under Section 35 of the Consumer Protection Act, 2019
3. Name of the Complainant	Vikas Gandhi, S/o Shri Surendra Gandhi
4. Age / Date of birth; Gender	About 53 years (DOB 09.10.1972); Male
5. Complete address of the Complainant	D-20, 1st Floor, Dayanand Nagar, Ghaziabad, Uttar Pradesh – 201001
6. Mobile / E-mail	9911198000 / 9amdeal@gmail.com
7. Complainant type; Senior Citizen; Widow	Individual; No; Not applicable
8. Name and address of the Opposite Party	ACKO General Insurance Limited, through its Managing Director / Authorised Signatory, 2nd Floor, 36/5 Hustle Hub One East, Somasandrapalya, 27th Main Road, Sector 2, HSR Layout, Bengaluru, Karnataka – 560102; E-mail: gro@acko.com; Phone: 1800 266 2256; IRDAI Regn. No. 157; CIN U66000KA2016PLC138288
9. Whether the Complainant is a "consumer" under Section 2(7)	Yes. The Complainant purchased a health insurance policy from the Opposite Party for consideration, for personal and family use.
10. Policy No. / Claim No.	Policy No. ASHP202400001607 (04.11.2024 to 03.11.2025; Sum Insured Rs. 10,00,000; since renewed as ASHP202400001607R1, 04.11.2025 to 03.11.2026). Claim No. 25062100350/1
11. Value of the goods or services paid as consideration	Annual premium Rs. 31,065 (Policy No. ASHP202400001607)
12. Fee payable	NIL. The consideration paid does not exceed Rs. 5,00,000 (Rule 7, Consumer Protection (Consumer Disputes Redressal Commissions) Rules, 2020)
13. Date on which cause of action arose	20.06.2025 and 21.06.2025 (hospitalisation); 25.06.2025 and 26.06.2025 (rejection and repudiation of claim); continuing thereafter; 02.07.2026 (Insurance Ombudsman Award with liberty to approach any other forum); 15.07.2026 (Opposite Party's GRO reiterating repudiation)
14. Whether the complaint is within limitation (Section 69)	Yes. Filed within two years of the cause of action.
15. Territorial jurisdiction	Section 34(2)(d). The Complainant resides at Ghaziabad and the entire hospitalisation and treatment took place at RS Maitri Health Care, J-278, Patel Nagar 1st, Ghaziabad.
16. Pecuniary jurisdiction	Within the limits of the District Commission. Consideration paid (premium Rs. 31,065) and total claim Rs. 1,62,358 plus interest.

17. Case category	Insurance; Health Insurance / Mediclaim; Rejection or denial of claim
18. Gist of the complaint	Wrongful repudiation of a fully documented and medically necessary hospitalisation claim of Rs. 37,358 (Circumcision and Meatoplasty for tight phimosis with balanitis, meatal stenosis and UTI, performed on the day of admission), on the ground of the 2-year "Specific Illness" waiting period, while ignoring the exception "unless necessitated by malignancy" contained in the insurer's own clause, the biopsy-confirmed pre-malignant condition (BXO), and the treating Urologist's certificate. This amounts to deficiency in service (Section 2(11)) and unfair trade practice (Section 2(47)) of the Consumer Protection Act, 2019.
19. Relief claimed	(a) Rs. 37,358 (claim amount) with interest at 12% per annum from 26.06.2025 till realisation; (b) Rs. 1,00,000 as compensation for mental agony and harassment; (c) Rs. 25,000 towards litigation costs; and (d) any other relief deemed fit. Total: Rs. 1,62,358 plus interest.
20. Earlier remedies availed	(i) The Opposite Party's grievance mechanism (Tickets 12568674, 12582411, 12582412), all rejected; (ii) the Insurance Ombudsman, Noida (Complaint NOI-H-056-2526-0961; Award IO/NOI/A/HI/0061/2026-2027 dated 02.07.2026, dismissing the complaint but holding medical necessity undisputed and the Permanent Exclusion inapplicable, with liberty to approach any other forum); and (iii) the National Consumer Helpline (INGRAM) Grievance No. 9719631 dated 14.07.2026, in process.
21. Whether any complaint on the same cause of action is pending before any other Consumer Commission or Court	No
22. Documents relied upon	Annexures C-1 to C-14, as per Index
23. Advocate for the Complainant (if any)	_____ (Enrolment No. _____) / Party-in-Person

I declare that the particulars stated above are true and correct to my knowledge and belief.

Place: Ghaziabad

Date: ____ July 2026

COMPLAINANT

(Vikas Gandhi)

BEFORE THE HON'BLE DISTRICT CONSUMER DISPUTES REDRESSAL COMMISSION, GHAZIABAD (U.P.)

CONSUMER COMPLAINT NO. _____ OF 2026

(Complaint under Section 35 of the Consumer Protection Act, 2019)

IN THE MATTER OF:

Vikas Gandhi ...Complainant

VERSUS

ACKO General Insurance Limited ...Opposite Party

SYNOPSIS

1. That the Complainant holds the Opposite Party's Personal Health Policy No. ASHP202400001607 (sum insured Rs. 10,00,000). In June 2025, more than seven months after the policy commenced, he developed acute obstruction in passing urine along with infection, and underwent an urgent Circumcision and Meatoplasty at RS Maitri Health Care, Ghaziabad on 20.06.2025, the very day of admission, performed by a specialist Urologist. The total hospital expense was Rs. 37,358.

2. That the Opposite Party repudiated Claim No. 25062100350/1 on 26.06.2025 citing only the general 2-year "Specific Illnesses" waiting-period clause. It did not name any listed condition, did not obtain any medical opinion, and did not deal with the exception contained in its own clause, namely "Any Surgery of the genitourinary system unless necessitated by malignancy." A day earlier, on 25.06.2025, it had already stated that the procedure "is not related to a malignancy", that is, five days before the histopathology report came into existence.

3. That the biopsy dated 30.06.2025 confirmed Balanitis Xerotica Obliterans (Lichen Sclerosus), a medically recognised pre-malignant condition, on a suspicious "cauliflower like lesion". The treating Urologist, Dr. Vaibhav Saxena, M.Ch. (AIIMS), has certified that the surgery "was must", that BXO is "a pre-malignant condition", and that without treatment there was a risk of life-threatening infection and penile cancer. The Opposite Party has not produced any contrary medical opinion.

4. That the Insurance Ombudsman, Noida, by Award dated 02.07.2026, dismissed the complaint in summary proceedings but recorded that the medical necessity of the procedure is undisputed and that the Permanent Exclusion clause is inapplicable, and granted liberty to approach any other forum. The only question that now remains is whether surgery for a biopsy-confirmed pre-malignant condition falls within the exception "unless necessitated by malignancy". The evidentiary gap noted by the Ombudsman no longer exists, as the treating specialist's certificate and standard medical literature are now on record.

5. That the repudiation, being unreasoned and contrary to the insurer's own clause and record, amounts to deficiency in service under Section 2(11) and to unfair trade practice under Section 2(47) of the Consumer

Protection Act, 2019. The Complainant seeks Rs. 37,358 with interest at 12% per annum from 26.06.2025, Rs. 1,00,000 as compensation for mental agony and harassment, and Rs. 25,000 towards costs.

LIST OF DATES AND EVENTS

Date	Event	Annexure
04.11.2024	ACKO Personal Health Policy (Standard Plan, Family Floater) No. ASHP202400001607 issued after underwriting; Sum Insured Rs. 10,00,000; premium Rs. 31,065 p.a.; period 04.11.2024 to 03.11.2025. Proposal Form records "No medical conditions"; only PED declared, Obesity. Policy Schedule records "Permanent disease exclusion: No exclusion".	C-1, C-2
Nov 2024 to Jun 2025	All premium instalments paid on time; policy continuously in force.	C-3
20.06.2025	Complainant admitted to RS Maitri Health Care, Ghaziabad at 06:58 hrs with acute urinary obstruction ("POOR FLOW OF URINE") and infection; Circumcision and Meatoplasty performed the same day under spinal anaesthesia by Dr. Vaibhav Saxena, M.Ch. (Urology); IV fluids and IV antibiotics administered; excised tissue sent for histopathology.	C-4
21.06.2025	Discharged in stable condition; total hospital expenses Rs. 37,358, paid by the Complainant. Reimbursement Claim No. 25062100350/1 lodged.	C-4, C-5, C-6
24.06.2025	Detailed representation and grievance sent to the Opposite Party (Ticket 12568674).	C-6
25.06.2025	"Update on Your Claim" e-mail rejects the claim, asserting that the procedure "is not related to a malignancy", five days before the histopathology report existed; the GRO rejects the representation in the same terms.	C-7
26.06.2025	Claim formally repudiated citing only the general "Specific Illnesses" 2-year waiting-period clause, without naming any listed condition and without dealing with the exception "unless necessitated by malignancy" contained in it.	C-8
30.06.2025	Histopathology Report, Max Lab, No. V-6630/25: gross "cauliflower like lesion 3.5 x 3 x 0.8 cm"; Impression: "Consistent with Lichen sclerosus (balanitis xerotica obliterans)", a recognised pre-malignant condition.	C-9
Jul to Aug 2025	Appeals and grievances to the Opposite Party (Tickets 12582411, 12582412), all rejected without dealing with the medical evidence.	C-10 (colly.)
07.08.2025	Complaint filed before the Insurance Ombudsman, Noida (NOI-H-056-2526-0961).	C-11
30.08.2025	Opposite Party itself renews the policy as No. ASHP202400001607R1 (04.11.2025 to 03.11.2026; Sum Insured Rs. 11,00,000), recording "First policy inception date: 04/11/2024".	C-1 (colly.)
24.06.2026	Personal hearing before the Insurance Ombudsman.	C-11
02.07.2026	Ombudsman Award IO/NOI/A/HI/0061/2026-2027: complaint dismissed, but paras 13 and 14 hold medical necessity undisputed and the Permanent Exclusion inapplicable; the forwarding letter grants liberty to approach any other forum.	C-11
12.07.2026	Treating Urologist's Medical Certificate: surgery "was must" in view of inability to pass urine and UTI; biopsy confirmed BXO, "a pre-malignant condition"; risk of life-threatening infection and penile cancer if not treated.	C-12
14.07.2026	Grievance registered on the National Consumer Helpline (INGRAM) portal, Grievance No. 9719631 (in process).	C-10 (colly.)
15.07.2026	Opposite Party's GRO replies, reiterating the "waiting period" ground of 26.06.2025 without adverting to the biopsy report, the treating doctor's certificate, or the Ombudsman's findings.	C-10 (colly.)
____.____.2026	Present complaint filed before this Hon'ble Commission.	—

Place: Ghaziabad

Date: ____ July 2026

COMPLAINANT

(Vikas Gandhi)

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BEFORE THE HON'BLE DISTRICT CONSUMER DISPUTES REDRESSAL COMMISSION, GHAZIABAD (U.P.)

CONSUMER COMPLAINT NO. _____ OF 2026

(Under Section 35 of the Consumer Protection Act, 2019)

MEMO OF PARTIES

COMPLAINANT:

Vikas Gandhi
S/o Shri Surendra Gandhi, aged about 53 years,
R/o D-20, 1st Floor, Dayanand Nagar,
Ghaziabad, Uttar Pradesh – 201001
Mobile: 9911198000 | E-mail: 9amdeal@gmail.com

VERSUS

OPPOSITE PARTY:

ACKO General Insurance Limited,
Through its Managing Director / Authorised Signatory,
Regd. Office: 2nd Floor, 36/5 Hustle Hub One East, Somasandrapalya,
27th Main Road, Sector 2, HSR Layout, Bengaluru, Karnataka – 560102
E-mail: gro@acko.com | Phone: 1800 266 2256
(IRDAI Regn. No. 157; CIN U66000KA2016PLC138288)

Place: Ghaziabad
Date: 22.July.2026

COMPLAINANT
(Vikas Gandhi)

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BEFORE THE HON'BLE DISTRICT CONSUMER DISPUTES REDRESSAL COMMISSION, GHAZIABAD (U.P.)

CONSUMER COMPLAINT NO. _____ OF 2026

(Complaint under Section 35 of the Consumer Protection Act, 2019)

IN THE MATTER OF:

Mr. Vikas Gandhi

S/o Shri Surendra Gandhi, aged about 53 years,

R/o D-20, 1st Floor, Dayanand Nagar,

Ghaziabad, Uttar Pradesh – 201001

Mobile: 9911198000 | Email: 9amdeal@gmail.com

...COMPLAINANT

VERSUS

ACKO General Insurance Limited

Through its Managing Director / Authorised Signatory

Regd. Office: 2nd Floor, 36/5 Hustle Hub One East,

Somasandrapalya, 27th Main Road, Sector 2, HSR Layout,

Bengaluru, Karnataka – 560102

Email: gro@acko.com | IRDAI Reg. No. 157

...OPPOSITE PARTY

COMPLAINT UNDER SECTION 35 OF THE CONSUMER PROTECTION ACT, 2019

MOST RESPECTFULLY SHOWETH:

This complaint arises from the wrongful repudiation by the Opposite Party of the Complainant's health insurance claim of Rs. 37,358/-. The Complainant was hospitalised in June 2025 for acute obstruction in passing urine, along with infection, and underwent an urgent surgery performed by a specialist Urologist. The tissue removed during the surgery was sent for biopsy, which confirmed a pre-malignant condition. The Opposite Party repudiated the claim on the ground of a waiting period, without examining the biopsy report, without obtaining any medical opinion, and without dealing with the exception written into its own policy clause. Each fact stated below is supported by a document filed along with this complaint.

PARTIES

1. That Complainant is a consumer within the meaning of Section 2(7) of the Consumer Protection Act, 2019, having purchased a health insurance policy from the Opposite Party for valuable consideration for his personal and family use.

2. That Opposite Party is a general insurance company registered with the Insurance Regulatory and Development Authority of India (Registration No. 157) and provides insurance services for consideration.

JURISDICTION AND LIMITATION

3. That Complainant resides at Ghaziabad and the entire hospitalisation and treatment took place at RS Maitri Health Care, J-278, Patel Nagar 1st, Ghaziabad, which is within the local limits of this Hon'ble Commission. This Hon'ble Commission, therefore, has territorial jurisdiction to entertain and try the present complaint under Section 34(2)(d) of the Act.

4. That claim amount, together with the compensation claimed, is within the pecuniary jurisdiction of this Hon'ble Commission, and the requisite court fee, if any, is being paid.

5. That claim was repudiated on 26.06.2025. The Complainant thereafter approached the grievance cell of the Opposite Party and the Insurance Ombudsman, Noida, whose Award was passed on 02.07.2026. The present complaint is filed within two years of the cause of

action, as required under Section 69 of the Act. The forwarding letter of the Insurance Ombudsman dated 02.07.2026 (Annexure C-11, Annex-XIII-A) itself records:

> *"In the eventuality of your disagreeing with the enclosed award, you may, move a fresh application at any other forum/court that may be considered by you as appropriate."*

BRIEF FACTS

6. That material dates and events, each supported by the document noted against it, are as follows:

Date	Event	Supporting Document
04.11.2024	ACKO Personal Health Policy (Standard Plan, Family Floater) No. ASHP202400001607 issued after underwriting; Sum Insured Rs. 10,00,000; premium Rs. 31,065 p.a.; policy period 04.11.2024 to 03.11.2025.	Annexure C-1 (Policy Schedule); Annexure C-2 (Proposal Form)
Nov 2024 to Jun 2025	All premium instalments paid on time; policy continuously in force.	Annexure C-3
June 2025	Complainant develops acute urinary complaints, namely poor flow of urine with infection.	Annexure C-4
20.06.2025	Admitted to RS Maitri Health Care, Ghaziabad at 06:58 hrs; Circumcision and Meatoplasty performed the same day under spinal anaesthesia by Dr. Vaibhav Saxena, M.Ch. (Urology); IV fluids and IV antibiotics given; excised tissue sent for histopathology.	Annexure C-4
21.06.2025	Discharged in stable condition; total hospital expenses Rs. 37,358, paid by the Complainant.	Annexure C-4; Annexure C-5
21/24.06.2025	Reimbursement Claim No. 25062100350/1 lodged with all documents; representation sent.	Annexure C-6
25.06.2025	Claim rejected by "Update on Your Claim" email stating that the procedure "is not related to a malignancy", five days before the histopathology report came into existence; the GRO rejected the representation in the same terms.	Annexure C-7
26.06.2025	Claim repudiated by email citing only the general "Specific Illnesses" waiting-period clause, without naming any listed condition and without dealing with the exception contained in it.	Annexure C-8
30.06.2025	Histopathology Report (Max Lab, No. V-6630/25): gross "cauliflower like lesion 3.5 x 3 x 0.8 cm"; Impression: "Consistent with Lichen sclerosus (balanitis xerotica obliterans)".	Annexure C-9

Jul to Aug 2025	**Appeals and grievances to the Opposite Party, all rejected without dealing with the medical evidence.**	**Annexure C-10 (colly.)**
07.08.2025	**Complaint filed before the Insurance Ombudsman, Noida (NOI-H-056-2526-0961).**	**Annexure C-11**
24.06.2026	**Personal hearing before the Ombudsman.**	**Annexure C-11**
02.07.2026	**Ombudsman Award IO/NOI/A/HI/0061/2026-2027: complaint dismissed, but medical necessity held undisputed and the Permanent Exclusion held inapplicable; liberty granted to approach any other forum.**	**Annexure C-11; Annex-XIII-A**
12.07.2026	**Treating Urologist issues Medical Certificate: surgery "was must" owing to inability to pass urine and UTI; biopsy confirmed BXO, "a pre-malignant condition"; risk of life-threatening infection and penile cancer if not treated.**	**Annexure C-12**
14.07.2026	**Grievance registered on the National Consumer Helpline (INGRAM) portal, Grievance No. 9719631 (in process).**	**Annexure C-10 (colly.)**
15.07.2026	**Grievance Redressal Officer of the Opposite Party again relies on the "waiting period" ground of 26.06.2025, without adverting to the biopsy report, the treating doctor's certificate, or the Ombudsman's findings.**	**Annexure C-10 (colly.)**

THE ILLNESS AND THE SURGERY

7. That in June 2025, more than seven months after the policy commenced, the Complainant developed acute obstruction in passing urine. The Discharge Summary of RS Maitri Health Care (Annexure C-4) records the following:

> **Diagnosis:** "TIGHT PHIMOSIS WITH BALANITIS WITH MEATAL STENOSIS"
>
> **Brief history of presenting illness:** "POOR FLOW OF URINE"
>
> **Procedure:** "CIRCUMCISION + MEATOPLASTY DONE UNDER SA ON 20/06/2025"

Course during hospitalization: "IV FLUIDS AND IV ANTIBIOTICS"

8. That these entries, made by the hospital in its own contemporaneous record, show that the Complainant had, first, acute urinary obstruction; secondly, active infection, being balanitis with urinary tract infection, for which intravenous antibiotics were given; thirdly, surgery on the very day of admission, which is done only where surgery cannot be postponed; and fourthly, treatment by a qualified surgical team under anaesthesia in a hospital.

9. That Complainant was discharged on 21.06.2025 and paid the entire hospital expenses of Rs. 37,358/- from his own funds. He thereafter lodged Reimbursement Claim No. 25062100350/1 with the Opposite Party along with all supporting documents (Annexure C-6). The quantum of the claim has, at no stage, been disputed by the Opposite Party.

10. That tissue removed during the surgery was sent for histopathological examination. The Histopathology Report of Max Lab dated 30.06.2025, No. V-6630/25 (Annexure C-9), records in its Gross Description: "Externally shows cauliflower like lesion measures 3.5 x 3 x 0.8 cm." The Impression of the same report records: "Consistent with Lichen sclerosus (balanitis xerotica obliterans)."

11. That biopsy thus confirmed Balanitis Xerotica Obliterans (BXO), also known as Lichen Sclerosus, which is a recognised pre-malignant condition of the penis. This is certified by the treating specialist referred to below, and is also supported by standard medical literature (Annexure C-13). The whole sequence of illness, surgery and diagnosis is therefore borne out by hospital and laboratory records, and not merely by the Complainant's own statement.

12. That for the convenience of this Hon'ble Commission, the medical terms used in these documents mean, in ordinary language, the following. Phimosis is an abnormal tightening of the foreskin which prevents it from being drawn back and, in a severe case, obstructs the passing of urine. Balanitis is inflammation and infection of the glans and foreskin. Meatal stenosis is a narrowing of the urinary opening which further obstructs the flow of

urine. Balanitis Xerotica Obliterans (BXO), also called Lichen Sclerosus, is a chronic scarring disease of the foreskin which is medically recognised as a pre-malignant condition, that is, a condition that can progress to cancer of the penis if it is not treated. None of these is a cosmetic complaint; each is a genuine illness recorded by the treating hospital, and together they explain why the surgery had to be done on the day of admission.

THE TREATING DOCTOR'S CERTIFICATE

13. That Complainant was treated and operated upon by Dr. Vaibhav Saxena, M.S. (BHU), M.Ch. Urology (AIIMS, New Delhi), U.P. Medical Council Reg. No. 47425. As appears from his letterhead and registration, he is the Senior Director of Urology, Uro-Oncology, Robotic Surgery and Renal Transplant at Yashoda Super Speciality Hospital, Kaushambi, and Consultant Urologist at RS Maitri Health Care, Ghaziabad, and is the very surgeon who operated upon the Complainant and followed up the biopsy.

14. That said doctor has issued a Medical Certificate dated 12.07.2026 (Annexure C-12), which states:

> *"Mr. Vikas Gandhi, 54 yrs Male, R/o D-20, Dayanand Nagar, Ghaziabad was suffering from Phimosis with Balanitis with meatal stenosis & UTI. His surgery was must in view of unable to pass urine & UTI. His Preputial Biopsy came out to be (BXO) which is a pre-malignant condition & not any elective or cosmetic surgery. There is life threatening infection risk & risk of penile cancer if not treated..."*

15. That certificate of the treating doctor establishes that the surgery was medically necessary and was not elective or cosmetic, that the biopsy confirmed a pre-malignant condition, and that without treatment there was a risk of life-threatening infection and of penile cancer. The opinion of the treating specialist, who himself examined and operated upon the Complainant, is the best medical evidence available in the matter. The Opposite Party has not, at the claim stage, at the grievance stage, or before the Ombudsman,

produced any medical opinion to the contrary, nor referred the matter to any medical board.

16. That BXO, or genital Lichen Sclerosus, is also recognised in standard medical literature (Annexure C-13) as a pre-malignant condition associated with cancer of the penis. The Guidelines on Penile Cancer of the European Association of Urology classify Lichen sclerosis among pre-malignant penile lesions, and record that phimosis, which is the Complainant's recorded diagnosis, is strongly associated with invasive penile cancer. This literature corroborates the certificate of the treating doctor and is placed on record in support of the complaint.

THE PROPOSAL FORM AND THE POLICY

17. That Proposal Form (Annexure C-2), on the basis of which the Opposite Party underwrote and accepted the risk, records the following answers given by the Complainant:

> *"Has anyone in this plan been diagnosed with a medical condition?" – "No medical conditions". "Did anyone in this plan take any prescribed medicines in the past week?" – "No". "Has anyone in this plan been advised to undergo or has ever undergone hospitalisation for any illness / surgery?" – "no". "Does anyone in this plan have any other ongoing health issues or symptoms for which you are planning a doctor visit?" – "No". "PED declared" – "Obesity".*

18. That the only condition declared was obesity. There was no previous medical condition, no ongoing symptom, no previous surgery and no previous hospitalisation. The Opposite Party accepted the proposal after underwriting and issued the policy, its Schedule (Annexure C-1) recording "Permanent disease exclusion: No exclusion."

19. That Opposite Party has never alleged suppression of any material fact, has never invoked the pre-existing disease clause, and has never questioned the genuineness of the

illness or of the treatment. Any suggestion that the disease existed before the policy commenced is thus ruled out by the insurer's own record.

20. That Opposite Party has, in fact, renewed the policy as Policy No. ASHP202400001607R1 for the period 04.11.2025 to 03.11.2026 (sum insured Rs. 11,00,000 including inflation-protection bonus; premium Rs. 36,656.86; Annexure C-1 colly.). The renewal document records "First policy inception date: 04/11/2024" and "Existing user: Yes". The Opposite Party thus continues to accept the Complainant's premium and to treat the cover as continuous from 04.11.2024, even while refusing the only claim ever made under it.

THE REPUDIATION OF THE CLAIM

21. That repudiation email dated 26.06.2025 (Annexure C-8), so far as it gives reasons, states:

> *"After carefully reviewing your claim, we noticed that you have filed a claim for the expenses incurred for treatment of a condition falling under waiting period. Unfortunately, these costs are not a part of your insurance coverage. As a result, we are unable to approve your claim."*

The email is followed only by an extract of the general clause 4.1.2 ("Specific Illnesses").

22. That a day earlier, by its "Update on Your Claim" email dated 25.06.2025 (Annexure C-7), the Opposite Party had cited the genitourinary entry and stated that "this procedure is not related to a malignancy". This medical conclusion was recorded on 25.06.2025, that is, five days before the Histopathology Report dated 30.06.2025 even came into existence, and without reference to any medical opinion.

23. That repudiation is thus not a reasoned decision. In deciding the claim, the Opposite Party did not examine the biopsy report, did not consider the pre-malignant character of BXO, did not consider the treating doctor's opinion, did not obtain any independent medical opinion, did not refer the matter to a medical board, and did not deal with the exception "unless necessitated by malignancy" contained in the very clause it relied upon.

It merely reproduced the words of a waiting-period clause without applying that clause to the medical facts of the case.

24. That the same unreasoned ground was repeated at every stage. The rejection email of 25.06.2025, the repudiation of 26.06.2025 and the reply of the Grievance Redressal Officer dated 15.07.2026 are, in substance, identical. None of them names any listed condition in the Schedule, refers to the Histopathology Report, refers to the treating doctor's certificate, or deals with the findings of the Ombudsman. Repetition of the same unreasoned ground at three successive stages is, by itself, a deficiency in service.

PROCEEDINGS BEFORE THE INSURANCE OMBUDSMAN

25. That Insurance Ombudsman, Noida, by Award No. IO/NOI/A/HI/0061/2026-2027 dated 02.07.2026 (Annexure C-11), dismissed the complaint, but recorded the following findings:

> *Para 13: "It is noted that it is not disputed that the procedure was medically necessary and it was undertaken to address the complaint/problem of poor flow of urine, and it has also been conveyed by complainant that he had serious infection."*
>
> *Para 14: "In this sense, the procedure involving circumcision + meatoplasty would not be covered under the Permanent Exclusion clause 4.2.3 of the policy as it has been necessitated by illness and forming part of treatment."*

26. That in view of these findings, the questions of medical necessity, genuineness of the treatment, continuance of the policy, and inapplicability of the Permanent Exclusion clause are no longer in dispute. The only question that now remains is whether a surgery performed for a biopsy-confirmed pre-malignant condition falls within the exception "unless necessitated by malignancy".

27. That Ombudsman decided this question against the Complainant only because, at the hearing, there was no post-operative medical paper certifying the malignant potential of BXO. That paper is now on record in the form of the treating doctor's certificate dated

12.07.2026 (Annexure C-12), read with the medical literature (Annexure C-13). The Award was passed in the summary procedure under the Insurance Ombudsman Rules, 2017, and does not bar this complaint; its forwarding letter itself grants liberty to approach any other forum.

THE POLICY CLAUSE AND ITS EXCEPTION

28. That Policy Schedule (Annexure C-1) provides: "Specific illness – 2 years waiting period on specific illness – NOTE: Refer Annexure for the complete disease list". The only entry of that Annexure relied upon by the Opposite Party reads: "Any Surgery of the genitourinary system unless necessitated by malignancy."

29. That this clause is not an absolute exclusion. The insurer itself has kept out of the waiting period any genitourinary surgery that is necessitated by malignancy. On the admitted facts, namely a suspicious "cauliflower like lesion", a biopsy-confirmed pre-malignant condition, and the treating doctor's certificate that BXO is a pre-malignant condition carrying a risk of penile cancer, the surgery undergone by the Complainant is covered by this exception.

30. That the policy nowhere defines the words "necessitated by malignancy", nowhere excludes pre-malignant conditions from the exception, nowhere requires an established cancer or a histologically proven invasive carcinoma, and nowhere states that preventive surgery for a pre-cancerous condition is excluded. The insurer's reading, that the exception applies only after cancer has fully developed, is not the only possible meaning, and it defeats the very purpose of the exception, because a surgery that prevents cancer ought not to be made to wait out two years when any delay is medically dangerous.

31. That Complainant does not claim to have had an established cancer. His case, certified by the treating doctor and supported by medical literature, is that BXO is a recognised pre-malignant condition carrying a known risk of progressing to penile cancer, and that the surgery was undertaken because of that risk, and not for any cosmetic or elective reason.

32. That in the alternative and without prejudice to the above, if this Hon'ble Commission were to find that the words "necessitated by malignancy" are capable of two reasonable meanings, then, the clause having been drafted solely by the insurer, the doubt must be resolved in favour of the insured, on the settled rule of contra proferentem. This is submitted only in the alternative; the Complainant's main case rests on the medical evidence itself.

GROUNDS

33. That the acts and omissions of the Opposite Party set out above amount to deficiency in service under Section 2(11) and to unfair trade practice under Section 2(47) of the Consumer Protection Act, 2019, on the following, among other, grounds:

(i) Because the Opposite Party repudiated a genuine, medically necessary and fully documented claim without examining the biopsy report and without obtaining any medical opinion of its own at any stage.

(ii) Because the Opposite Party failed to apply the exception "unless necessitated by malignancy" contained in the very clause on which it relied, and did not even disclose the specific sub-clause said to bar the claim.

(iii) Because the Opposite Party recorded, on 25.06.2025, a medical conclusion that the procedure was "not related to a malignancy", five days before the histopathology report came into existence.

(iv) Because the repudiation is a non-speaking order which merely reproduces the words of a waiting-period clause without applying it to the medical facts, and the same unreasoned ground was repeated at the claim, grievance and post-Award stages.

(v) Because the findings of the Insurance Ombudsman that the surgery was medically necessary and that the Permanent Exclusion is inapplicable have not been challenged, and the only gap noted by the Ombudsman has since been filled by the treating doctor's certificate and the medical literature now on record.

(vi) Because the Opposite Party has continued to accept the Complainant's premium and has renewed the policy, while refusing the only claim made under it.

34. That wrongful repudiation has kept the Complainant out of his money since 26.06.2025 and has compelled him to pursue the matter through the grievance cell, the Insurance Ombudsman and now this Hon'ble Commission, over a period of more than a year, thereby causing him financial loss, mental agony and harassment, for which he is entitled to compensation, interest and costs.

CAUSE OF ACTION

35. That cause of action arose on 20.06.2025 and 21.06.2025 when the Complainant was hospitalised and operated upon, on 25.06.2025 and 26.06.2025 when the claim was rejected and repudiated, on the subsequent dates when the grievances were rejected, and finally on 02.07.2026 when the Insurance Ombudsman passed the Award with liberty to approach any other forum. The cause of action is a continuing one and has arisen within the jurisdiction of this Hon'ble Commission.

PRAYER

36. That it is, therefore, most respectfully prayed that this Hon'ble Commission may be pleased to:

(a) direct the Opposite Party to pay to the Complainant Rs. 37,358/- (Rupees Thirty-Seven Thousand Three Hundred Fifty-Eight only), being the admissible claim amount under Claim No. 25062100350/1;

(b) award interest on the said amount at the rate of 12% per annum, or at such rate as this Hon'ble Commission may deem fit, from 26.06.2025 till the date of realisation;

(c) award compensation of Rs. 1,00,000/-, or such amount as this Hon'ble Commission may deem fit, for the mental agony and harassment caused to the Complainant by the wrongful repudiation and the prolonged proceedings;

(d) award Rs. 25,000/- towards the cost of the present proceedings; and

(e) pass such other or further order as this Hon'ble Commission may deem fit and proper in the facts and circumstances of the case.

AND FOR THIS ACT OF KINDNESS, THE COMPLAINANT SHALL EVER PRAY.

Place: Ghaziabad

Date: 25 July 2026

COMPLAINANT

(Vikas Gandhi)

VERIFICATION

I, Vikas Gandhi, the Complainant above-named, do hereby verify that the contents of the above complaint are true and correct to my knowledge and belief, based on the documents annexed with the complaint, and that nothing material has been concealed therefrom.

Verified at Ghaziabad on this __25__ day of July, 2026.

COMPLAINANT

(Vikas Gandhi)

BEFORE THE HON'BLE DISTRICT CONSUMER DISPUTES REDRESSAL COMMISSION, GHAZIABAD (U.P.)

CONSUMER COMPLAINT NO. _____ OF 2026

(Complaint under Section 35 of the Consumer Protection Act, 2019)

IN THE MATTER OF:

Mr. Vikas Gandhi …Complainant

VERSUS

ACKO General Insurance Limited …Opposite Party

LIST OF ANNEXURES

Annexure	Document
C-1 (colly.)	Policy Schedule, Policy No. ASHP202400001607 (with "Additional conditions" and Annexure – Specific illness list); renewal Policy Schedule No. ASHP202400001607R1 (04.11.2025–03.11.2026)
C-2	Proposal Form (recording "No medical conditions"; "PED declared — Obesity")
C-3	Premium payment records / policy active status
C-4	Discharge Summary, RS Maitri Health Care, Ghaziabad (20–21.06.2025)
C-5	Hospital bills with break-up of charges (₹37,358/-)
C-6	Claim registration and Complainant's representation dated 24.06.2025
C-7	GRO's rejection email dated 25.06.2025
C-8	Repudiation email dated 26.06.2025
C-9	Histopathology Report, Max Lab, dated 30.06.2025 (No. V-6630/25)
C-10 (colly.)	Subsequent appeals/grievance correspondence with the Opposite Party
C-11	Award of Insurance Ombudsman, Noida, No. IO/NOI/A/HI/0061/2026-2027 dated 02.07.2026, with forwarding letter (Annex-XIII-A)
C-12	Medical Certificate dated 12.07.2026 of the treating super-specialist, Dr. Vaibhav Saxena, M.S. (BHU), M.Ch. Urology (AIIMS, New Delhi), Senior Director – Urology, Uro-Oncology, Robotic Surgery & Renal Transplant, Yashoda Super Speciality Hospital, Kaushambi; U.P. Medical Council Reg. No. 47425
C-13 (colly.)	Medical literature on BXO / Lichen Sclerosus as a premalignant condition: (C-13A) EAU Guidelines on Penile Cancer 2022 (highlighted pages); (C-13B) Bunker CB, Shim TN, "Male Genital Lichen Sclerosus", Indian J Dermatol (review) — see note below
C-14	Copy of Aadhaar / address proof of Complainant

Note on Annexure C-13 (Medical Literature — on record, colly.)

Two authoritative medical documents are annexed as C-13 (colly.), establishing that BXO / genital Lichen Sclerosus is a recognised premalignant condition associated with penile squamous cell carcinoma, and that phimosis (the Complainant's recorded diagnosis) is itself strongly associated with invasive penile cancer:

1. C-13A — European Association of Urology (EAU) Guidelines on Penile Cancer (2022): Table 1 records phimosis as a risk factor with an odds ratio of 11–16 versus no phimosis and states that "Phimosis is strongly associated with invasive penile cancer … due to associated chronic infection"; and Table 4 expressly classifies "Lichen sclerosis" among "Premalignant penile lesions (precursor lesions)" — the very biopsy diagnosis in the present case.

2. C-13B — Bunker CB, Shim TN, "Male Genital Lichen Sclerosus" (Review Article, Indian Journal of Dermatology): records that the feared complication of male genital lichen sclerosus is squamous cell carcinoma of the penis; that "the association of GLSc and SCC is widely recognized and the risk has

been estimated at between 2% and 12.5%" (bracketing the "about 6%" figure noted at the Ombudsman hearing); and that lichen sclerosus is one of the two recognised pathways for the development of penile cancer. This review also cites both Nasca et al. (J Am Acad Dermatol 1999) and Depasquale et al. (BJU Int 2000), the primary sources on the point.

(Relevant highlighted pages are annexed; the treating specialist may also be requested to reference/append this literature to his certificate at Annexure C-12.)

Place: Ghaziabad
Date: 25 July 2026

COMPLAINANT
(Vikas Gandhi)

स्टाम्प विक्रेता
संगीता
लाईसेन्स नं0-598
सदर तहसील

06-Affidavit

IN-UP84509160308257Y

26

INDIA NON JUDICIAL
Government of Uttar Pradesh

सत्यमेव जयते

e-Stamp

₹0000000010

Certificate No.	:	IN-UP84509160308257Y
Certificate Issued Date	:	22-Jul-2026 10:06 AM
Account Reference	:	NEWIMPACC (SV)/ up16003904/ GHAZIABAD SADAR/ UP-GZB
Unique Doc. Reference	:	SUBIN-UPUP16003904623620245460 86Y
Purchased by	:	VIKAS GANDHI
Description of Document	:	Article 4 Affidavit - Registerable
Property Description	:	.
Consideration Price (Rs.)	:	
First Party	:	VIKAS GANDHI
Second Party	:	ACKO GENERAL INSURANCE LIMITED
Stamp Duty Paid By	:	VIKAS GANDHI
Stamp Duty Amount(Rs.)	:	10 (Ten only)

NOTARY
Attested
Reg. No - 3640
Advocate
Notary Public
Ghaziabad
NOTARIAL
GOVT. OF INDIA

IN-UP84509160308257Y

I, VIKAS GANDHI
DECLARE THAT THIS E-STAMP CERTIFICATE IS BEING USED IN ORIGINAL FORM AND IS NOT A PHOTOCOPY

Please write or type below this line

NOTARY
Attested
Reg. No - 3640
Advocate
Notary Public
Ghaziabad
GOVT. OF INDIA

Statutory Alert:

(1) No amount other than the value printed on the e-Stamp is payable in any form for the purchase of e-Stamp. (2) If the above information/declaration is found to be false or incorrect, I shall be personally responsible for the same. (3) The authority before whom the deed is presented shall ensure that the e-stamp is being used only for the purpose and in the name for which it was purchased. (4) The authenticity of this Stamp certificate should be verified at www.shcilestamp.com or using 'E-Stamping' Mobile App of Stock Holding. Any discrepancy in the details on this Certificate and as available on the website/Mobile App renders it invalid.

22-Jul-2026 10:06 AM 22-Jul-2026 10:06 AM 22-Jul-2026 10:06 AM

27
25

BEFORE THE HON'BLE DISTRICT CONSUMER DISPUTES REDRESSAL COMMISSION, GHAZIABAD (U.P.)

CONSUMER COMPLAINT NO. _____ OF 2026

(Under Section 35 of the Consumer Protection Act, 2019)

IN THE MATTER OF:

Vikas Gandhi ...Complainant

VERSUS

ACKO General Insurance Limited ...Opposite Party

AFFIDAVIT

I, Vikas Gandhi, S/o Shri Surendra Gandhi, aged about 53 years, resident of D-20, 1st Floor, Dayanand Nagar, Ghaziabad, Uttar Pradesh – 201001, the Complainant above-named, do hereby solemnly affirm and state on oath as under:

1. That I am the Complainant in the accompanying complaint and am fully conversant with the facts and circumstances of the case, and am competent to swear this affidavit.

2. That the contents of the accompanying complaint under Section 35 of the Consumer Protection Act, 2019 (paragraphs 1 to 30, along with the chronology and list of annexures) have been drafted under my instructions; the contents thereof are true and correct to my knowledge and belief, based upon the documents annexed thereto as Annexures C-1 to C-14, and nothing material has been concealed therefrom.

3. That the documents filed along with the complaint are true copies of their respective originals.

4. That no complaint on the same cause of action is pending before any other Consumer Commission, Court or authority.

DEPONENT
(Vikas Gandhi)

VERIFICATION

Verified at Ghaziabad on this 22nd day of July 2026, that the contents of paragraphs 1 to 4 of the above affidavit are true and correct to my knowledge and belief; no part of it is false and nothing material has been concealed therefrom.

DEPONENT
(Vikas Gandhi)

NOTARY Attested Reg. No.- 3640 Advocate Notary Public Ghaziabad GOVT. OF INDIA

(To be signed before, and attested by, a Notary Public / Oath Commissioner with seal and stamp)

ATTESTED

NOTARY PUBLIC
GOVT. OF INDIA
Reg. No.-3640
Ghaziabad U.P.
22/7/2026

Scanned with OKEN Scanner

ACKO Personal Health Policy

Here's a snapshot of your policy.

Policy holder name:	VIKAS GANDHI
Phone number:	9911198000
Email address:	9amdeal@gmail.com
Pincode:	201001
Policy number:	ASHP202400001607
Plan Name:	Standard Plan
Policy type:	Family Floater
Sum insured:	₹ 10L

Policy validity:	04 November 2024 - 03 November 2025 (1 year)
Payment plan:	Monthly
Instaments paid:	1 out of 12
Premium paid for:	04 November 2024 - 03 December 2024
	Monthly payments are required to keep your policy active
Portability:	No
Existing user:	Yes

Insured members

Name	Date of birth (Age)	Relation to policy holder	Gender	First policy inception date
Sarika Gandhi	02 / 10 / 1977 (47)	Spouse	Female	04 / 11 / 2024
VIKAS GANDHI	09 / 10 / 1972 (52)	Self	Male	04 / 11 / 2024

NOTE: This document contains health cards for all insured members, you can carry them digitally or print a copy.

Health cards

Sarika Gandhi

UHID: ACK.ASHP202400001607.2

DOB: 02-Oct-1977

IPD POLICY NUMBER: ASHP202400001607

VALID TILL: 03-11-2025

VIKAS GANDHI

UHID: ACK.ASHP202400001607.1

DOB: 09-Oct-1972

IPD POLICY NUMBER: ASHP202400001607

VALID TILL: 03-11-2025

Top features

Sum insured

₹10L will never fall short during times of need.

Room rent

No limit on the type of hospital room

ICU fees

ICU charges are fully covered with no limit

Inflation protection

Annual sum insured increased by 10%

Consumables

No deductions on consumables like gloves, masks etc.

What's covered

Benefit	Coverage
In-patient hospitalization	Hospitalization expenses due to any illness, accident, & covid are covered up to ₹10L
Pre & post hospitalisation	Medical expenses incurred 60 days before and 120 days after hospitalisation or day care treatments are covered
Day care treatment	Surgeries and treatments that require less than 24 hours of hospitalization are covered up to sum insured
Covid-19 treatment	Hospitalization costs due to covid-19 will be covered after waiting period
Ambulance charges	No limit on ambulance charges
Domestic evacuation	Evacuation in case of emergency (domestic) covered with no limit
Domiciliary treatment	At-home treatment required due to a condition or injury that normally would be done in a hospital is covered up to sum insured
Organ donor expenses	The cost of the tests, surgery, and treatments involved in getting an organ transplant is covered up to sum insured
Second opinion	Get a second opinion from our extensive doctor network on a diagnosis or high-risk procedure
Ayush treatment	AYUSH (Ayurveda, Yoga and Naturopathy, Unani, Siddha, and Homeopathy.) treatments that require overnight hospitalisation
Annual preventive health checkup	Free health checkup offered once every policy year
Restore SI	Expensive treatments could exhaust your coverage, this benefit will replenish it. The new amount, however, can only be used for a different illness
Doctor on call	This feature will connect you with a doctor on call who can help you by providing medical helpline services

Additional conditions

Sarika Gandhi	Initial waiting period	30 days
	PED waiting period	3 years • Pre-Existing Diseases
	Permanent disease exclusion	No exclusion
	Specific illness	2 years waiting period on specific illness NOTE: Refer Annexure for the complete disease list
	Co-pay	No co-pay
VIKAS GANDHI	Initial waiting period	30 days
	PED waiting period	3 years • Pre-Existing Diseases
	Permanent disease exclusion	No exclusion
	Specific illness	2 years waiting period on specific illness NOTE: Refer Annexure for the complete disease list
	Co-pay	No co-pay

The fine print

Plan details

	Benefit	Coverage
Basic benefits	Base sum insured	₹10L
	In-patient hospitalization	₹10L
	Room rent	Covered up to sum insured
	ICU	Covered up to sum insured
	Day care treatment	All
	Pre or post hospitalization medical expenses	Pre: 60 days | Post: 120 days
	Road Ambulance Limit	Covered up to sum insured
	Domestic Emergency Evacuation Limit	Covered up to sum insured
	Domiciliary Treatment Cover	Covered up to sum insured
	Organ Donor Expenses	Covered up to sum insured
	Second Opinion	Covered up to sum insured
	AYUSH Treatment	Covered up to sum insured
	Modern Treatment	Covered up to sum insured
Basic optional / Add-on benefits	Inflation protect sum insured	10% of sum insured
	Waiver of non-payable medical expenses	Consumables covered
	Doctor on call	Covered up to sum insured
	Annual Preventive Health Check-up	Once a year
	Restore sum insured	Unlimited restore

*It is applicable to members who have a one-year waiting period or no waiting period on specific illnesses

Premium Details

	(₹)	(₹)
Basic Premium		₹ 23,133.43
• Waiver of non-payable medical expenses • Inflation Protect SI • Doctor on call • Preventive Health Check-up • Restore SI	₹ 2,313.35 ₹ 1,041.01 ₹ 134.27 ₹ 1,155 ₹ 462.67	
Basic optional / Add-ons premium		₹ 5,106.3
Loading Amount		₹ 2,825.43
IGST (18%)		₹ 5,591.724
Total annual premium		**₹ 36,656.86**

Frequency of payment:	Monthly	Bank name:	
Monthly premium	₹ 3054.74	Mode of payment:	Online
Instalments paid:	1 out of 12		

Note: You are on a monthly plan. When you file a claim, we might need you to complete the payments for the entire year before we can start processing your claim.

Consolidated Stamp Duty is paid with the Department of Stamps, Bengaluru – Karnataka

Acko General Insurance Limited ACKO IRDAI Regn. No. 157

For ACKO General Insurance Ltd. Duly Constituted Attorney

Get policy wording

This policy schedule needs to be read in conjunction with policy wording.

Download policy wording

Annexure

Specific illness

Any medical expenses related to the following medical disease will not be covered for years mentioned in 'Additional Conditions' section.

- Gastro Disease: Gastroesophageal Reflux Disorder
- Arthrosis of first carpometacarpal joint
- Cleft palate
- Cleft palate with cleft lip
- Cleft lip
- Gout
- Other congenital malformations of circulatory system
- Ventral hernia
- Inflammatory polyneuropathy
- Umbilical hernia
- Pilonidal Sinus
- Gastric ulcer
- Other arthrosis
- Any Surgery of the genitourinary system unless necessitated by malignancy
- Peptic ulcer, site unspecified
- Diaphragmatic hernia
- Other rheumatoid arthritis
- Other polyneuropathies
- Other abdominal hernia
- Retinal vascular occlusions
- Leiomyoma of uterus
- Other abnormal uterine and vaginal bleeding
- Ligament, Tendon & Meniscal Tear (other than caused by Accident)
- Other disorders of lens
- Inguinal hernia
- Seropositive rheumatoid arthritis
- Unspecified kidney failure
- Vascular dementia
- Coxarthrosis [arthrosis of hip]
- Other disorders of nose and nasal sinuses
- Other retinal disorders
- Duodenal ulcer
- Endometriosis
- Suppurative and unspecified otitis media
- Cholelithiasis
- Congenital malformations of cardiac chambers and connections
- Chorioretinal inflammation
- Haemorrhoids and perianal venous thrombosis
- Female genital prolapse
- Gastrojejunal ulcer
- Systemic sclerosis
- Hydrocele and spermatocele
- Oesophageal varices
- Dementia in other diseases classified elsewhere
- Senile cataract
- Retinal disorders in diseases classified elsewhere
- Fissure and fistula of anal and rectal regions
- Abscess of anal and rectal region
- Other congenital malformations of heart
- Dementia in Alzheimer disease
- Other intervertebral disc disorders
- Glaucoma
- Other spondylopathies
- Joint Replacement Surgeries (other than caused by Accident)
- Other diseases of pancreas
- Retinal detachments and breaks

- Calculus of lower urinary tract
- Acute pancreatitis
- Other congenital malformations of peripheral vascular system
- Down's syndrome
- Varicose veins of lower extremities
- Perforation of tympanic membrane
- Polyarteritis nodosa and related conditions
- Congenital malformations of great veins
- Gastritis and duodenitis
- Chronic diseases of tonsils and adenoids
- Alzheimer's
- Gonarthrosis [arthrosis of knee]
- Other dorsopathies, not elsewhere classified
- Parkinson's
- Nonsuppurative otitis media
- Unspecified dementia
- Congenital malformations of aortic and mitral valves
- Cholesteatoma of middle ear
- Chronic kidney disease
- Polyarthrosis
- Congenital malformations of pulmonary and tricuspid valves
- Surgery for Prolapsed intervertebral disc (other than caused by Accident)
- Benign neoplasm of ovary
- Hyperplasia of prostate
- Congenital malformations of great arteries
- Calculus of kidney and ureter
- Femoral hernia
- Rectal Prolapse under Rectal disease
- Psoriatic and enteropathic arthropathies

Annexure

Conditions declared by you

You have declared the following conditions in our conversations.

VIKAS GANDHI

- Obesity

CUSTOMER INFORMATION SHEET

(Description is illustrative and not exhaustive)

Sr. No	Title	Description	Policy Clause Number
1	**Product Name**	Acko Personal Health Policy	
2	**Type of Insurance Product/Policy**	Indemnity	
3	**Sum Insured (Basis)**	Family Floater - 10L	
4	**Policy Number**	ASHP202400001607	
5	**Policy Coverage**	All the below mentioned benefits are covered up to sum insured unless specified otherwise	
		Hospitalization cost for admissions more than 24 hours	Section 3.2.1 & 3.2.2
		All necessary Day Care treatments	Section 3.2.3
		Pre hospitalization medical expenses up to 60 days before admission	Section 3.2.4
		Post Hospitalization medical expenses up to 120 days post discharge	Section 3.2.5
		Road ambulance cost	
		Emergency evacuation from anywhere in India	Section 3.2.6
		Medical treatments or procedures taken at home	Section 3.2.7
		Organ Donor expenses	Section 3.2.8
		Second Opinion for alternate evaluation	Section 3.2.9
		AYUSH Treatment	Section 3.2.10
		Modern Treatment	Section 3.2.11
		Refill of sum insured up to amount specified in the policy schedule	Section 3.3.2
		Preventive Health Check-up to insured above 18 years of age	Section 3.3.11
		Additional sum insured in the subsequent policy namely 'Inflation Protect Sum Insured'	Section 3.3.12
		Access to a doctor or a general medical practitioner any time of the day for a medical consultation.	Section 3.4.1
		Please refer to section 6 below for type of treatments, diseases, situations, expenses etc which are not covered in the product	

6	**Exclusions**	Waiting period or exclusion of some pre-existing diseases and its direct complications are applicable as specified in the policy schedule	Section 4.1.1
		Specified disease/procedure listed in section 4.1.2 of the policy wordings excluded until the number of months as specified	Section 4.1.2
		Treatment of any kind for the first 30 days since first policy commencement	Section 4.1.3
		Expenses only for diagnostics and evaluation purposes	Section 4.1.4
		Expenses for enforced bed rest and not for receiving treatment.	Section 4.1.5
		Surgical treatment of obesity as per conditions specified in section 4.1.6 of the policy wordings	Section 4.1.6
		Expenses for changing characteristics of the body to those of the opposite sex.	Section 4.1.7
		Cosmetic or plastic surgery or any treatment to change appearance unless required due to an Accident, Burn(s) or Cancer	Section 4.1.8
		Treatment in any hospital or by any Medical Practitioner or any other provider specifically excluded by the Insurer	Section 4.1.9
		Treatments in health hydros, nature cure clinics, spas or similar establishments	Section 4.1.10
		Dietary supplements and substances that can be purchased without prescription, unless prescribed by a medical practitioner as part of hospitalization claim or day care procedure.	Section 4.1.11
		Treatment for correction of eye sight due to refractive error less than 7.5 dioptres.	
		Expenses related to sterility and infertility.	Section 4.1.12
		Medical treatment expenses traceable to childbirth or miscarriage	Section 4.1.13
		Expenses for Non-Medical Expenses as listed in Annexure 1 in the policy wordings for any hospitalization, day care or domiciliary treatment	Section 4.1.14
		.Self-inflicted Injury .Breach of law .Other sexually transmitted diseases .Hazardous or Adventure Sports .Unproven and Experimental Treatment .Treatment taken outside India .External Congenital Anomaly or defects .Other Specific Treatments (please refer the section in the policy wordings) .Sleep Disorders .Substance abuse and addictions	Section 4.2.1 Section 4.2.2
		.Dental Treatment unless necessitated due to an Accident .Medically unnecessary Treatment .Prosthetics and Other Devices unless necessitated due to an Accident .War and Exposure to Hazardous Substances Hormonal Therapies	Section 4.2.3

7	**Waiting period**	Sarika Gandhi	Initial waiting period	30	Section 4.1.1 Section 4.1.2 Section 4.1.3
			PED waiting period	3 years • Pre-Existing Diseases	
			PED Permanent Exclusions	No exclusion	
			Specific illness	2 years waiting period on specific illness	
		VIKAS GANDHI	Initial waiting period	30	
			PED waiting period	3 years • Pre-Existing Diseases	
			PED Permanent Exclusions	No exclusion	
			Specific illness	2 years waiting period on specific illness	

8	**Financial limits of coverage** **i. Sub-limit** **ii. Co-payment** **iii. Deductible** **iv. Any other limit (as applicable)**	This product has no copay or sub-limits applicable on any reasonable costs for treatments covered. Deductible of Rs. None on aggregate claim basis	Section 3.2.2, 3.3.4, 3.3.5, 3.3.6 Section 3.3.7
9	**Claims/ Claims Procedure**	Cashless claim facility can be availed in all network hospitals. The list of network hospitals are available on our website or can be checked at the customer care centre. For reimbursement of a claim, please submit all necessary documents on our App or email to us. We may ask for original hard copy of the documents in some cases. Please find the important links/numbers below :- i. Network Hospital Details: Acko App or www.acko.com ii. Helpline Number: 1860 266 2256 iii. Hospitals which are backlisted or from where no claims will be accepted by the insurer: Acko App or www.acko.com iv. Downloading getting the claim form: Acko App or www.acko.com	Section 6.1
10	**Policy Servicing/ Grievances/Complaints**	● Company Officials: Acko General Insurance Limited, 2nd floor, #36/5, Hustlehub One East, 27th Main Rd, Sector 2, HSR Layout, Bengaluru, Karnataka - 560102 ● Our website: www.acko.com ● Email: grievance@acko.com ● Toll Free: 1860 266 2256	Section 6.3

11	**Things to remember**	You may cancel the policy within 15 days of receipt without any charges. And if there are no claims during this period. We will not deny renewal of your policy if you wish to renew with us as long as the applicable premium is paid on time. Migration and Porting of this Policy is allowed as per the applicable rules. For Detailed Guidelines on Migration, kindly refer the link:- https://irdai.gov.in/document-detail?documentId=393128 For Detailed Guidelines on Portability, kindly refer the link:- https://irdai.gov.in/document-detail?documentId=393128 Changes in Policy coverage such as change in sum insured, addition/deletion of insured etc are allowed subject to underwriting by the company. For other product related documents, kindly refer the link:- http://www.acko.com/download	Section 5.1.18 Section 5.1.15 Section 5.1.14 Section 5.2.13 Section 5.1.20
12	**Insured's Obligations**	If any of the facts provided to us to purchase this Policy are found to be incorrect, incomplete, suppressed or not disclosed, the policy shall be canceled without refund of premium after 30 days' notice. Any claim made under such Policy, shall be rejected.	Section 5.1.1, Section 5.2.2
Legal Disclaimer Note: The information must be read in conjunction with the product brochure and policy document. In case of any conflict between the CIS and the policy document, the terms and conditions mentioned in the policy document shall prevail.			

Declaration by the Policy Holder;

I have read the above and confirm having noted the details.

Place:

Date:

_____

Signature of the Policy Holder

Gmail

9AM DEAL <9amdeal@gmail.com>

Your ACKO health plan is ready.

1 message

Acko health <health@acko.com> Tue, Nov 5, 2024 at 6:12 PM
To: 9amdeal@gmail.com

Welcome to ACKO health

Thank you for choosing ACKO to cover you and your family.
Please find attached your policy document.

Access your ACKO health plan in the ACKO app.

Get the app GET IT ON Google Play AVAILABLE ON App Store

Please confirm that you have received the policy document and Customer Information Sheet (CIS) for policy number ASHP202400001607

I confirm

Acko Standard Health Plan

Family members:
Sarika Gandhi and VIKAS GANDHI

Sum insured:
₹ 10L

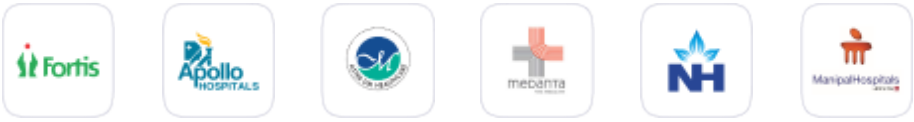
Premium paid:
₹ 36656.88

Policy valid till:
3 Nov 2025

We are here to help. You can reach us at health@acko.com or call **1800 266 2256**.

Quick cashless claims at 7100+ hospitals

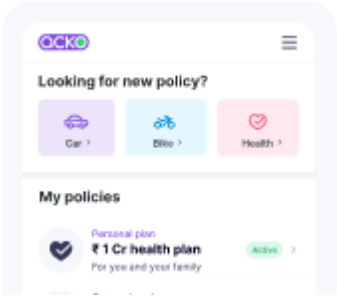
1. Use the ACKO app to locate an ACKO Network Hospital in your area.
2. Get your e-cards from the app & share it with the mediclaim helpdesk at the hospital
3. The helpdesk will then get the cashless treatment approved for you.
4. Post treatment, go to the billing section to pay any remaining balance and get a quick discharge.



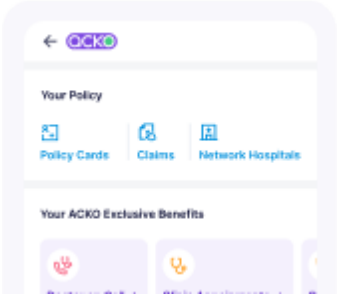
See all the 7100+ hospitals

Hassle-free reimbursement claims with the ACKO app

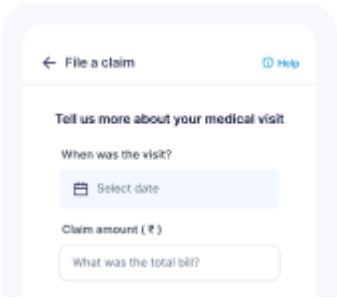
1 Download the app & select your policy



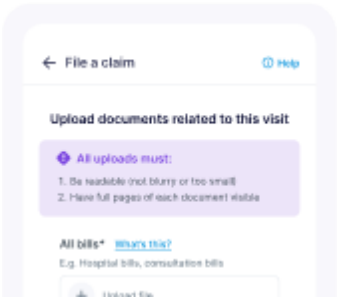
2 Tap on claims & then click on “File a claim”



3 Share hospitalisation info



4 Upload all required documents



Get the ACKO app

Manage your policies & claims at your fingertips

GET IT ON Google Play AVAILABLE ON App Store

4.7 ★ out of 5 4.9 ★ out of 5

Acko General Insurance Ltd.

36/5, Somasandra palya, Haralukunte village, Adjacent 27th main road, sector 2, HSR layout, Bengaluru Urban, Bengaluru, Karnataka, 560102, Contact No. 1800 266 2256, Email: hello@acko.com
website: www.acko.com

IRDAI Regn No. : 157 | CIN : U66000KA2016PLC138288 | ACKO health insurance policy, Acko General Insurance | UIN : ACKHLIP25035V022425 | For more details on the risk factors, terms and conditions please read the policy wordings available on www.acko.com carefully before concluding the sale.

Trade Logo displayed above belongs to Acko Technology & Services Pvt Ltd. and used by Acko General Insurance Ltd under license.

3 attachments

VIKAS GANDHI proposal form.pdf
73K

VIKAS GANDHI policy document.pdf
275K

VIKAS GANDHI Customer Information Sheet.pdf
50K

CUSTOMER INFORMATION SHEET

(Description is illustrative and not exhaustive)

Sr. No	Title	Description	Policy Clause Number
1	**Product Name**	Acko Personal Health Policy	
2	**Type of Insurance Product/Policy**	Indemnity	
3	**Sum Insured (Basis)**	Family Floater - 10L	
4	**Policy Number**	ASHP202400001607R1	
5	**Policy Coverage**	All the below mentioned benefits are covered up to sum insured unless specified otherwise	
		Hospitalization cost for admissions more than 24 hours	Section 3.2.1 & 3.2.2
		All necessary Day Care treatments	Section 3.2.3
		Pre hospitalization medical expenses up to 60 days before admission	Section 3.2.4
		Post Hospitalization medical expenses up to 120 days post discharge	Section 3.2.5
		Road ambulance cost	
		Emergency evacuation from anywhere in India	Section 3.2.6
		Medical treatments or procedures taken at home	Section 3.2.7
		Organ Donor expenses	Section 3.2.8
		Second Opinion for alternate evaluation	Section 3.2.9
		AYUSH Treatment	Section 3.2.10
		Modern Treatment	Section 3.2.11
		Refill of sum insured up to amount specified in the policy schedule	Section 3.3.2
		Preventive Health Check-up to insured above 18 years of age	Section 3.3.11
		Additional sum insured in the subsequent policy namely 'Inflation Protect Sum Insured'	Section 3.3.12
		Access to a doctor or a general medical practitioner any time of the day for a medical consultation.	Section 3.4.1
		Please refer to section 6 below for type of treatments, diseases, situations, expenses etc which are not covered in the product	

6	**Exclusions**	Waiting period or exclusion of some pre-existing diseases and its direct complications are applicable as specified in the policy schedule	Section 4.1.1
		Specified disease/procedure listed in section 4.1.2 of the policy wordings excluded until the number of months as specified	Section 4.1.2
		Treatment of any kind for the first 30 days since first policy commencement	Section 4.1.3
		Expenses only for diagnostics and evaluation purposes	Section 4.1.4
		Expenses for enforced bed rest and not for receiving treatment.	Section 4.1.5
		Surgical treatment of obesity as per conditions specified in section 4.1.6 of the policy wordings	Section 4.1.6
		Expenses for changing characteristics of the body to those of the opposite sex.	Section 4.1.7
		Cosmetic or plastic surgery or any treatment to change appearance unless required due to an Accident, Burn(s) or Cancer	Section 4.1.8
		Treatment in any hospital or by any Medical Practitioner or any other provider specifically excluded by the Insurer	Section 4.1.9
		Treatments in health hydros, nature cure clinics, spas or similar establishments	Section 4.1.10
		Dietary supplements and substances that can be purchased without prescription, unless prescribed by a medical practitioner as part of hospitalization claim or day care procedure.	Section 4.1.11
		Treatment for correction of eye sight due to refractive error less than 7.5 dioptres.	
		Expenses related to sterility and infertility.	Section 4.1.12
		Medical treatment expenses traceable to childbirth or miscarriage	Section 4.1.13
		Expenses for Non-Medical Expenses as listed in Annexure 1 in the policy wordings for any hospitalization, day care or domiciliary treatment	Section 4.1.14
		.Self-inflicted Injury .Breach of law .Other sexually transmitted diseases .Hazardous or Adventure Sports .Unproven and Experimental Treatment .Treatment taken outside India .External Congenital Anomaly or defects .Other Specific Treatments (please refer the section in the policy wordings) .Sleep Disorders .Substance abuse and addictions	Section 4.2.1 Section 4.2.2
		.Dental Treatment unless necessitated due to an Accident .Medically unnecessary Treatment .Prosthetics and Other Devices unless necessitated due to an Accident .War and Exposure to Hazardous Substances Hormonal Therapies	Section 4.2.3

7	**Waiting period**	Sarika Gandhi	PED waiting period	2 years • Pre-Existing Diseases	Section 4.1.1 Section 4.1.2 Section 4.1.3
			PED Permanent Exclusions	No exclusion	
			Specific illness	1 years waiting period on specific illness	
		VIKAS GANDHI	PED waiting period	2 years • Pre-Existing Diseases	
			PED Permanent Exclusions	No exclusion	
			Specific illness	1 years waiting period on specific illness	

8	**Financial limits of coverage** **i. Sub-limit** **ii. Co-payment** **iii. Deductible** **iv. Any other limit (as applicable)**	This product has no copay or sub-limits applicable on any reasonable costs for treatments covered. Deductible of Rs. None on aggregate claim basis	Section 3.2.2, 3.3.4, 3.3.5, 3.3.6 Section 3.3.7
9	**Claims/ Claims Procedure**	Cashless claim facility can be availed in all network hospitals. The list of network hospitals are available on our website or can be checked at the customer care centre. For reimbursement of a claim, please submit all necessary documents on our App or email to us. We may ask for original hard copy of the documents in some cases. Please find the important links/numbers below :- i. Network Hospital Details: Acko App or www.acko.com ii. Helpline Number: 1860 266 2256 iii. Hospitals which are backlisted or from where no claims will be accepted by the insurer: Acko App or www.acko.com iv. Downloading getting the claim form: Acko App or www.acko.com	Section 6.1
10	**Policy Servicing/ Grievances/Complaints**	● Company Officials: Acko General Insurance Limited, 2nd floor, #36/5, Hustlehub One East, 27th Main Rd, Sector 2, HSR Layout, Bengaluru, Karnataka - 560102 ● Our website: www.acko.com ● Email: grievance@acko.com ● Toll Free: 1860 266 2256 The contact details of the Insurance Ombudsman offices have been provided in the policy wordings.	Section 6.2

11	**Things to remember**	You may cancel the policy within 30 days of receipt without any charges. And if there are no claims during this period. We will not deny renewal of your policy if you wish to renew with us as long as the applicable premium is paid on time. Migration and Porting of this Policy is allowed as per the applicable rules. For Detailed Guidelines on Migration, kindly refer the link:- https://irdai.gov.in/document-detail?documentId=393128 For Detailed Guidelines on Portability, kindly refer the link:- https://irdai.gov.in/document-detail?documentId=393128 Changes in Policy coverage such as change in sum insured, addition/deletion of insured etc are allowed subject to underwriting by the company. We will not deny any claim after the moratorium period of 5 years of continuous coverage unless due to fraud or permanent exclusion. For other product related documents, kindly refer the link:- http://www.acko.com/download	Section 5.1.18 Section 5.1.15 Section 5.1.14 Section 5.2.13 Definition 31 in Section 2.1 Section 5.1.20
12	**Insured's Obligations**	Please disclose all pre-existing disease/s or condition/s before buying a policy. Non-disclosure may affect the claim settlement.	
Legal Disclaimer Note: The information must be read in conjunction with the product brochure and policy document. In case of any conflict between the CIS and the policy document, the terms and conditions mentioned in the policy document shall prevail.			

Declaration by the Policy Holder;

I have read the above and confirm having noted the details.

Place:

Date:

_____

Signature of the Policy Holder

ACKO Personal Health Policy

Here's a snapshot of your policy.

Policy holder name:	VIKAS GANDHI	Policy validity:	04 November 2025 - 03 November 2026 (1 year)
Phone number:	9911198000	Payment plan:	Monthly
Email address:	9amdeal@gmail.com	Instaments paid:	1 out of 12
Pincode:	201001	Premium paid for:	04 November 2025 - 03 December 2025
Policy number:	ASHP202400001607R1		Monthly payments are required to keep your policy active
Plan Name:	Standard Plan	Portability:	No
Policy type:	Family Floater	Existing user:	Yes
Sum insured:	₹ 11L		
Product category:	Renewal		

Insured members

Name	Date of birth (Age)	Relation to policy holder	Gender	First policy inception date
VIKAS GANDHI	09 / 10 / 1972 (53)	Self	Male	04 / 11 / 2024
Sarika Gandhi	02 / 10 / 1977 (48)	Spouse	Female	04 / 11 / 2024

NOTE: This document contains health cards for all insured members, you can carry them digitally or print a copy.

Sum insured

Base sum insured	Inflation protection bonus	Total sum insured
₹10L	₹1L	₹11L

Health cards

VIKAS GANDHI

UHID: ACK.ASHP2024000 01607R1.1

DOB: 09-Oct-1972

IPD POLICY NUMBER: ASHP202400001607 R1

VALID TILL: 03-11-2026

ACKO FHPL

Sarika Gandhi

UHID: ACK.ASHP2024000 01607R1.2

DOB: 02-Oct-1977

IPD POLICY NUMBER: ASHP202400001607 R1

VALID TILL: 03-11-2026

ACKO FHPL

Top features

Sum insured

11L coverage for health expenses

100% bill payment

Your full hospital bill will be settled.

No limit on room rent

Pick any hospital room with no restrictions

ICU charges

ICU charges covered up to sum insured

Inflation protection

Your sum insured will increase by 10% every year

Single-use disposables covered

Masks, gloves, syringes, etc are covered

What's covered

Benefit	Details
In-patient Hospitalization	Hospitalization expenses due to any illness, accident, & COVID-19 are covered up to 11L
Pre & Post Hospitalization	Medical expenses incurred 60 days before and 120 days after hospitalization or day care treatments are covered
Day Care Treatment	Surgeries and treatments that require less than 24 hours of hospitalization are covered up to the sum insured
COVID-19 Treatment	Hospitalization costs due to COVID-19 will be covered after the waiting period
Ambulance Charges	No limit on ambulance charges
Domestic Evacuation	Evacuation in case of emergency (domestic) covered with no limit
Domiciliary Treatment	At-home treatment required due to a condition or injury that normally would be done in a hospital is covered up to the sum insured
Organ Donor Expenses	The cost of the tests, surgery, and treatments involved in getting an organ transplant is covered up to the sum insured
Second Opinion	Get a second opinion from our extensive doctor network on a diagnosis or high-risk procedure
Ayush treatment	AYUSH (Ayurveda, Yoga and Naturopathy, Unani, Siddha, and Homeopathy.) treatments that require overnight hospitalisation
Annual Preventive Health Checkup	Free health checkup offered once every policy year
Restore SI	Expensive treatments could exhaust your coverage, this benefit will replenish it. The new amount, however, can only be used for a different illness
Doctor on Call	This feature will connect you with a doctor on call who can help you by providing medical helpline services

Member-level waiting periods

Initial waiting period - No policy benefits are covered during the initial waiting period. The insured members can get treated or submit a claim after this waiting period of 30 days is over. As an exception, hospitalisation/treatment for injuries caused due to an accident is covered from day 1.

Specific illness waiting period - Treatment for some medical conditions are not covered during the specific illness waiting period. The insured members can get treated or submit a claim for those medical conditions after this waiting period is over. See list of specific illnesses on page 6.

Pre-existing disease (PED) waiting period - Treatments for health conditions that are declared during health evaluation will not be covered during the pre-existing diseases (PED) waiting period. The insured members can get treated or submit a claim for their previously declared health conditions only after this waiting period is over. Please note that treatments for health conditions that are not declared during health evaluation will not be covered.

Sarika Gandhi	Initial waiting period	No waiting period
	PED waiting period	2 years • Pre-Existing Diseases
	Permanent disease exclusion	No exclusion
	Specific illness	1 years waiting period on specific illness NOTE: Refer Annexure for the complete disease list
	Co-pay	No co-pay
VIKAS GANDHI	Initial waiting period	No waiting period
	PED waiting period	2 years • Pre-Existing Diseases
	Permanent disease exclusion	No exclusion
	Specific illness	1 years waiting period on specific illness NOTE: Refer Annexure for the complete disease list
	Co-pay	No co-pay

Acko General Insurance Limited
36/5, Somasandra palya, Haralukunte village, Adjacent 27th main road, sector 2, HSR layout, Bengaluru Urban, Bengaluru, Karnataka, 560102

IRDAI Reg No.: 157
HSN: 997133
GST: 29AAOCA9055C1ZF

Product: ACKO Personal Health Policy
CIN: U66000KA2016PLC138288
UIN: ACKHLIP25035V022425

The fine print

Plan details

	Benefit	Coverage
Basic benefits	Base sum insured	₹10L
	In-patient hospitalization	₹10L
	Room rent	Covered up to sum insured
	ICU	Covered up to sum insured
	Day care treatment	All
	Pre or post hospitalization medical expenses	Pre: 60 days | Post: 120 days
	Road Ambulance Limit	Covered up to sum insured
	Domestic Emergency Evacuation Limit	Covered up to sum insured
	Domiciliary Treatment Cover	Covered up to sum insured
	Organ Donor Expenses	Covered up to sum insured
	Second Opinion	Covered up to sum insured
	AYUSH Treatment	Covered up to sum insured
	Modern Treatment	Covered up to sum insured
Basic optional / Add-on benefits	Waiver of non-payable medical expenses	Consumables covered
	Doctor on call	Covered up to sum insured
	Inflation protect sum insured	10% of sum insured
	Annual Preventive Health Check-up	Once a year
	Restore sum insured	100% restore

Acko General Insurance Limited
36/5, Somasandra palya, Haralukunte village, Adjacent 27th main road, sector 2, HSR layout, Bengaluru Urban, Bengaluru, Karnataka, 560102

IRDAI Reg No.: 157
HSN: 997133
GST: 29AAOCA9055C1ZF

Product: ACKO Personal Health Policy
CIN: U66000KA2016PLC138288
UIN: ACKHLIP25035V022425

Premium Details

	(₹)	(₹)
Basic Premium		₹ 23,133.44
• Doctor on call • Waiver of non-payable medical expenses • Restore SI • Inflation Protect SI • Preventive Health Check-up	₹ 134.27 ₹ 2,313.34 ₹ 462.66 ₹ 1,041 ₹ 1,155	
Basic optional / Add-ons premium		₹ 5,106.27
Loading Amount		₹ 2,825.43
IGST (18%)		₹ 5,591.71
Total annual premium		**₹ 36,656.86**

Frequency of payment:	Monthly	Bank name:	
Monthly premium	₹ 3054.74	Mode of payment:	Online
Instalments paid:	1 out of 12		

Note: You are on a monthly plan. When you file a claim, we might need you to complete the payments for the entire year before we can start processing your claim.

Consolidated Stamp Duty is paid with the Department of Stamps, Bengaluru – Karnataka

For ACKO General Insurance Ltd. Duly Constituted Attorney

Get policy wording

This policy schedule needs to be read in conjunction with policy wording.

Download policy wording

Annexure

Specific illness

Any medical expenses related to the following medical disease will not be covered for years mentioned in 'Additional Conditions' section.

- Cholesteatoma
- Disease of middle ear and mastoid including otitis media
- Nasal septum
- Nasal sinuses
- Perforation of tympanic membrane
- Tonsils and adenoids

- Cataract
- Disorders of lens
- Disorders of retina
- Glaucoma

- Abnormal utero-vaginal bleeding
- Endometriosis/Adenomyosis
- Female genital prolapse
- Fibroids
- PCOD, or any condition requiring dilation and curettage or hysterectomy

- Arthrosis such as RA
- ADorsopathies
- Gout
- Inflammatory polyarthropathies
- Intervertebral disc disorders
- Osteoarthritis
- Spondylopathies
- Systemic connective tissue disorders

- Chronic kidney disease and failure

- Pancreatitis

- Hyperplasia of prostate
- Hydrocele
- Spermatocele

- Fissure or Fistula
- Haemorrhoids

- Erosion
- Ulcer
- Varices of upper gastrointestinal tract

- Varicose veins of lower extremities

- All internal or external benign or In situ neoplasms/tumours, cyst, sinus, polyp, nodules, swelling, mass or lump
- Any birth(congenital) disorders
- Hernia
- Stones in biliary and urinary system

Acko General Insurance Limited
36/5, Somasandra palya, Haralukunte village, Adjacent 27th main road, sector 2, HSR layout, Bengaluru Urban, Bengaluru, Karnataka, 560102

IRDAI Reg No.: 157
HSN: 997133
GST: 29AAOCA9055C1ZF

Product: ACKO Personal Health Policy
CIN: U66000KA2016PLC138288
UIN: ACKHLIP25035V022425

Annexure

Conditions declared by you

You have declared the following conditions in our conversations.

VIKAS GANDHI

- Obesity

Proposal form

Here's a snapshot of all the information you gave us.

Name:	VIKAS GANDHI	Plan Name:	Standard Plan
Phone number:	9911198000	Gender:	Male
Email address:	9amdeal@gmail.com	Nationality:	Indian
Pincode:	201001	Portability:	No
Date of Birth:	9 Oct 1972	Existing user:	Yes

Insured members

Name	Date of birth (Age)	Relation	Gender	Height (feet, inch)	Weight (kgs)
Sarika Gandhi	02 / 10 / 1977 (47)	Spouse	Female	5'4	59
VIKAS GANDHI	09 / 10 / 1972 (52)	Self	Male	5'6	102

What's covered

	Benefit	Coverage
Basic benefits	Base sum insured	₹10L
	In-patient hospitalization	₹10L
	Room rent	Covered up to sum insured
	ICU	Covered up to sum insured
	Day care treatment	All
	Pre or post hospitalization medical expenses	Pre: 60 days | Post: 120 days
	Road Ambulance Limit	Covered up to sum insured
	Domestic Emergency Evacuation Limit	Covered up to sum insured
	Domiciliary Treatment Cover	Covered up to sum insured
	Organ Donor Expenses	Covered up to sum insured
	Second Opinion	Covered up to sum insured
	Ayush Treatment	Covered up to sum insured
	Modern Treatment	Covered up to sum insured
Basic optional / Add-on benefits	Inflation protect sum insured	10% of sum insured
	Waiver of non-payable medical expenses	Consumables covered
	Doctor on call	Covered up to sum insured
	Annual Preventive Health Check-up	Once a year
	Restore sum insured	Unlimited restore

Additional conditions

Sarika Gandhi	Initial waiting period	30 days
	PED waiting period	3 years • Pre-Existing Diseases
	Permanent disease exclusion	No exclusion
	Specific illness	2 years waiting period on specific illness NOTE: Refer Annexure for the complete disease list
	Co-pay	No co-pay
VIKAS GANDHI	Initial waiting period	30 days
	PED waiting period	3 years • Pre-Existing Diseases
	Permanent disease exclusion	No exclusion
	Specific illness	2 years waiting period on specific illness NOTE: Refer Annexure for the complete disease list
	Co-pay	No co-pay

Premium Details

	(₹)
Basic Premium	₹ 23,133.43
Basic optional / Add-ons premium	₹ 5,106.3
Loading amount	₹ 2,825.43
IGST (18%)	₹ 5,591.724
Total annual premium	**₹ 36,656.86**

Frequency of payment: Monthly
Monthly premium ₹ 3054.74
Instalments paid: 1 out of 12

Bank name:
Mode of payment: Online

Medical history of insured person(s)

	Your answer
Has anyone covered in this plan smoked or chewed tobacco in the past year?	No
Has anyone covered in this plan consumed alcohol in the past year?	No
Has anyone in this plan been diagnosed with a medical condition?	No medical conditions
Did anyone in this plan take any prescribed medicines in the past week?	No
Is anyone in this plan currently experiencing any of the following symptoms?	No symptoms
Has anyone in this plan been advised to undergo or has ever undergone hospitalisation for any illness / surgery?	no
Are you or your spouse pregnant?	No
Does anyone in this plan have any other ongoing health issues or symptoms for which you are planning a doctor visit?	No
PED declared	Obesity

Declaration

1. I hereby declare, on my behalf and on behalf of all persons proposed to be insured, that the above statements, answers and/or particulars given by me are true and complete in all respects to the best of my knowledge and that I am authorised to propose on behalf of these other persons.
2. I understand that the information provided by me will form the basis of the insurance policy, is subject to the Board approved underwriting policy of the insurer and that the policy will come into force only after full payment of the premium chargeable.
3. I further declare that I will notify in writing any change occurring in the occupation or general health of the life to be insured/proposer after the proposal has been submitted but before communication of the risk acceptance by the company.
4. I declare that I consent to the company seeking medical information from any doctor or hospital who/which at any time has attended on the person to be insured/proposer or from any past or present employer concerning anything which affects the physical or mental health of the person to be insured/proposer and seeking information from any insurer to whom an application for insurance on the person to be insured /proposer has been made for the purpose of underwriting the proposal and/or claim settlement.
5. I authorize the company to share information pertaining to my proposal including the medical records for the sole purpose of proposal underwriting and / or claims settlement and with any Governmental and / or Regulatory authority.
6. I hereby permit/ authorise Acko General Insurance to collect, store, communicate and process information relating to the policy(ies)and all transactions related therewith, including the sharing and disclosing the public authorities, of any confidential information as required by law and to send me information in relation to the Policy and Acko General Insurance products & services, irrespective of whether I am registered with National Customer Preference Register (NCPR) [(Formerly the National Do Not Call Registry (NDNC)] or not.
7. To protect the environment and save paper, I hereby give my consent to Acko General Insurance to send me the executed policy copy and all related documents and other communications in electrionic form by way of email to the aforesaid email id instead of physical form and also to share all such documents and any updates & alerts via Whatsapp on my registered mobile number with the company.

I submit that the foregoing information is true to the best of my knowledge and accept that if found to be untrue in any form, the Company reserves the right to alter/ cancel the coverage available under this Policy.

NOTE: The liability of the Company does not commence until full premium has been realized by the Company and the acceptance of the proposal has been formally intimated to the insured.

Prohibition of rebates (Section 41 of insurance Act, 1938, as amended)

1. No person shall allow or offer to allow, either directly or indirectly, as an inducement to any person to take out or continue an insurance in respect of any kind of risk relating to lives or property in India, any rebate of the whole or part of the commission payable or any rebate of premium shown on the policy, nor shall any person taking out or renewing or continuing a policy accept any rebate, except such rebate as may be allowed in accordance with the prospectus or tables of the insurers.
2. Any person making default in complying with the provisions of this section shall be liable for penalty which may extend to ₹ 10 lakh.

Proposal number: 505202411310963

Annexure

Waiting period

Any medical expenses related to the following medical disease will not be covered for years mentioned in 'Additional Conditions' section.

- Gastro Disease: Gastroesophageal Reflux Disorder
- Arthrosis of first carpometacarpal joint
- Cleft palate
- Cleft palate with cleft lip
- Cleft lip
- Gout
- Other congenital malformations of circulatory system
- Ventral hernia
- Inflammatory polyneuropathy
- Umbilical hernia
- Pilonidal Sinus
- Gastric ulcer
- Other arthrosis
- Any Surgery of the genitourinary system unless necessitated by malignancy
- Peptic ulcer, site unspecified
- Diaphragmatic hernia
- Other rheumatoid arthritis
- Other polyneuropathies
- Other abdominal hernia
- Retinal vascular occlusions
- Leiomyoma of uterus
- Other abnormal uterine and vaginal bleeding
- Ligament, Tendon & Meniscal Tear (other than caused by Accident)
- Other disorders of lens
- Inguinal hernia
- Seropositive rheumatoid arthritis
- Unspecified kidney failure
- Vascular dementia
- Coxarthrosis [arthrosis of hip]
- Other disorders of nose and nasal sinuses
- Other retinal disorders
- Duodenal ulcer
- Endometriosis
- Suppurative and unspecified otitis media
- Cholelithiasis
- Congenital malformations of cardiac chambers and connections
- Chorioretinal inflammation
- Haemorrhoids and perianal venous thrombosis
- Female genital prolapse
- Gastrojejunal ulcer
- Systemic sclerosis
- Hydrocele and spermatocele
- Oesophageal varices
- Dementia in other diseases classified elsewhere
- Senile cataract
- Retinal disorders in diseases classified elsewhere
- Fissure and fistula of anal and rectal regions
- Abscess of anal and rectal region
- Other congenital malformations of heart
- Dementia in Alzheimer disease
- Other intervertebral disc disorders
- Glaucoma
- Other spondylopathies
- Joint Replacement Surgeries (other than caused by Accident)
- Other diseases of pancreas
- Retinal detachments and breaks
- Calculus of lower urinary tract
- Acute pancreatitis
- Other congenital malformations of peripheral vascular system

- Down's syndrome
- Varicose veins of lower extremities
- Perforation of tympanic membrane
- Polyarteritis nodosa and related conditions
- Congenital malformations of great veins
- Gastritis and duodenitis
- Chronic diseases of tonsils and adenoids
- Alzheimer's
- Gonarthrosis [arthrosis of knee]
- Other dorsopathies, not elsewhere classified
- Parkinson's
- Nonsuppurative otitis media
- Unspecified dementia
- Congenital malformations of aortic and mitral valves
- Cholesteatoma of middle ear
- Chronic kidney disease
- Polyarthrosis
- Congenital malformations of pulmonary and tricuspid valves
- Surgery for Prolapsed intervertebral disc (other than caused by Accident)
- Benign neoplasm of ovary
- Hyperplasia of prostate
- Congenital malformations of great arteries
- Calculus of kidney and ureter
- Femoral hernia
- Rectal Prolapse under Rectal disease
- Psoriatic and enteropathic arthropathies

Annexure

Conditions declared by you

You have declared the following conditions in our conversations.

VIKAS GANDHI

- Obesity

Gmail

9AM DEAL <9amdeal@gmail.com>

Payment successful for Acko General Insurance Limited

1 message

Payments <no-reply@razorpay.com> Sat, Jun 28, 2025 at 8:26 AM
Reply-To: no-reply@razorpay.com
To: 9amdeal@gmail.com

Acko General Insurance Limited

₹3,054.74

Paid Successfully

Payment Id	pay_QmSVQHqvbPAv6t
Method	upi 9911198000@kotak
Paid On	28 Jun, 2025 08:26:05 AM UTC+5:00
Email	9amdeal@gmail.com
Mobile Number	9911198000

For any order related queries please reach out to Acko General Insurance Limited

Please report this payment if you find it to be suspicious or fraudulent Report Payment

Powered By Razorpay

If you do not wish to receive such e-mail communication in the future, Click here

Gmail

9AM DEAL <9amdeal@gmail.com>

Payment successful for the Acko Standard Health Plan

1 message

Acko health <no-reply@acko.com> Sat, Jun 28, 2025 at 8:26 AM
To: 9amdeal@gmail.com

ACKO

Saturday, June 28, 2025

Hi VIKAS,

Your payment is successful

for the Acko Standard Health Plan

Amount	Payment status
₹3,054	**Paid**

Need help? You can call us any time on 1800 266 2256 or write to us at no-reply@acko.com

Stay healthy!
Team ACKO

Download our app

People love to talk about us

4.7 ★
Google rating

4.9 ★
Facebook rating

Acko General Insurance Ltd.

36/5, Somasandra palya, Haralukunte village, Adjacent 27th main road, sector 2, HSR layout, Bengaluru Urban, Bengaluru, Karnataka, 560102 | Contact No. 1800 266 2256
Email: no-reply@acko.com | website: www.acko.com

IRDAI Regn No. : 157 | CIN : U66000KA2016PLC138288 | ACKO health insurance policy, Acko General Insurance | UIN : ACKHLIP25035V022425

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Gmail

9AM DEAL <9amdeal@gmail.com>

Upcoming AutoPay reminder for monthly instalment

1 message

Acko health <no-reply@acko.com> Sat, Jun 28, 2025 at 8:05 AM
To: 9amdeal@gmail.com

ACKO

Saturday, June 28, 2025

Hi VIKAS,

This is your auto-payment reminder

for the ACKO Standard

We are all set to auto-debit your monthly instalment amount. Please maintain sufficient account balance for us to process your transaction successfully.

Due amount	Due on
3054.74	**29 Jun**

View details

Need help? You can call us any time on 1800 266 2256 or write to us at no-reply@acko.com

Stay healthy!
Team ACKO

Download our app

People love to talk about us

4.7 ★ Google rating

4.9 ★ Facebook rating

Acko General Insurance Ltd.

ACKO General Insurance Ltd. Unit No. 301, 3rd Floor, E Wing, Lotus Corporate Park, Goregaon (E), Mumbai- 400063 | Contact No. 1800 266 2256
Email: no-reply@acko.com | website: www.acko.com

IRDAI Regn No. : 157 | CIN : U66000KA2016PLC138288 | ACKO health insurance policy, Acko General Insurance | UIN : ACKHLIP25035V022425

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Gmail

9AM DEAL <9amdeal@gmail.com>

Upcoming AutoPay reminder for monthly instalment

1 message

Acko health <no-reply@acko.com> Fri, Jun 27, 2025 at 8:48 AM
To: 9amdeal@gmail.com

ACKO

Friday, June 27, 2025

Hi VIKAS,

This is your auto-payment reminder

for the ACKO Standard

We are all set to auto-debit your monthly instalment amount. Please maintain sufficient account balance for us to process your transaction successfully.

Due amount	Due on
3054.74	**29 Jun**

View details

Need help? You can call us any time on 1800 266 2256 or write to us at no-reply@acko.com

Stay healthy!
Team ACKO

Download our app

GET IT ON Google Play AVAILABLE ON App Store

People love to talk about us

4.7 ★ Google rating

4.9 ★ Facebook rating

Acko General Insurance Ltd.
ACKO General Insurance Ltd. Unit No. 301, 3rd Floor, E Wing, Lotus Corporate Park, Goregaon (E), Mumbai- 400063 | Contact No. 1800 266 2256
Email: no-reply@acko.com | website: www.acko.com

IRDAI Regn No. : 157 | CIN : U66000KA2016PLC138288 | ACKO health insurance policy, Acko General Insurance | UIN : ACKHLIP25035V022425

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80D Certificate

Your details

Policy number:	ASHP202400001607	Cover start date:	04 November 2024
Policy holder:	VIKAS GANDHI	Cover end date:	03 November 2025
Product name:	ACKO Health Insurance Policy		

Dear VIKAS GANDHI,

Subject : Certificate for the purpose of deduction under section 80D of Income Tax Act, 1961

This is to certify that Acko General Insurance Ltd. has received an amount of ₹3,054.74/- from you towards Payment of Health insurance premium as per the details mentioned above. The premium paid for this policy is eligible for applicable tax benefits u/s 80D of the Income Tax Act, 1961 and amendments thereof.

NOTE:

1. This is subject to the provisions of Section 80D of Income Tax Act, 1961 as amended from time to time.
2. This certificate must be surrendered to the company in case of cancellation of this policy. In case this certificate is incorrectly used, the policy holder will be held responsible for the same.
3. Please note that this certificate will not be issued if the premium payment has been made in cash.
4. In case the method of the premium payment fails (eg cheque bounce etc.), then the policy will be considered cancelled/void since the start date. This certificate will not be applicable in that case.
5. 80D benefit is applicable for only Self, Spouse, Dependent Children, and Dependent parents.

Date of issue:	04 November 2024
Policy issue office:	Bengaluru

Consolidated Stamp Duty is paid with the Department of Stamps, Bengaluru – Karnataka

Payment date	Instalments paid	Bank name	Mode of payment	Amount paid
04 Nov 2024	1 out of 12	SBM BANK INDIA	CREDIT CARD	2776.9
05 Nov 2024	2 out of 12	SBM BANK INDIA	CREDIT CARD	277.84
			Total	**3,054.74**

Acko General Insurance Limited IRDAI Regn. No. 157

For ACKO General Insurance Ltd. Duly Constituted Attorney

Acko General Insurance Limited
36/5, Somasandra palya, Haralukunte village, Adjacent 27th main road, sector 2, HSR layout, Bengaluru Urban, Bengaluru, Karnataka, 560102

IRDAI Reg No.: 157
HSN: 9971
GST: 29AAOCA9055C1ZF

Product: ACKO personal health policy
CIN: U66000KA2016PLC138288
UIN: ACKHLIP25035V022425

RS MAITRI HEALTH CARE
J-275, Patel Nagar, Ist Ghaziabad-201001, Ph.:0120-4130264
E-mail : rsmaitrihealthcare@gmail.com

Discharge Summary

Name	: Mr. Vikas Gandhi	UHID	: 6033
Phone	: 9871956000	Bill No	: 151/25-26
Age	: 52 y 8 m 12 d	IPD No	: IPD-155
Gender	: Male	Admission Date	: 20-06-2025 06:58
N/A	: N/A	Discharge Date	: 21-06-2025 09:59
Address	: N/A	Bed No	: /
Consultant	: Dr. Vaibhav Saxena/	Panel	: Cash

Diagnosis
TIGHT PHIMOSIS WITH BALANITIS WITH MEATAL STENOSIS.

Admission complaints
TIGHT PHIMOSIS

Brief history of presenting illness
POOR FLOW OF URINE

Relevant past medical/ surgical history

General Physical examination (at the time of admission)
PR :- 80/ Minutes RR :- 20/Minutes
BP :- 130/80mm of Hg SPO2 : 97 % on room air

Systemic Examination (at the time of admission)
CVS :- S1S2+P/A :- Soft, BS+
Chest :- B/L AE +CNS :- Conscious, Oriented

Lab Investigations
ATTACHED

Imaging Studies
ATTACHED

Procedure
CIRCUMCISION + MEATOPLASTY DONE UNDER SA ON 20/06/2025

Course during hospitalization
IV FLUIDS AND IV ANTIBIOTICS

Condition at Discharge
STABLE

Advice on Discharge

Name	Dose	Count	Freq	Duration	When	Remarks
1) TAB FEROCHECK 300			1-0-1	5 days	After Meal	
2) CAP RABRON- D			1-0-0	5 days	Before Breakfast	
3) TAB ADVISOR - P			1-0-1	5 days	After Meal	
4) CAP SILOTIME D 8 MG			0-0-1	5 days	BED TIME	
5) CAP L,B GUT			1-0-1	5 days	After Meal	

RS MAITRI HEALTH CARE
J-278, Patel Nagar, 1st Ghaziabad-201001, Ph.:0120-4130964
E-mail : rsmaitrihealthcare@gmail.com

Discharge Summary

WHEN AND HOW TO OBTAIN URGENT CARE
IN CASE OF FEVER, GIDDINESS, VOMITING, BREATHLESNESS BLEEDING, PROFUSE DISCHARGE, ANY DISCOLOURATION AT SURGICAL SITE OR ANY OTHER EMERGENCIES

PLEASE CONTACT:- 0120-4130964
8527066995

Follow-up Appointment
TO REVIEW IN OPD ON AFTER 3 DAYS DRESSING . REMOVAL OF FOLEYS CATHETER AFTER 3 DAYS , NORMAL DIET, PLENTY OF WATER AVOID STRENUOUS ACTIVITIES.

RMO Signature

Dr. VAIBHAV SAXENA
M.B.B.S., M.S. (Delhi)
Director, Urology, Uro-onco Consultant
R.S Maitri Health Care Reg. No.:
J-278, Patel Nagar 1st Ghaziabad U.P. Phone No.:0120-4130964

Dr. Vaibhav Saxena
UROLOGY

Note :
This is an important document please keep this for further reference & bring on your next visit. In case of any discrepancy due to machine error or typing error, please get it rectified immediately.

RS MAITRI HEALTH CARE
J-278, Patel Nagar, 1st Ghaziabad-201001, Ph.:0120-4130964
E-mail : rsmaitrihealthcare@gmail.com

OPD Bill

Name	: Mr. Vikas Gandhi	UHID	: 6033
Age/Sex	: 52 y 8 m 30 d/Male	OPD No	: OPD-2580
Mobile No	: 9911198000	Bill Date	: 08-07-2025 10:52
Consultant	: Dr. Vaibhav Saxena	Bill ID	: 2785
Department	: UROLOGY	Panel	: Cash
Referred By	: N/A	Token No	: 6
Address	: N/A		

Services

S.No.	Date	Description	Code	Rate	Unit	Dis%	Dis	Amt(Rs)
1	08-07-2025	Consultation Consultation		1,000.00	1	0	0	1,000.00

Gross Amount :	₹1,000.00
Discount :	₹0.00
Net Amount :	₹1,000.00

Payments

S.no	Type	Date	Receipt no	Category	Mode	Note	Payment Details	Amount(₹)
1	Payment	08-07-2025 10:52	2980	Paid	Cash	N/A		1,000.00

09AAMHV5282R1ZW D.L.No.: UP14200000938

RETAIL INVOICE/CASH MEMO

R S MAITRI PHARMACY

J 278, PATEL NAGAR IST, PATEL NAGAR, GHAZIABAD 201001

Ph.

CASH MEMO NO. : 2404 DATE : 22/06/2025

PATIENT NAME : MR VIKASH GANDHI

PATIENT ADD :

PRESCRIBED BY : VAIBHAV SAXENA

PARTICULARS	PACK	QTY	BATCH No.	Exp.	GST%	RATE	Amount
GLOVES 7.5 NO.	1PAIR	1	24K1018	11/29	12.00	97.00	97.00
GLOVES LATEX EXAMINATION	1*1	2	2507202401	01/29	12.00	13.50	27.00
COBAN 4 INCH	1PCS	1	2022-10BS	01/30	12.00	220.00	220.00
BACTIGRAS 10 X 10 CM	1 PCS	1	GS0652	09/27	12.00	35.10	35.10
PUROCIN OINT. 15GM	15GM	1	F3504	01/27	12.00	340.00	340.00
SOLUPERZ TAB	10TAB	10	MT242371A	05/26	12.00	666.00	666.00

Medicine Return Can Be Done Within 45 Days..

INCL. GST DETAILS : 1236.70 X 12 %= 148.42 ,

CGST: 74.21
SGST: 74.21

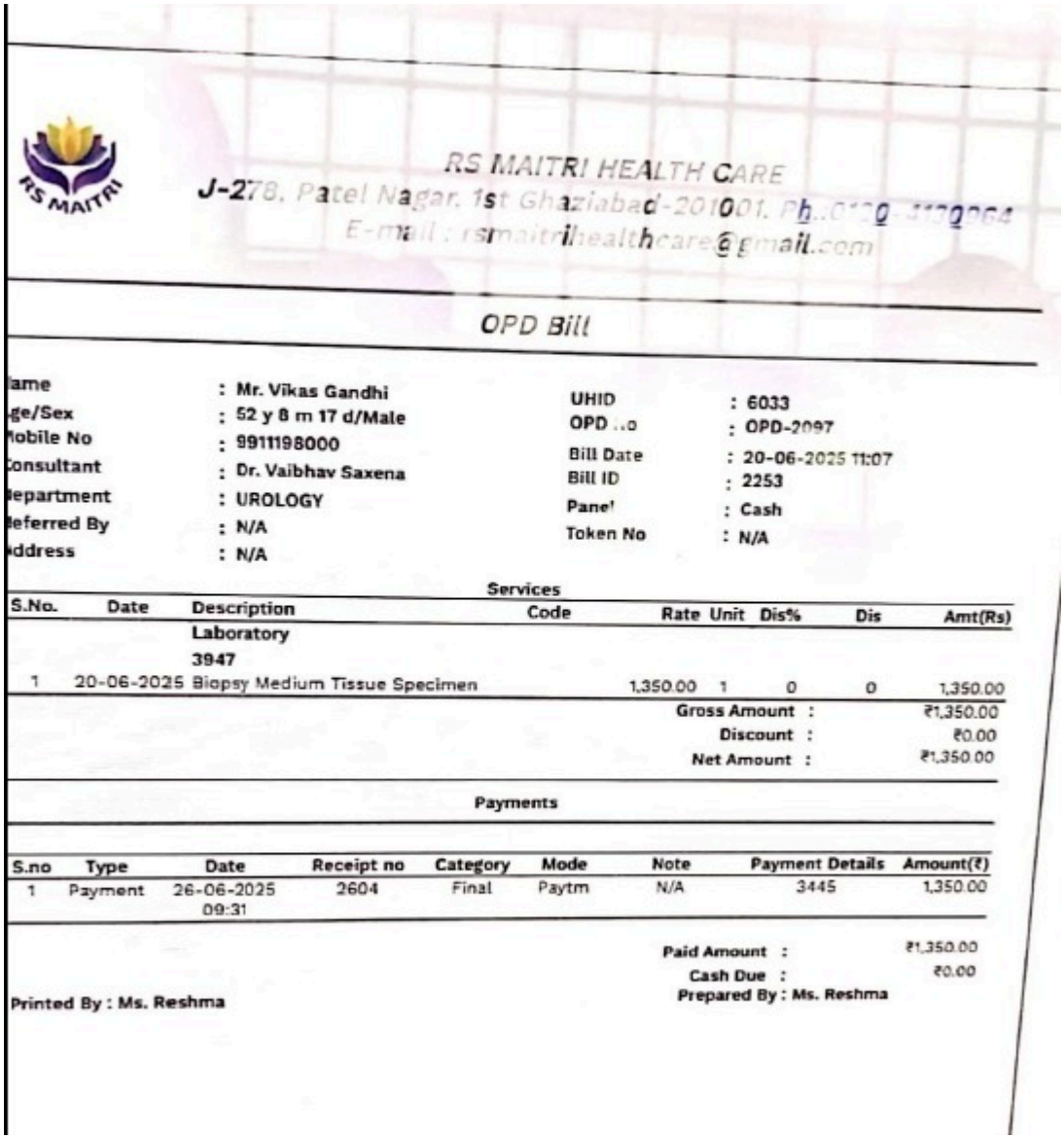
TOTAL AMOUNT : 1385.10
LESS 135.00
Net Amt.(R/0): **1250.00**

Rupees: One Thousand Two Hundred Fifty Only E.& O.E.

All disputes are subject to U.P. Jurisdiction.
Goods once sold will not be taken back without bill
Hence requires no sign.

For R S MAITRI PHARMACY

(Computer Generated Invoice)



RS MAITRI HEALTH CARE

J-278, Patel Nagar, 1st Ghaziabad-201001, Ph..0120-3129064

E-mail : rsmaitrihealthcare@gmail.com

OPD Bill

ame	: Mr. Vikas Gandhi	UHID	: 6033
ge/Sex	: 52 y 8 m 17 d/Male	OPD ..o	: OPD-2097
obile No	: 9911198000	Bill Date	: 20-06-2025 11:07
onsultant	: Dr. Vaibhav Saxena	Bill ID	: 2253
epartment	: UROLOGY	Panel	: Cash
eferred By	: N/A	Token No	: N/A
ddress	: N/A		

Services

S.No.	Date	Description	Code	Rate	Unit	Dis%	Dis	Amt(Rs)
		Laboratory						
		3947						
1	20-06-2025	Biopsy Medium Tissue Specimen		1,350.00	1	0	0	1,350.00

Gross Amount :	₹1,350.00
Discount :	₹0.00
Net Amount :	₹1,350.00

Payments

S.no	Type	Date	Receipt no	Category	Mode	Note	Payment Details	Amount(₹)
1	Payment	26-06-2025 09:31	2604	Final	Paytm	N/A	3445	1,350.00

Paid Amount :	₹1,350.00
Cash Due :	₹0.00

Printed By : Ms. Reshma

Prepared By : Ms. Reshma

JAGGI MEDICOS

SHOP NO-196, PLOT NO-214,
GANDHI NAGAR GHAZIABAD (U.P.)-201001
Phone : 0120-41[illegible] WhatsApp 9319826400
E-Mail : jaggimedicos@gmail.com

Patient Name : CASH
Patient Address :
Dr Name :
Dr.Reg no. :
Card no. :

GSTIN : 09AKDPJ1678C1ZA
D. Licence No. : JP14200001946,UP14210001943
FSSAI NO. 22717[illegible]

TAX INVOICE

Invoice No. :A017940 Date: 21-06-2025
Oprater ID : 004 Time : 12:43

SN.	PRODUCT NAME	PACK	RACK NO.	BATCH	EXP.	QTY	UNIT	MRP	AMT	DIS%	N.AMT.
1.	AKU BIOTIC – 4	10	B-6,K	AVH01ACA	12/26	1:0	TAB.	133.10	133.10	15.00%	113.13
2.	DOLOGESIC SP – 2	10		TMR-250033	12/26	1:0	TAB.	107.55	107.55	15.00%	91.42
3.	FAROSPECT ER 300 – 5	6	A-48	LPL24363	11/26	1:4	TAB.	750.00	1250.00	15.00%	1062.50
4.	RABESEC DSR – 3	10	GK-15 A	RDC241113	10/26	0:5	PCS	177.15	88.58	15.00%	75.29
5.	SILOTIME D 8 – (1)	15	L-TYPE-11	MLPIAE35	9/26	0:5	TAB.	786.50	262.17	15.00%	222.84

GST 1397.48*6+6%=83.85SGST+83.85CGST, [0

EAT HEALTHY LIVE HAPPY

Terms & Conditions
All disputes subject to GHAZIABAD Jurisdiction only.
Medicines are Not Sale/Taken Without Bill

For JAGGI MEDICOS

You Save Total Discount : 276.22

Authorised Signatory

Rs. One Thousand Five Hundred Sixty Five Only

GRAND TOTAL 1565.00

H·O·D
HOUSE of DIAGNOSTICS

Patel Nagar-1
Ghaziabad

Bill Of Supply

Patient Name:	**Vikas Gandhi**	**Visit No.:**	**GZB25062838**
Age / Sex:	52 Y/M	Invoice No.:	SI25-26UP0043229
Mobile:	9911198000	Invoice Date:	15-Jun-25 08:01
UHID:	UKKD.0000138009	Centre:	GHAZIABAD
Referred By:	Dr. ASHISH TYAGI		
Address:	D20, 1st Floor, DayaNand Nagar		HSN / SAC : 999316

Code	Description	MRP	Rate	Net
US0002	Ultrasound KUB	1798	899	INR 899.00
PL0006	Hb A1c	598	299	INR 299.00
CS0001	Urine C/S	998	499	INR 499.00
PL0007	Urine R/M	198	99	INR 99.00

Payment Details

Settled At	Receipt No	Txn Type	Payment Mode		Amount
15-Jun-25	RGZBR/25-26/00010171	Settlement	EDC	(0615H080123)	INR 1796.00
			Total		**INR 1796**

Total	**INR 1796.00**
Paid	INR 1796.00
Balance Due	**INR 0.00**

RS MAITRI HEALTH CARE
J-278, Patel Nagar, 1st Ghaziabad-201001, Ph.:0120-4130964
E-mail : rsmaitrihealthcare@gmail.com

OPD Bill

Name	: Mr. Vikas Gandhi	UHID	: 6033
Age/Sex	: 52 y 8 m 10 d/Male	OPD No	: OPD-2040
Mobile No	: 9911198000	Bill Date	: 19-06-2025 10:49
Consultant	: Dr. Vaibhav Saxena	Bill ID	: 2191
Department	: UROLOGY	Panel	: Cash
Referred By	: N/A	Token No	: N/A
Address	: N/A		

Services

S.No.	Date	Description	Code	Rate	Unit	Dis%	Dis	Amt(Rs)
		Laboratory						
		3934						
1	19-06-2025	Complete Blood Count (CBC)		300.00	1	0	0	300.00
2	19-06-2025	Kidney Function Test. (KFT)		700.00	1	0	0	700.00
3	19-06-2025	PROTHROMBIN TIME (PT/INR)		300.00	1	0	0	300.00
4	19-06-2025	Thyroid Stimulating Hormone (TSH)		250.00	1	0	0	250.00
5	19-06-2025	PSA		700.00	1	0	0	700.00
6	19-06-2025	HIV & HIV II Antibody (RAPID)		350.00	1	0	0	350.00
7	19-06-2025	HCV Rapid	122	500.00	1	0	0	500.00
8	19-06-2025	Hepatitis B Surface Antigen (HBsAg)		250.00	1	0	0	250.00
		Radiology						
9	19-06-2025	X-Ray Chest		400.00	1	0	0	400.00
		Additional Service						
10	19-06-2025	ECG		300.00	1	0	0	300.00

Gross Amount :	₹4,050.00
Discount :	₹0.00
Net Amount :	₹4,050.00

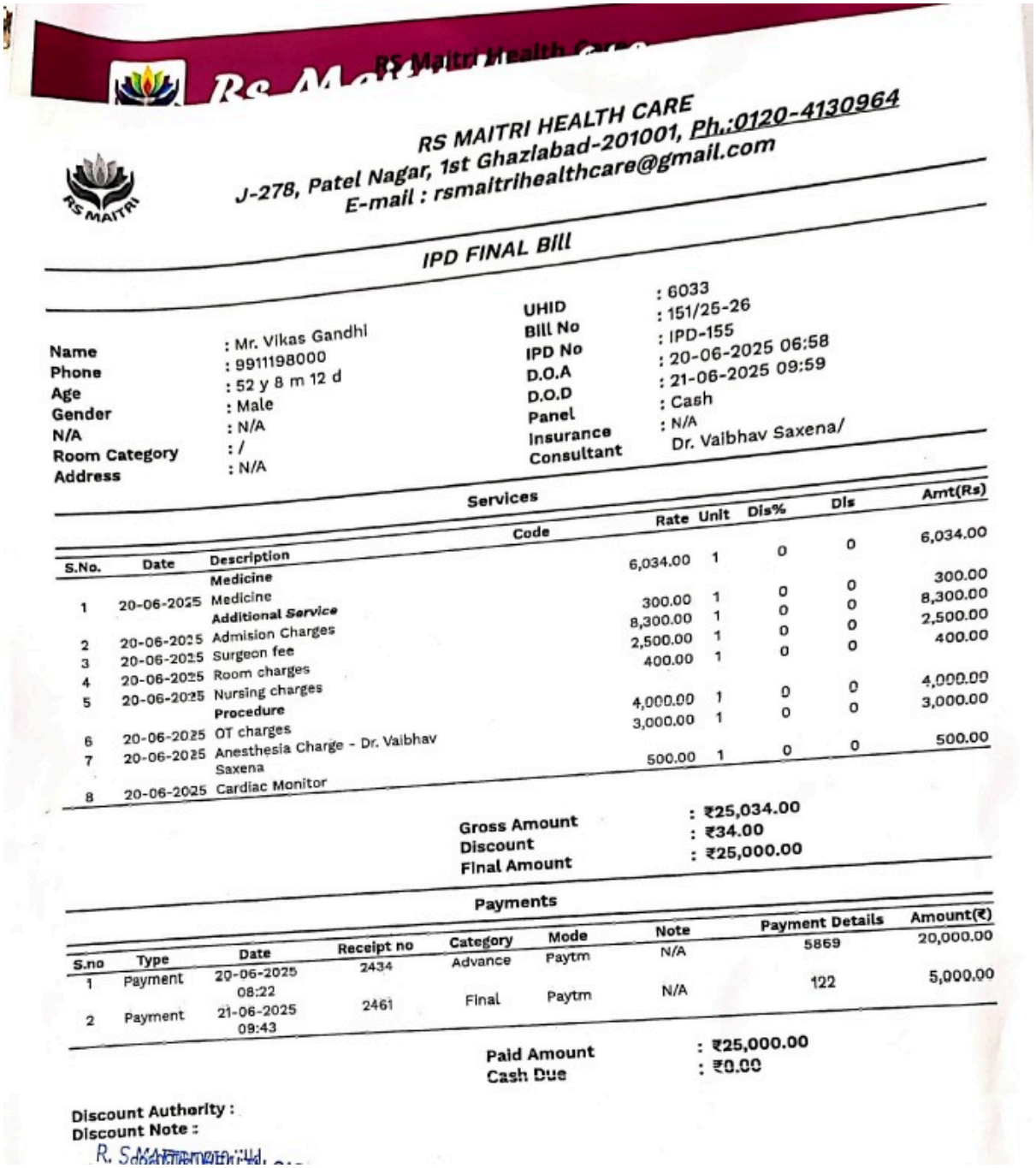
Payments

S.no	Type	Date	Receipt no	Category	Mode	Note	Payment Details	Amount(₹)
1	Payment	19-06-2025 10:49	2380	Paid	Paytm	N/A	122	4,050.00

Paid Amount :	₹4,050.00
Cash Due :	₹0.00

Printed By : Ms. Reshma

Prepared By : Ms. Reshma



RS MAITRI HEALTH CARE
J-278, Patel Nagar, 1st Ghaziabad-201001, Ph.:0120-4130964
E-mail : rsmaitrihealthcare@gmail.com

IPD FINAL Bill

Name	: Mr. Vikas Gandhi	UHID	: 6033
Phone	: 9911198000	Bill No	: 151/25-26
Age	: 52 y 8 m 12 d	IPD No	: IPD-155
Gender	: Male	D.O.A	: 20-06-2025 06:58
N/A	: N/A	D.O.D	: 21-06-2025 09:59
Room Category	: /	Panel	: Cash
Address	: N/A	Insurance	: N/A
		Consultant	Dr. Vaibhav Saxena/

Services

S.No.	Date	Description	Code	Rate	Unit	Dis%	Dis	Amt(Rs)
		Medicine						
1	20-06-2025	Medicine		6,034.00	1	0	0	6,034.00
		Additional Service						
2	20-06-2025	Admision Charges		300.00	1	0	0	300.00
3	20-06-2025	Surgeon fee		8,300.00	1	0	0	8,300.00
4	20-06-2025	Room charges		2,500.00	1	0	0	2,500.00
5	20-06-2025	Nursing charges		400.00	1	0	0	400.00
		Procedure						
6	20-06-2025	OT charges		4,000.00	1	0	0	4,000.00
7	20-06-2025	Anesthesia Charge - Dr. Vaibhav Saxena		3,000.00	1	0	0	3,000.00
8	20-06-2025	Cardiac Monitor		500.00	1	0	0	500.00

Gross Amount	: ₹25,034.00
Discount	: ₹34.00
Final Amount	: ₹25,000.00

Payments

S.no	Type	Date	Receipt no	Category	Mode	Note	Payment Details	Amount(₹)
1	Payment	20-06-2025 08:22	2434	Advance	Paytm	N/A	5869	20,000.00
2	Payment	21-06-2025 09:43	2461	Final	Paytm	N/A	122	5,000.00

Paid Amount	: ₹25,000.00
Cash Due	: ₹0.00

Discount Authority :
Discount Note :

R. S[illegible]

JAGGI MEDICOS

SHOP NO-196, PLOT NO-214,
GANDHI NAGAR,GHAZIABAD (U.P.)-201001
Phone : 0120-4157997 WhatsApp : 9319826400
E-Mail : jaggimedicosgzb@gmail.com

GSTIN : 09AKDPJ1678C1ZA
D. Licence No. : UP14200001946,UP14210001943
FSSAI NO. 22717696000031

Patient Name : CASH
Patient Address :
Dr Name :
Dr.Reg no. :
Card no. :

TAX INVOICE

Invoice No. :A016540 Date: 14-06-2025
Oprater ID : 001 Time : 22:39

SN.	PRODUCT NAME	PACK	RACK NO.	BATCH	EXP.	QTY	UNIT	MRP	AMT	DIS%	N.AMT.
1.	LOBATE GM CREAM	20G	TUB-43	NE092	3/26	1	TAB.	153.60	153.60	15.00%	130.56
2.	SILOHIGH 8	10	L-TYPE-1,K	P1AA04	2/26	1:0	TAB.	255.00	255.00	15.00%	216.75

GST 310.11*6+6%=18.6SGST+18.6CGST, ['000+

EAT HEALTHY LIVE HAPPY

Terms & Conditions
All disputes subject to GHAZIABAD Jurisdication only.
Medicines are Not Sale/Taken Without Bill

You Save Total Discount : 61.29

Rs. Three Hundred Forty Seven Only

For JAGGI MEDICOS

Authorised Signatory

GRAND TOTAL 347.00

RS MAITRI HEALTH CARE

J-278, Patel Nagar, 1st Ghaziabad-201001, Ph.:0120-4130964
E-mail : rsmaitrihealthcare@gmail.com

OPD Bill

Name	: Mr. Vikas	UHID	: 6033
Age/Sex	: 53 y /Male	OPD No	: OPD-2031
Mobile No	: 9911188000	Bill Date	: 19-06-2025 09:53
Consultant	: Dr. Vaibhav Saxena	Bill ID	: 2182
Department	: UROLOGY	Panel	: Cash
Referred By	: N/A	Token No	: 2
Address	: N/A		

Services

S.No.	Date	Description	Code	Rate	Unit	Dis%	Dis	Amt(Rs)
		Consultation						
1	19-06-2025	Consultation		1,000.00	1	0	0	1,000.00
					Gross Amount :		₹1,000.00	
					Discount :		₹0.00	
					Net Amount :		₹1,000.00	

Payments

S.no	Type	Date	Receipt no	Category	Mode	Note	Payment Details	Amount(₹)
1	Payment	19-06-2025 09:53	2371	Paid	Cash	N/A		1,000.00

Paid Amount : ₹1,000.00

Gmail

9AM DEAL <9amdeal@gmail.com>

Claim #25062100350-1 registered

1 message

Acko no-reply <no-reply@acko.com> Sat, Jun 21, 2025 at 12:12 PM
To: 9amdeal@gmail.com
Cc: 9amdeal@gmail.com

ACKO

Your reimbursement claim is registered successfully

Hey Vikas,

We have registered your reimbursement claim #25062100350. Rest assured, we are here for you and will close your request at the earliest.

Please keep your claim ID handy for easy and quick follow-ups.

Claim details

Claim number 25062100350-1	Claim amount ₹ 31846
Patient's name VIKAS GANDHI	Admission date 20-Jun-2025

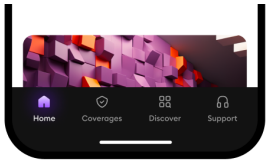
Expected completion by **26-Jun-2025**

Track claim

Log in to the ACKO app with your mobile number and receive real-time claim updates

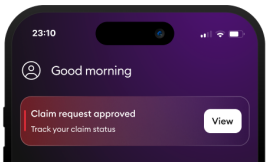
STEP 1

Open the
Acko App



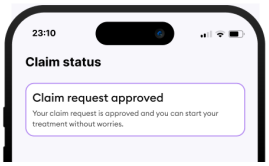
STEP 2

Check your
claim alerts



STEP 3

Track your
claim updates



Need help? We are always available at 1800 266 2256 and health@acko.com
Please mention your claim number for quick and easy resolutions.

Take care!

Download our app

GET IT ON Google Play AVAILABLE ON App Store

People love to talk about us

4.7 ★ Google Rating

4.9 ★ Facebook Rating

Acko General Insurance Ltd.
#36/5, Hustlehub One East, Somasandrapalya, 27th Main Rd, Sector 2, HSR Layout, Bengaluru, Karnataka 560102, Contact No. 1800 266 2256, health@acko.com
website: www.acko.com

IRDAI Regn No. : 157 | CIN : U66000KA2016PLC138288 | ACKO health insurance policy, Acko General Insurance | UIN : ACKHLIP20039V01202 ACKHLIA22210V012122 | For more details on the risk factors, terms and conditions please read the policy wordings available on www.acko.com c before concluding the sale.

Trade Logo displayed above belongs to Acko Technology & Services Pvt Ltd. and used by Acko General Insurance Ltd under license.

Gmail

9AM DEAL <9amdeal@gmail.com>

Grievance regarding unjust claim rejection – Policyholder: Vikas Gandhi

1 message

9AM DEAL <9amdeal@gmail.com> Tue, Jun 24, 2025 at 8:00 AM
To: "grievance@acko.com" <grievance@acko.com>

To,
Grievance Redressal Officer
ACKO General Insurance Ltd.

Subject: Grievance regarding unjust claim rejection – Policyholder: Vikas Gandhi

Respected Sir/Madam,

I, Vikas Gandhi, resident of D-20, Dayanand Nagar, Ghaziabad – 201001, am writing this letter to lodge a formal complaint regarding the denial of my health insurance claim.

I paid my insurance premium in good faith. Unfortunately, I developed a medical infection and was hospitalized for a day. During the hospitalization, I underwent surgery, received medication, and underwent various necessary medical tests. The total expenditure incurred was INR 34,661.

After submitting my claim along with all required documents, I was dismayed to receive a denial from your side. I find this decision unjust and disappointing. As a responsible policyholder, I fulfilled my duty by paying the premium. In return, I expected timely and fair support during my time of genuine medical need.

It appears that the process has been unnecessarily complicated, causing mental stress and financial burden. I request that you review my case once again and provide a justified explanation along with the rightful settlement.

Attached:
- Discharge Summary
- Claim documents
- ID proof
- Other relevant bills/reports

I hope for a prompt and fair resolution to my grievance.

Sincerely,
Vikas Gandhi
D-20, Dayanand Nagar, Ghaziabad – 201001
+91 99111 98000
9amdeal@gmail.com
Date: 24-06-2025

Policy Number: ASHP202400001607

Vikas Gandhi

Gandhi Sadan | D-20 | DayaNand Nagar | Ghaziabad 201 001 | India
Tel: 91 0120 4131079 | Tel-Fax: 91 0120 414 22 33 |
Mobile +91 99111 98000 | Email: vikas@gandhi.im

2 attachments

Vikas_UIDAI.jpg
460K

VikasGandhi_Acko.pdf
2762K

Gmail

9AM DEAL <9amdeal@gmail.com>

Urgent Request for Review of Claim Rejection – Violation of Natural Justice and Insurance Ethics Policy No.: ASHP202400001607

1 message

9AM DEAL <9amdeal@gmail.com> Tue, Jun 24, 2025 at 8:17 AM
To: "hello@acko.com" <hello@acko.com>

To,

Halo Acko

ACKO General Insurance Ltd.

Subject: Urgent Request for Review of Claim Rejection – Violation of Natural Justice and Insurance Ethics

Policy No.: ASHP202400001607

Respected Sir/Madam,

I, Vikas Gandhi, resident of D-20, Dayanand Nagar, Ghaziabad – 201001, am writing to express my deep concern and disappointment regarding the rejection of my claim under the above-mentioned policy.

When I purchased this health insurance policy and paid my premium in good faith, I entered into a contract based on trust — a promise that in the event of a genuine medical need, I would receive support. Unfortunately, that moment did arrive. I contracted a serious infection, was hospitalized for one full day, and underwent surgical treatment, medications, and necessary diagnostic tests. The total cost incurred was ₹34,661, all of which is well-documented and attached.

According to the basic principles of natural justice and fairness:

- I fulfilled my obligation by timely paying the premium.
- I did not misuse the policy or raise a fraudulent claim.
- My treatment was genuine, medically necessary, and confirmed by the discharge summary and hospital records.
- I had no control over the infection that led to hospitalization.

Then why was I denied the rightful reimbursement?

The infection was neither elective nor avoidable. The very purpose of health insurance is to help individuals cope with such sudden, unfortunate, and medically justified events.

Denying my claim after I acted in good faith undermines the essence of insurance itself. It sends a message that even genuine policyholders cannot rely on the promise they paid for.

I urge you to review this case thoroughly, not merely from a technical perspective, but from a humanitarian and ethical angle. I request that my claim of ₹34,661 be honored and reimbursed, in accordance with:
- The terms of the policy,
- The principles of natural justice, and
- The moral obligation of the insurer to protect its clients.

Attached:

- Discharge Summary
- Medical reports
- Hospital bills
- Claim application

I sincerely hope this matter will receive your urgent and fair consideration.

Sincerely,
Vikas Gandhi
Policy No: ASHP202400001607
Mobile: +91 99111 98000
Email: vikasgandhi@email.com
Date: 24-06-2025

Vikas Gandhi

Gandhi Sadan | D-20 | DayaNand Nagar | Ghaziabad 201 001 | India
Tel: 91 0120 4131079 | Tel-Fax: 91 0120 414 22 33 |
Mobile +91 99111 98000 | Email: vikas@gandhi.im

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2 attachments

Vikas_UIDAI.jpg
460K

VikasGandhi_Acko.pdf
2762K

C-06 Claim and Representation

92

Gmail

9AM DEAL <9amdeal@gmail.com>

Ticket Received - Urgent Request for Review of Claim Rejection – Violation of Natural Justice and Insurance Ethics Policy No.: ASHP202400001607 - 12568674

1 message

Team Acko <hello@acko.com> Tue, Jun 24, 2025 at 8:18 AM
Reply-To: Team Acko <hello@acko.com>
To: 9amdeal@gmail.com

Dear 9amdeal,

We have received your email and will get back to you within 24 hours.

ACKO empowers you to **Add/Edit** the following details and download your updated policy instantly in a few easy steps by visiting ACKO website/app.

Self-Serve Options Available

- Phone Number
- Engine Number
- Loan Provider
- IDV Value
- Purchase Year
- GSTIN
- Nominee Details
- Accessories Cover
- Previous Insurer
- Insured Name
- Chassis Number
- Variant/CC/Fuel
- NCB Value
- Passenger Cover
- Address
- Policy Start Date
- Email Id
- Personal Accident Cover
- Driver Car Cover

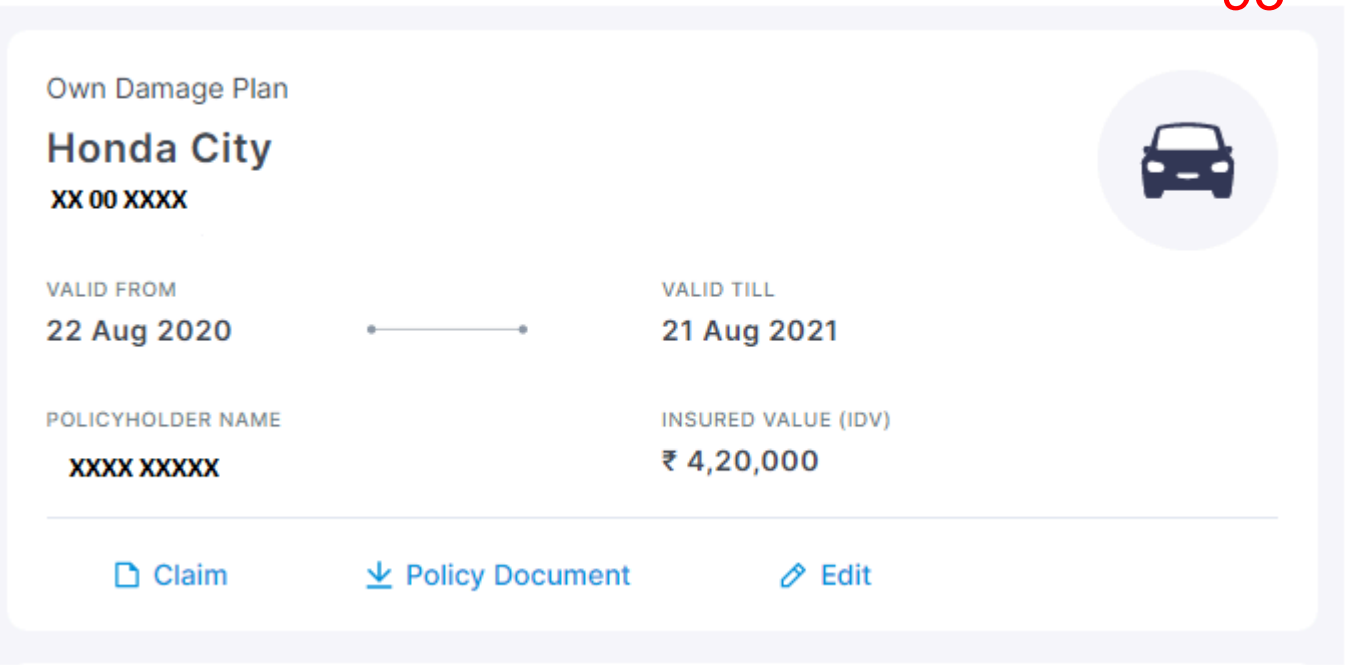
1. Log in to the ACKO app or by visiting www.acko.com using your registered mobile number.

2. Once you've logged in, you will be able to see the vehicle details. You can proceed by clicking on 'Edit'.

23/07/2026

7

C-06 Claim and Representation
93



3. Now you can initiate the change request.

Customer Relationship Manager
Team ACKO

23/07/2026
8

Gmail

9AM DEAL <9amdeal@gmail.com>

Update on Your Claim No. 25062100350

1 message

Team Acko <hello@acko.com> Wed, Jun 25, 2025 at 12:23 PM
Reply-To: Team Acko <hello@acko.com>
To: 9amdeal@gmail.com

Dear Mr. Gandhi,
We hope you are doing well.
This is regarding your recent claim (Claim No. **25062100350**) under Policy No. **ASHP202400001607**. After reviewing the documents submitted, we regret to inform you that the claim has not been approved.

Reason for Rejection:

The treatment undergone — **Phimosis with Meatoplasty under spinal anaesthesia** — falls under the **Specific Illness Waiting Period** clause of your policy. As per Clause **4.1.2**, *"Any surgery of the genitourinary system unless necessitated by malignancy"* is excluded from coverage during the applicable waiting period.
Since this procedure is not related to a malignancy and falls under this specific exclusion, we are unable to process the claim at this stage.

> **Please note:** This claim decision was made after discussion and review by our internal medical team, in accordance with your policy terms.

You can refer to the **'Additional Conditions'** section of your policy document, where the complete list of illnesses subject to specific waiting periods is mentioned for your reference.

Need Help or Want to Escalate?

If you would like to raise a concern or escalate this matter, you can follow ACKO's **grievance redressal matrix**:

1. **Level 1 – Email Support**
   Write to us at: **hello@acko.com**
2. **Level 2 – Escalation**
   If not resolved, escalate to: **grievance@acko.com**
3. **Level 3 – Insurance Ombudsman**
   If the issue remains unresolved beyond 30 days, you may approach the Insurance Ombudsman. Details are available in your policy booklet.

For immediate assistance, feel free to call our toll-free number: **1800 266 2256**.

Thank you for choosing ACKO. We're here to support you and help you better understand your policy coverage.
Warm regards,
Team ACKO

Gmail

9AM DEAL <9amdeal@gmail.com>

Important update on your ACKO health claim

1 message

health.claims@acko.com <health.claims@acko.com> Thu, Jun 26, 2025 at 5:45 PM
To: 9amdeal@gmail.com

Subject: Important update on your ACKO health claim: 25062100350/1
Date: 26/06/2025

Dear Vikas Gandhi

Thanks for choosing ACKO health. We have reviewed your recent claim for hospitalization cost reimbursement and would like to share our findings with you:

Claim No: 25062100350/1

UHID No:AGIL.ACK.ASHP202400001607.1

Policy holder Name: Vikas Gandhi

Claimant Name:Vikas Gandhi

Date of Admission:20-Jun-25

Date of Discharge:21-Jun-25

Diagnosis:TIGHT PHIMOSIS WITH BALANITIS WITH MEATAL STENOSIS

After carefully reviewing your claim , we noticed that you have filed a claim for the expenses incurred for treatment of a condition falling under waiting period. Unfortunately, these costs are not a part of your insurance coverage. As a result, we are unable to approve your claim.

Clarity on our claim decision

We understand this can be disheartening, as it may significantly impact your plans and financial situation. We also acknowledge that such situations might affect our reputation as a trustworthy insurance provider. Recognizing these difficulties, ACKO makes it a priority to address each claim with the highest level of seriousness and attention.

Our senior team and a special committee carefully reviewed your claim to ensure a fair and transparent outcome. Our guiding principle is to maintain consistent standards for all customers. Since insurance is meant to share risks among policyholders, we cannot allow one person's situation to negatively affect another's. This approach enables us to protect the interests of all our customers and remain legally compliant.

Please refer to the attached image for the relevant policy information, explaining the risks not covered at the time of purchase and how it affects our claim decision.

4.1.2 Specific Illnesses
Any medical expenses related to the following medical disease will not be covered for 2 years.

a. Expenses related to the treatment of the listed Conditions, surgeries/treatments shall be excluded until the expiry of number of months, as specified in the Schedule, of continuous coverage after the date of inception of the first policy with Us. This exclusion shall not be applicable for claims arising due to an accident.

b. In case of enhancement of sum insured the exclusion shall apply afresh to the extent of sum insured increase.

c. If any of the specified disease/procedure falls under the waiting period specified for pre-Existing diseases, then the longer of the two waiting periods shall apply.

d. The waiting period for listed conditions shall apply even if contracted after the policy or declared and accepted without a specific exclusion.

e. If the Insured Person is continuously covered without any break as defined under the applicable norms on portability stipulated by IRDAI, then waiting period for the same would be reduced to the extent of prior coverage.

Your options:

If you feel there has been an oversight or misinterpretation, please contact our Claims Internal Grievance Department within 30 days from the date of this email. We are more than willing to review any additional information that may influence our decision.

Contact Information:

Grievance Redressal Officer

ACKO GENERAL INSURANCE LIMITED.,

2nd Floor, Acko General Insurance

36/5 Hustle hub One East, Somasandrapalya,

27th Main Road, Sector 2, HSR Layout,

Bangalore, Karnataka – 560102

Email: gro@acko.com

Phone: 918062370005 / 918037680069

If you're not satisfied with our decision, you can seek assistance from the Insurance Ombudsman, an external grievance redressal platform. To learn more about the process and their offices click here.

Please don't hesitate to reach out to us in case you have any questions or concerns.

Thank you,

Health Claim – ACKO

The information in this mail and any attachment(s) to this message is/are confidential and intended solely for the addressee or organisation to whom it is addressed. If you have erroneously received this message, please notify mailadmin@acko.com immediately and destroy the message and attachment(s). If you are not the intended recipient, any copying, forwarding, altering or disclosing the contents of this email message may be unlawful. The information, attachment(s) or the opinions expressed in this mail are those of the individual sender and not necessarily those of ACKO. ACKO accepts no responsibility for any loss or damage arising from the use of this email message or its attachment(s).

MAX Lab

Laboratory Investigation Report

Patient Name	: Mr.Vikas Gandhi	Centre	: 6755 - RS MAITRI HEALTH CARE LLP
Age/Gender	: 52 Y 0 M 0 D /M	OP/IP No/UHID	: ///
MaxID/Lab ID	: ML05864405/6235062500013	Collection Date/Time	: 20/Jun/2025 11:14AM
Ref Doctor	: Dr.VAIBHAV SAXENA MS M.Ch	Reporting Date/Time	: 30/Jun/2025 11:54AM

Histopathology

SIN No:B2B7349921

Histo Biopsy Medium Specimens

Histopathology Number:
V– 6630/25

Specimen Type:
Prepuce of glans

Clinical Data:
Prepuce of glans.

Gross Description:
Received two skin covered unoriented soft tissue pieces altogether measuring 6 x 3 x 1.6 cm.
Externally shows cauliflower like lesion measures 3.5 x 3 x 0.8 cm. (A to F; 6 blocks - A/E).

Microscopic Examination:
Sections examined from preputial skin shows dense lymphocytic infiltrate just at and below the squamous epithelium intermixed with hyalinised lamina propria below the basement membrane and in the underlying deep connective tissue. Hyper and parakeratosis is seen at the epithelial surface with epidermal hyperplasia. No evidence of granuloma/ dysplasia/ malignancy seen.

Impression:
Prepuce of glans:

- **Consistent with Lichen sclerosus (balanitis xerotica obliterans).**

Note:
1. The gross specimens will be retained for a period of 30 days post report sign-out.
2. Slides & blocks for second opinion will be issued (slides are chargeable) to the patient / attendant after 48 hours of submitting a formal request. (Time between 9 AM to 5 PM).
3. Cold ischemic time – 30-35 minutes; Fixation time - 6-72 hrs.

*** End Of Report ***

Dr. Asif Baliyan
Consultant - Histopathology

Test Performed at :794 - Max Hospital - Vaishali, W-3, Sector-1, Vaishali, Ghaziabad-201012, U.P
Booking Centre :6755 - RS MAITRI HEALTH CARE LLP, J -278, Patel Nagar 1st, Near Old Bus Stand
The authenticity of the report can be verified by scanning the Q R Code on top of the page

MC-2004

Max Lab Limited (A Wholly Owned Subsidiary of Max Healthcare Institute Ltd.)
Max Lab, Max Super Speciality Hospital, Vaishali: W-3, Sector-1, Vaishali, Ghaziabad-201012, (U.P.).
Phone: +91-0120-4173 000, 4188 000 | (CIN No.: U85100DL2021PLC381826)
Helpline No. 7982 100 200 www.maxlab.co.in feedback@maxlab.co.in

Conditions of Reporting: 1. The tests are carried out in the lab with the presumption that the specimen belongs to the patient name as identified in the bill/test request form. 2. The test results relate specifically to the sample received in the lab and are presumed to have been generated and transported per specific instructions given by the physicians/laboratory. 3. The reported results are for the information and interpretation by the referring doctor only. 4. Some tests are referred to other laboratories to provide a wider test menu to the customer. 5. Max Healthcare shall in no event be liable for accidental damages loss, or destruction of specimen which is not attributable to any direct and mala fide act or omission of Max Healthcare or its employees. Liability of Max Healthcare for deficiency of services, or other errors and omissions shall be limited to fee paid by the patient for the relevant laboratory services.

C-10_Subsequent_Appeals_Correspondence
99

Gmail

9AM DEAL <9amdeal@gmail.com>

Appeal Against Rejection of Claim No. 25062100350 – Request for Reconsideration

1 message

9AM DEAL <9amdeal@gmail.com> Wed, Jun 25, 2025 at 12:58 PM
To: "grievance@acko.com" <grievance@acko.com>
Cc: hello@acko.com

Appeal Letter – Claim Reconsideration

Date: 25-06-2025

To
The Grievance Officer
ACKO General Insurance Ltd.
36/5, Somasandra Palya,
HSR Layout, Sector 2,
Bengaluru, Karnataka – 560102
Email: grievance@acko.com
CC: hello@acko.com

Subject: Appeal Against Rejection of Claim No. 25062100350 – Request for Reconsideration

Policy Number: ASHP202400001607
Insured Name: Mr. Vikas Gandhi
UHID: ACK.ASHP202400001607.1
Claim Amount: ₹34,661
Treatment: Phimosis with Meatoplasty (under Spinal Anaesthesia)
Date of Admission: [Insert date]

Dear Sir/Madam,

I am writing to formally appeal the rejection of my claim referenced above, which was denied on the grounds that the treatment falls under the “Specific Illness Waiting Period” clause (Clause 4.1.2) of my policy.
While I appreciate the detailed response from your claims department, I respectfully submit the following points for your review and reconsideration:

- 1. Nature and Urgency of the Treatment

The procedure (Phimosis with Meatoplasty) was performed due to a medically advised necessity after consistent urological issues, not as an elective or cosmetic surgery.

- 2. Good Faith & Transparent Disclosures

I have been fully transparent during the proposal and onboarding process. The only disclosed condition was obesity, which is unrelated to this claim.

23/07/2026
1

C-10_Subsequent_Appeals_Correspondence
100

- 3. Policy Inception & Contribution

I have paid all monthly installments till June 2025 and have fulfilled my payment obligations in good faith. My intent has always been to maintain active coverage and use the policy responsibly.

- 4. Request for Reasonable Consideration

I urge ACKO to consider that this treatment was unforeseen, medically necessary, not pre-existing, and not related to malignancy.

- 5. Financial Impact

The hospitalization cost me ₹34,661, which was a significant and sudden expense. I request reimbursement based on medical merit.

Additional Justification:

This appeal is further backed by the following valid grounds:

- The treatment was medically necessary and not elective or cosmetic in nature.
- No pre-existing condition was present or declared related to this ailment.
- All premium installments have been paid up to June 2025, fulfilling the financial commitment in good faith.
- The denial appears to be based on technicalities, not on the core purpose of health insurance which is to help in genuine medical emergencies.
- The treatment was advised by a certified specialist and conducted at a registered hospital.

As per the spirit of IRDAI regulations and principles of natural justice, this claim deserves reconsideration and approval.

Conclusion & Request
I now request you to:
- Reopen Claim No. 25062100350 for re-evaluation.
- Initiate reimbursement of eligible expenses.
- Provide a written response within 15 working days as per IRDAI guidelines.

If this matter remains unresolved, I reserve the right to escalate the issue to the Insurance Ombudsman.

Sincerely,
Vikas Gandhi
Phone: +91-9911198000
Email: 9amdeal@gmail.com
Policy No: ASHP202400001607

Vikas Gandhi

Gandhi Sadan | D-20 | DayaNand Nagar | Ghaziabad 201 001 | India
Tel: 91 0120 4131079 | Tel-Fax: 91 0120 414 22 33 |

23/07/2026
2

Mobile +91 99111 98000 | Email: vikas@gandhi.im

C-10 Subsequent Appeals Correspondence
102

Gmail

9AM DEAL <9amdeal@gmail.com>

Ticket Received - Appeal Against Rejection of Claim No. 25062100350 – Request for Reconsideration - 12582411

1 message

Team Acko <hello@acko.com> Wed, Jun 25, 2025 at 12:59 PM
Reply-To: Team Acko <hello@acko.com>
To: 9amdeal@gmail.com

Dear 9amdeal,

We have received your email and will get back to you within 24 hours.

ACKO empowers you to **Add/Edit** the following details and download your updated policy instantly in a few easy steps by visiting ACKO website/app.

Self-Serve Options Available

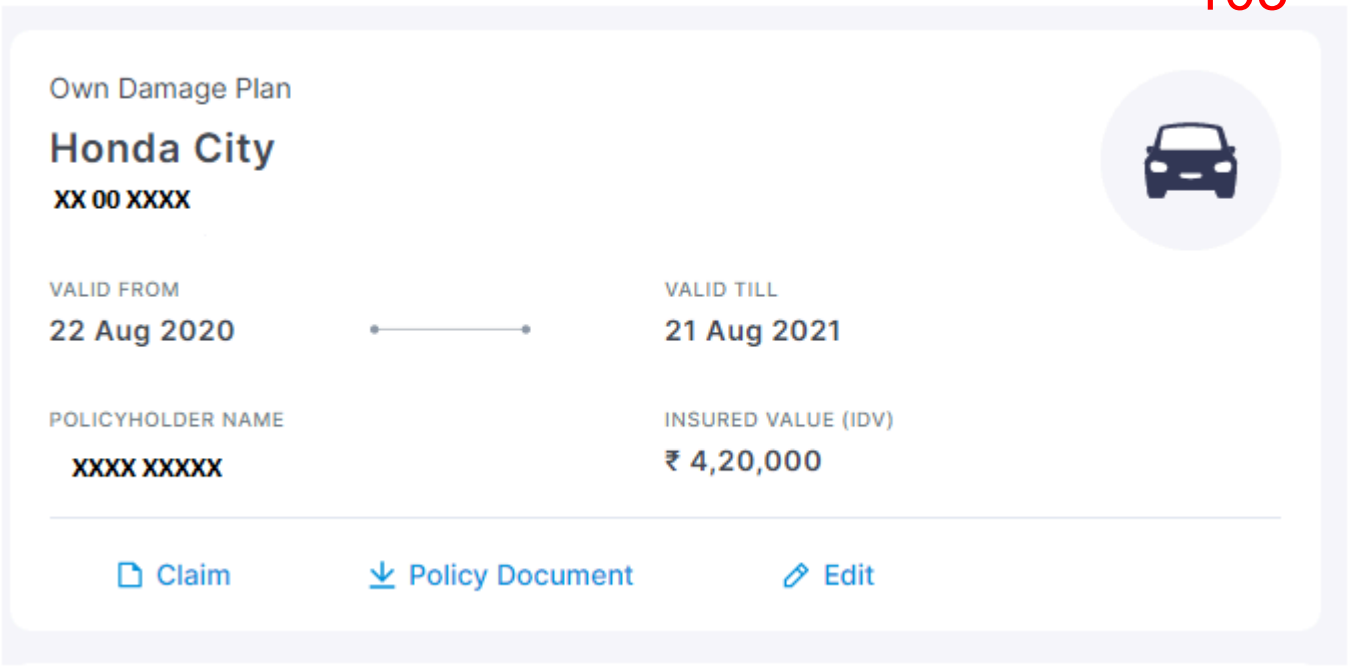
- Phone Number
- Engine Number
- Loan Provider
- IDV Value
- Purchase Year
- GSTIN
- Nominee Details
- Accessories Cover
- Previous Insurer
- Insured Name
- Chassis Number
- Variant/CC/Fuel
- NCB Value
- Passenger Cover
- Address
- Policy Start Date
- Email Id
- Personal Accident Cover
- Driver Car Cover

1. Log in to the ACKO app or by visiting www.acko.com using your registered mobile number.

2. Once you've logged in, you will be able to see the vehicle details. You can proceed by clicking on 'Edit'.

23/07/2026
4

C-10 Subsequent Appeals Correspondence
103



3. Now you can initiate the change request.

Customer Relationship Manager
Team ACKO

C-10 Subsequent Appeals Correspondence
104

Gmail

9AM DEAL <9amdeal@gmail.com>

Ticket Received - Appeal Against Rejection of Claim No. 25062100350 – Request for Reconsideration - 12582412

1 message

Grievance Officer <grievance@acko.com> Wed, Jun 25, 2025 at 12:59 PM
Reply-To: Grievance Officer <grievance@acko.com>
To: 9amdeal@gmail.com

Dear 9amdeal,

We have received your email and will get back to you within 24 hours.

ACKO empowers you to **Add/Edit** the following details and download your updated policy instantly in a few easy steps by visiting ACKO website/app.

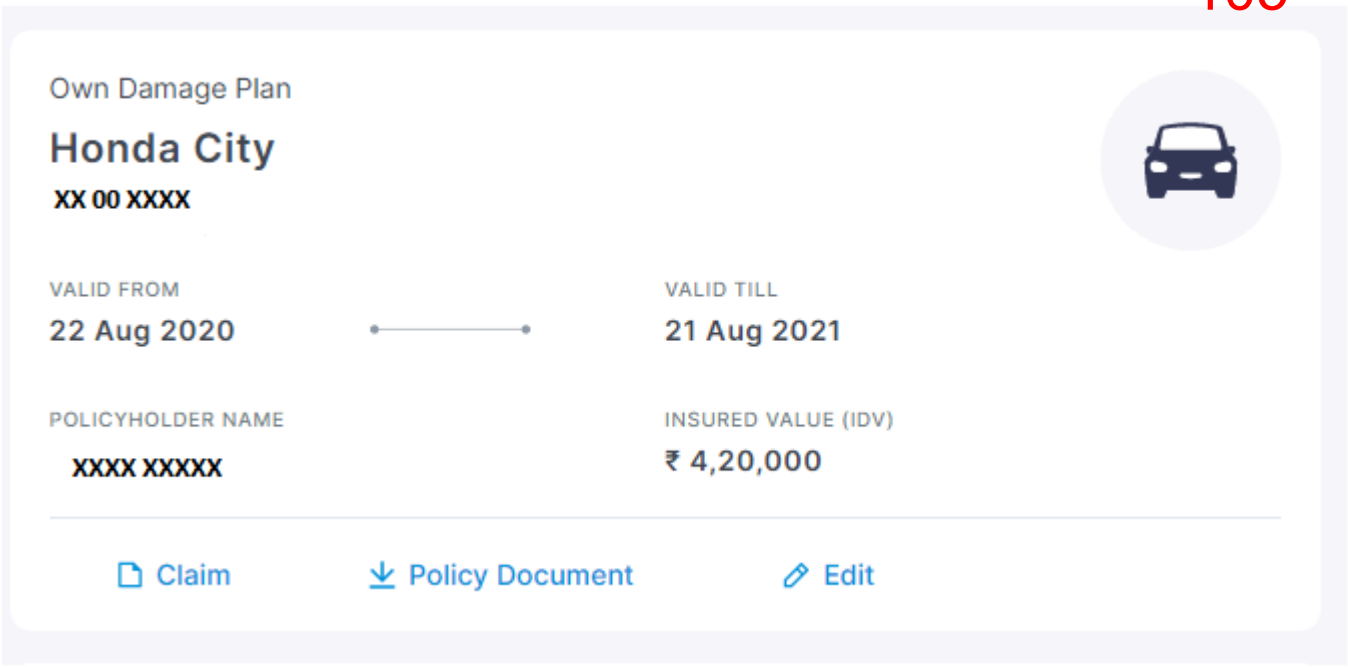
Self-Serve Options Available

- Phone Number
- Engine Number
- Loan Provider
- IDV Value
- Purchase Year
- GSTIN
- Nominee Details
- Accessories Cover
- Previous Insurer
- Insured Name
- Chassis Number
- Variant/CC/Fuel
- NCB Value
- Passenger Cover
- Address
- Policy Start Date
- Email Id
- Personal Accident Cover
- Driver Car Cover

1. Log in to the ACKO app or by visiting www.acko.com using your registered mobile number.

2. Once you've logged in, you will be able to see the vehicle details. You can proceed by clicking on 'Edit'.

23/07/2026
6



3. Now you can initiate the change request.

Customer Relationship Manager
Team ACKO

Gmail

9AM DEAL <9amdeal@gmail.com>

Reconsideration Request – Claim No. 25062100350/1 (Policy No. ASHP202400001607)

1 message

9AM DEAL <9amdeal@gmail.com> Thu, Jun 26, 2025 at 6:37 PM
To: gro@acko.com

Subject: Reconsideration Request – Claim No. 25062100350/1 (Policy No. ASHP202400001607)

Dear Grievance Redressal Officer,
I am writing to submit my detailed rebuttal against the rejection of my health insurance claim (Claim No. 25062100350/1). Kindly find the attached response letter with all relevant supporting documents.

I humbly request you to re-evaluate my case in light of the evidence and clarifications provided.

Attachments:

1. Counter Letter in English and Hindi (Detailed Response)
2. ACKO Policy Document (Full Terms & Annexures)
3. Proposal Form (as submitted at policy inception)
4. ACKO's Claim Rejection Letter (Dated 26 June 2025)
5. Hospital Discharge Summary
6. Doctor's Prescription / Advice Note

I have already paid all monthly premiums up to June 2025, and the policy was active at the time of hospitalization.

Kindly confirm receipt of this email and let me know if anything further is required from my side.

Sincerely,
Vikas Gandhi
Policy No.: ASHP202400001607
Phone: 9911198000
Email: 9amdeal@gmail.com

प्रिय ग्रेवियंस रिड्रेसल अधिकारी,
मैं अपनी स्वास्थ्य बीमा क्लेम संख्या 25062100350/1 के अस्वीकार किए जाने के विरोध में अपना विस्तृत प्रत्युत्तर एवं सभी आवश्यक दस्तावेज़ संलग्न करके प्रेषित कर रहा हूँ।

आपसे विनम्र निवेदन है कि कृपया मेरे दावे की समीक्षा पुनः करें, जिससे मुझे मेरा विधिसम्मत लाभ प्राप्त हो सके।

संलग्नक (Attachments):

1. विस्तृत प्रत्युत्तर पत्र (अंग्रेज़ी व हिंदी दोनों में)
2. ACKO की पॉलिसी डॉक्युमेंट (पूर्ण शर्तें एवं अनुबंध)

3. प्रस्ताव पत्र (Proposal Form)
4. ACKO द्वारा भेजा गया क्लेम अस्वीकृति पत्र (दिनांक 26 जून 2025)
5. अस्पताल का डिस्चार्ज समरी
6. डॉक्टर की सलाह / पर्चा

मैं जून 2025 तक की सभी किश्तें समय से चुका चुका हूँ और क्लेम के समय मेरी पॉलिसी पूर्णतः सक्रिय थी।

कृपया इस मेल की प्राप्ति की पुष्टि करें और यदि कोई और जानकारी चाहिए तो अवश्य बताएं।

सादर,
विकास गांधी
पॉलिसी संख्या: ASHP202400001607
मो.: 9911198000
ईमेल: 9amdeal@gmail.com

To:

Grievance Redressal Officer
ACKO GENERAL INSURANCE LIMITED
2nd Floor, Hustle Hub One East, Somasandrapalya,
27th Main Road, Sector 2, HSR Layout,
Bangalore, Karnataka – 560102
Email: gro@acko.com

Date: 26 June 2025

Subject: Detailed Rebuttal against Rejection of Claim No. 25062100350/1 – Hospitalization for Urological Emergency

---

Dear Sir/Madam,

I am writing to formally appeal against your decision dated 26/06/2025 rejecting my claim number 25062100350/1. Kindly consider the following point-wise rebuttal against the reasons provided in your rejection letter.

✅ ACKO's Reason: "Condition falls under waiting period (Specific Illness 4.1.2)"

My Counterpoint: 1. The diagnosis is "Tight Phimosis with Balanitis with Meatal Stenosis." 2. Nowhere in your Annexure or Clause 4.1.2 is "Phimosis", "Balanitis", or "Meatal Stenosis" explicitly listed. 3. The clause applies to clearly listed ailments only. Applying it to an unlisted, acute condition is an overreach.

✅ ACKO's Reason: "Clause 4.1.2 excludes surgeries related to Specific Illnesses for 2 years"

My Counterpoint: 1. The surgery conducted was a **medical necessity** to relieve pain and restore proper urination, not a cosmetic procedure. 2. The condition is **not pre-existing**, nor was it declared or known before policy inception. 3. Your own policy terms exclude these waiting periods **only if the condition is listed**, which this is not.

✅ ACKO's Statement: "We apply consistent standards to all customers"

My Counterpoint: 1. Applying exclusion to conditions not listed violates natural justice and IRDAI norms of fair claims assessment. 2. Your statement should be backed by logic, not blanket denial. A claim for a clearly required emergency procedure cannot be denied without valid listing.

✅ ACKO's Statement: "Diagnosis is subject to 2-year waiting even if contracted after inception"

My Counterpoint: 1. Clause 4.1.2(d) only applies to conditions mentioned in the list. The diagnosis in my case is **not on that list**. 2. Hence, generalizing this clause for an unrelated urological condition is legally and ethically unjust.

✅ ACKO's Statement: "Policy must be continuously covered"

My Counterpoint: 1. I have **paid all monthly premiums timely till June 2025**, which means the policy was fully active at the time of hospitalization. 2. Clause regarding continuity is irrelevant here as **this was the first year of coverage and all dues were cleared**.

📄 Additional Grounds for Reconsideration:

1. As per your policy, **60 days pre and 120 days post hospitalization expenses** are covered. This shows acknowledgment of both planned and sudden treatments.
2. Your policy (page 3) says "*No permanent disease exclusions*." This strengthens my entitlement.
3. The treating doctor recommended immediate surgery. I have submitted all supporting clinical documents.

🚫 Prayer:

Based on the above clarifications and fair interpretation of the terms of your policy, I request you to: - Reassess the rejection of my claim, - Approve it on the basis of valid documentation, and - Avoid unnecessary delay as it impacts genuine policyholders' trust in ACKO.

If needed, I am willing to submit further medical evidence or written clarification from the hospital.

C-10_Subsequent_Appeals_Correspondence
109

Failing redressal within the 30-day timeline, I will be constrained to escalate this matter to the **Insurance Ombudsman** as per IRDAI guidelines.

Sincerely,
Vikas Gandhi
Policy No.: ASHP202400001607
UHID: ACK.ASHP202400001607.1
Phone: 9911198000
Email: 9amdeal@gmail.com

Vikas Gandhi

Gandhi Sadan | D-20 | DayaNand Nagar | Ghaziabad 201 001 | India
Tel: 91 0120 4131079 | Tel-Fax: 91 0120 414 22 33 |
Mobile +91 99111 98000 | Email: vikas@gandhi.im

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10 attachments

Doctor's Prescription Advice Note.jpg
184K

Hospital Discharge Summary (2).jpg
140K

Hospital Discharge Summary.jpg
117K

23/07/2026
11

C-10_Subsequent_Appeals_Correspondence
110

ACKO's Claim Rejection Letter (Dated 26 June 2025).txt
5K

acko_claim_counter.md
7K

Acko Claim Counter(1).docx
13K

Acko Claim Counter.docx
13K

Acko Claim Counter.pdf
41K

VIKAS GANDHI policy document.pdf
275K

VIKAS GANDHI proposal form.pdf
73K

23/07/2026
12

C-10 Subsequent Appeals Correspondence
111

Gmail

9AM DEAL <9amdeal@gmail.com>

Ticket Received - Reconsideration Request – Claim No. 25062100350/1 (Policy No. ASHP202400001607)

1 message

Acko Grievance Officer <gro@acko.com> Thu, Jun 26, 2025 at 6:39 PM
Reply-To: Acko Grievance Officer <gro@acko.com>
To: 9amdeal@gmail.com

Dear 9amdeal,

Thank you for writing to us, we have noted your query/request. Our Customer Relationship Manager will reach out to you shortly for a speedy resolution.

Ticket No. 12598436

Regards
Team Acko

23/07/2026
13

Re: Vikas Gandhi/25062100350

1 message

Grievance Officer <grievance@acko.com> Wed, 15 Jul, 2026 at 10:19
Reply to: Grievance Officer <grievance@acko.com>
To: 9amdeal@gmail.com

Dear Vikas,

As discussed, we would like to inform you that after carefully reviewing your claim , we noticed that you have filed a claim for the expenses incurred for treatment of a condition falling under waiting period. Unfortunately, these costs are not a part of your insurance coverage. As a result, we are unable to approve your claim.

Even after following Escalation Level 1 to 3 as stated in the below mentioned link, your issue remains unresolved within 30 days of lodging a complaint with us and you wish to pursue other avenues for redressal of grievances, you may approach the Insurance Regulatory & Development Authority of India or the Insurance Ombudsman, whose details can be found on the link below :
https://www.acko.com/gi/customer-service/grievance-redressal/

You can also lodge online complaint through the website of Council for Insurance Ombudsmen (CIO)
www.cioins.co.in

Thanks,
Abdul Nayeem
Grievance Redressal Team

ACKO

BEWARE OF FRAUDULENT CALLS AND OFFERS. IRDAI and its officials do not get involved in selling insurance, announcing bonuses, or investing premiums. Please report such calls to the police.

PROCEEDINGS BEFORE THE INSURANCE OMBUDSMAN,Noida
(State of Uttarakhand & State of Uttar Pradesh (Districts of western part))
(Under Rule No.16/17 of Insurance Ombudsman Rules 2017)
Ombudsman Name : RAJIV TALWAR
CASE OF COMPLAINANT - VIKAS GANDHI
VS
RESPONDENT: ACKO GENERAL INSURANCE CO. LTD
COMPLAINT REF: NO: NOI-H-056-2526-0961
AWARD NO:IO/NOI/A/HI/0061/2026-2027

<table>
<tr><td>1.</td><td>Name & Address Of The Complainant</td><td>VIKAS GANDHI
D-20, 1st FLOOR, DAYANAND NAGAR</td></tr>
<tr><td>2.</td><td colspan="2">Type Of Policy: Health
Policy Details:
<table>
<tr><th>Policy Number</th><th>Sum Assured/Insured</th><th>From Date</th><th>To Date</th><th>DOC</th><th>Premium</th><th>Policy Term/Mode</th><th>Paying Term</th></tr>
<tr><td>ASHP202400001607</td><td>1000000</td><td>04-Nov-2024</td><td>03-Nov-2025</td><td>04-Nov-2024</td><td>31065</td><td>1 YEAR/ANNUAL</td><td>SINGLE</td></tr>
</table></td></tr>
<tr><td>3.</td><td>Name of Insured
Name of the Policy Holder</td><td>VIKAS GANDHI
VIKAS GANDHI</td></tr>
<tr><td>4.</td><td>Name of insurer /Broker</td><td>ACKO GENERAL INSURANCE CO. LTD</td></tr>
<tr><td>5.</td><td>Date of Repudiation</td><td></td></tr>
<tr><td>6.</td><td>Reason For Repudiation</td><td>Filed a claim for the expenses incurred for treatment of a condition falling under waiting period. These costs are not a part of your insurance coverage.</td></tr>
<tr><td>7.</td><td>Date of Receipt of the complaint</td><td>07-Aug-2025</td></tr>
<tr><td>8.</td><td>Nature of Complaint</td><td>REPUDIATION OF HEALTH CLAIM</td></tr>
<tr><td>9.</td><td>Amount of Claim</td><td>37358.00</td></tr>
<tr><td>10.</td><td>Date of Partial Settlement</td><td></td></tr>
<tr><td>11.</td><td>Amount of relief sought</td><td>37358</td></tr>
<tr><td>12.</td><td>Complaint registered under Insurance Ombudsman Rules 2017</td><td>Rule 13(1)(b) — any partial or total repudiation of claims by an insurer</td></tr>
<tr><td>13.</td><td>Date of Hearing</td><td>24-Jun-2026</td></tr>
<tr><td>14.</td><td>Representation at the hearing</td><td></td></tr>
<tr><td></td><td>a)For the Complainant</td><td>VIKAS GANDHI</td></tr>
<tr><td></td><td>b)For the Insurer</td><td>RAVI INKOLLU (DGM LITIGATION)</td></tr>
<tr><td>15.</td><td>Complaint how disposed</td><td>AWARD</td></tr>
</table>

COMPLAINT REF: NO: NOI-H-056-2526-0961

Brief Facts of the Case:

1.Complaint NOI-H-056-2526-0961 (claim no. 25062100350-1 for Rs. 37,358) is filed by Mr. Vikas Gandhi (Complainant) against the rejection of his hospitalization claim by Acko General Insurance Limited (Insurer).

2.The policy no. is ASHP202400001607 for the period of 04.11.2024 to 03.11.2025 for the Sum Insured of Rs 10,00,000/-. The first policy date is 04.11.2024.

3.He was admitted in RS Maitri Health Care, Ghaziabad from 20.06.2025 to 21.06.2025. The discharge summary reveals the diagnosis as "Tight phimosis with Balanitis with meatal stenosis". The history of the illness is: Poor flow of urine: The procedure at the Hospital: Circumcision + Meatoplasty.

4.Reimbursement claim for Rs 37,358/- (Insurer recorded claimed amount as Rs 31,846/-) was filed for admission 20.06.2025 to 21.06.2025.

5.Insurer rejected this claim under email dated 26.06.2025 - After carefully reviewing your claim, we noticed that you have filed a claim for the expenses incurred for treatment of a condition falling under waiting period. Unfortunately, these costs are not a part of your insurance coverage. As a result, we are unable to approve your claim.

4.1.2 Specific Illnesses
Any medical expenses related to the following medical disease will not be covered for 2 years.

a. Expenses related to the treatment of the listed Conditions, surgeries/treatments shall be excluded until the expiry of number of months, as specified in the Schedule, of continuous coverage after the date of inception of the first policy with Us. This exclusion shall not be applicable for claims arising due to an accident.

b. The waiting period for listed conditions shall apply even if contracted after the policy or declared and accepted without a specific exclusion.

6.Complainant represented vide email dated 24.06.2025 -

(a) I contracted a serious infection, was hospitalized for one full day, and underwent surgical treatment, medications, and necessary diagnostic tests.

(b) The total cost incurred was ?34,661, all of which is well-documented.

(c) My treatment was genuine, medically necessary, and confirmed by the discharge summary and hospital records.

(d) I had no control over the infection that led to hospitalization. The infection was neither elective nor avoidable.

(e) Denying my claim after I acted in good faith undermines the essence of insurance itself. It sends a message that even genuine policyholders cannot rely on the promise they paid for.

(f)The claim be reimbursed in accordance with the terms of the policy, the principles of natural justice, and the moral obligation of the insurer to protect its clients.

7.The insurance Grievance Redressal Officer replied vide e-mail dated 25.6.25-

As discussed over the call, we regret to inform you that it cannot be honored. After carefully reviewing your claim, we noticed that you have filed a claim for the expenses incurred for the treatment of a condition falling under the waiting period. Unfortunately, these costs are not a part of your insurance coverage. As a result, we are unable to approve your claim.

Contention of the complainant:
8. Complainant reiterated aspects on lines mentioned in brief facts above.

Contention of the Respondent:
9. The insurer's Self-contained Notes dated 8.10.25 reiterated the contentions as mentioned in brief facts above.

10.Insurer also informed the claim is declined in accordance with the terms and conditions of the policy. The clause reads as under: "4.1.2 Specific Illnesses. Any medical expenses related to the following medical disease will not be covered for 2 years. a. Expenses related to the treatment of the listed Conditions, surgeries/treatments shall be excluded until the expiry of number of months, as specified in the Schedule, of continuous coverage after the date of inception of the first policy with Us.

The insured had undergone surgery of the genitourinary system which is a specific procedure listed under section 4.1.2.f (3) on page no.29 of the policy wordings.

Observation and conclusions:
Hearing

11.The complainant Mr Vikas Gandhi, and insurer represented by Mr. Ravi Inkollu, DGM Litigation were present online for personal hearing on 24/06/2026.

During the hearing, briefly put -

(a)Complainant drew attention to the biopsy report dated 30/6/2025 to say that Impression of "BXO" mentioned therein is a pre-malignant condition and 6 % chances of malignancy are there, so the claim is payable.

(b)Complainant was asked if there was any post operative medical paper to this effect, to which there was no clear reply.

(c)Complainant submitted that the policy wording" Any Surgery of the genitourinary system unless necessitated by malignancy" would cover something that has chances of being malignant.

(d)Insurer submitted that in the light of the above words in the policy, the words cannot be reconstructed to include cases of the type when there is no evidence of malignancy.

Further discussion and findings

12. Since the matter is contested, the documents on records and submission have been examined for passing an award.

13. It is noted that it is not disputed that the procedure was medically necessary and it was undertaken to address the complaint /problem of poor flow of urine, and it has also been conveyed by complainant that he had serious infection.

14. In this sense, the procedure involving circumcision + meatoplasty would not be covered under the Permanent Exclusion clause 4.2.3 of the policy as it has been necessitated by illness and forming part of treatment.

15. However, since it is not arising from an "accident", it is needed to decide if the procedure, definitely being Surgeries of the genitourinary System, is hit by waiting period of 2years clause 4.1.2 of policy.

16. To be not hit, the procedure should have been necessitated by malignancy. The Histopathology report dated 30.06.2025 states - "No evidence of granuloma/dysplasia/Malignancy seen".

Thus, it is concluded that the Impression of Prepuce of glans recorded in the report as "consistent with lichen sclerosis (Balanitis Xerotica Obliterans)" relates to the other aspects recorded in the microscopic examinations, in relation to which the "no evidence of granuloma/dysplasia/Malignancy seen" has been recorded in the report.

17. It is found that the words of the Histopathology Report rule out malignancy . It is viewed that a hypothetical projection of likelihood of malignancy devoid of medical papers (in the case) to this effect, would not be an objective methodology to assess the terms of the policy wording.

18. Accordingly, read holistically and harmoniously, no reason is found to interfere with the decision of the Insurer.

AWARD

COMPLAINT REF: NO: NOI-H-056-2526-0961

Based on the pleadings and evidence brought on record in this case, and the discussion and findings above, and further keeping in mind provisions of the Insurance Ombudsman Rules 2017, no grounds are found to support the complaint and no reason is found to interfere with the decision of the Insurer.

Accordingly, the complaint is dismissed.

AWARD NO:IO/NOI/A/HI/0061/2026-2027
Date:02/Jul/2026

INSURANCE OMBUDSMAN
Noida

ANNEX-XIII-A

4th Floor, Bhagwan Sahai Palace,
Main Road, Naya Bans,
Gautam Buddh Nagar Sector 15,
Noida 201301
oio.noida@cioins.co.in
Date : 02-JUL-2026

Ref. No. : NOI-H-056-2526-0961
Mr. VIKAS GANDHI
D-20, 1st FLOOR, DAYANAND NAGAR,
Ghaziabad, Uttar Pradesh-201001
Mobile No. : 9911198000
Dear Sir/Madam,

Re: Award No. : IO/NOI/A/HI/0061/2026-2027
on complaint with the Insurance Ombudsman,
under Policy No. : ASHP202400001607

We enclose herewith a copy of the Award No. IO/NOI/A/HI/0061/2026-2027 pronounced by the Insurance Ombudsman, Noida, for your information.

In the eventuality of your disagreeing with the enclosed award, you may, move a fresh application at any other forum/court that may be considered by you as appropriate.

Thus, the complaint is closed.

Kindly acknowledge.

Yours faithfully,

Asstt./Dy./Secretary
Encl. Award.

Hearing Scheduled for your Complaint number NOI-H-056-2526-0961

1 message

<no-reply@cioins.co.in> Tue, 9 Jun, 2026 at 00:07
Reply to: no-reply@cioins.co.in
To: 9amdeal@gmail.com, gro@acko.com

प्रिय महोदय/महोदया,

आपको सूचित किया जाता है कि शिकायत संख्या NOI-H-056-2526-0961 की सुनवाई 24/06/2026 को 11:00 बजे **WEBEX ऐप के माध्यम से निर्धारित की गई है।**
लिंक नीचे दिया गया है:

https://cioins.webex.com/cioins/j.php?MTID=m54f96b47b3ab7d7b807d86a618b47320

शिकायतकर्ता से अनुरोध है कि वे फोटो पहचान पत्र की एक प्रति जमा करें और साथ ही अपना वैकल्पिक मोबाइल नंबर भी बताएं।
प्रतिवादी/बीमाकर्ता से अनुरोध है कि वे सुनवाई में उपस्थित होने वाले अधिकारी का नाम और उनका वैकल्पिक मोबाइल नंबर भी सूचित करें।
कृपया इस ईमेल की प्राप्ति की पुष्टि करें।

सहायक सचिव/उप सचिव
चौथी मंजिल, भगवान सहाय पैलेस, मेन रोड, नया बांस, गौतम बौद्ध नगर सेक्टर 15, नोएडा 201301
ईमेल: oio.noida@cioins.co.in
फ़ोन: 0120-4027589

यह सिस्टम द्वारा जनरेट किया गया ईमेल है। कृपया इस ईमेल का जवाब न दें।

C-11_Insurance_Ombudsman
120

Gmail

9AM DEAL <9amdeal@gmail.com>

Annexure VI and VI A with reference to your Complaint number NOI-H-056-2526-0961

1 message

no-reply@cioins.co.in <no-reply@cioins.co.in> Sat, Sep 27, 2025 at 10:06 PM
Reply-To: no-reply@cioins.co.in
To: 9amdeal@gmail.com

Dear Sir/Madam,

Ref: Complaint No. NOI-H-056-2526-0961

Your above complaint has been registered with us.
Kindly download the Annexure VI & VI-A and submit Annexure VI-A duly completed in all respects.
If you are submitting the documents by email, please send it on oio.noida@cioins.co.in only.

Asst./Dy./Secretary
4th Floor, Bhagwan Sahai Palace, Main Road, Naya Bans, Gautam Buddh Nagar Sector 15, Noida 201301
Email:oio.noida@cioins.co.in
Phone:0120-2514251

This is system generated mail. Please do not reply to this mail.

2 attachments

NOI-H-056-2526-0961_Ann_VI_27Sep2025132426.pdf
29K

NOI-H-056-2526-0961_Ann_VI_A_27Sep2025132427.pdf
38K

23/07/2026
8

To,

The Insurance Ombudsman,

4th Floor, Bhagwan Sahai Palace,

Main Road, Naya Bans,

Gautam Buddh Nagar Sector 15,

Noida – 201301

Date: 28 September 2025

Subject: Submission of Annexure VI-A – Complaint No. NOI-H-056-2526-0961

Dear Sir/Madam,

With reference to your notice dated 27-09-2025 regarding my complaint No. NOI-H-056-2526-0961 against ACKO General Insurance Co. Ltd., I am hereby submitting:

1. Annexure VI-A, duly completed and signed.

2. My unconditional and irrevocable consent in terms of Rule 13(2) of the Insurance Ombudsman Rules, 2017, for the Ombudsman to act as a mediator between myself and the Insurance Company.

3. Copies of supporting documents including the policy, complaint letter, company’s response (if any), and reminders already submitted.

Kindly acknowledge receipt and take the complaint forward for consideration.

Thank you for your assistance.

Yours faithfully,

Vikas Gandhi

D-20, 1st Floor, Dayanand Nagar,

Ghaziabad, Uttar Pradesh – 201001

Mobile: 9911198000

Email: 9amdeal@gmail.com

RS MAITRI HEALTH CARE

DR. VAIBHAV SAXENA
M.S. M.Ch. (UROLOGY)
(AIIMS - New Delhi)

Head Urology, Uro-oncology (Cancer) & Renal Transplant
Visiting Consultant-Yashoda Hospital, Kaushamb
Manipal Hospital
Metro Hospital, Noida
Up Medical Council No. 47425
DMC No. 53574
E-mail : rsmaitrihealthcare@gmail.com

Timings : Morning 9:30 am to 11:00 am
Evening 6:00 pm to 8:00 pm
Wed. & Sun. Evening Close

Facilities Available

Stone Surgery
- Kidney - PCNL, RIRS (Laser)
- Ureter - URS
- Urinary Bladdar - CLT

Prostate Surgery
- TURP
- TURIS
- HOLEP

Reconsructive Surgery
- Urethroplasty for Strictures

Female Urology
- Vesicovaginal fistula Repair
- Ureterovaginal fistula Repair

Pediatric Urology
- Hypospadias
- PUV
- PUJ Obstrution
- Undescended testies

Laser Surgery By Holmium Laser
- For Prostate
- For Stones
- Stricture Urethra

Laparoscopic Surgery
- For Stones
- PUJ Obstruction

Cancer Surgery
- Adrenal
- Kidney
- Ureter
- Urinary Bladder
- Penis
- Prostate
- Testis

Renal Transplant
AV Fistula for hemodialysis
Urinary Infections
Male Infertility
Uroflowmetry

Ref No.. :

Date..... 19/6/2026

5341M

Patient Name : Vilas Kaundhi

UHID No.

History Uro - Pan flow

Investigation :

CBC KFT PT-INR
TSH Typle H CXR ECG
SPIRA

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Treatment Advised :

Atm
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7:00 am

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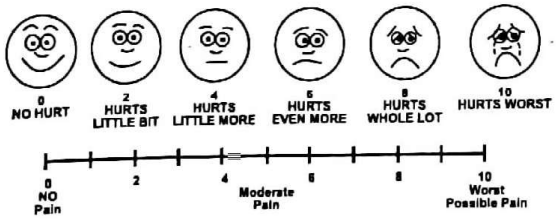
Pain Rating Scale**



Next Follow up Visit:

Consultant Sign.

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RS MAITRI HEALTH CARE

DR. VAIBHAV SAXENA
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- Undescended testies

Laser Surgery By Holmium Laser
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- For Stones
- Stricture Urethra

Laparoscopic Surgery
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- Urinary Bladder
- Penis
- Prostate
- Testis

Renal Transplant
AV Fistula for hemodialysis
Urinary Infections
Male Infertility
Uroflowmetry

Ref No.. :

Date.....31.7.25.....

Patient Name : Mr. Vikas Chaudhri 52y/M

UHID No.

History : Post circumcision 29/6/25

Investigation : Regular Bx - BXO

Treatment Advised : Tab Clav Z Kamdays
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USG KUB PVR
Kidney
Ureter
Bladder
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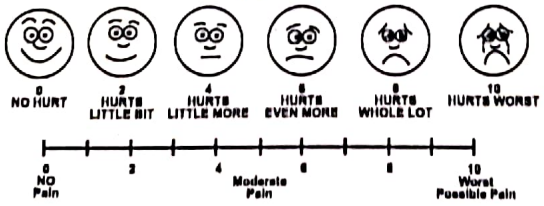
Pain Rating Scale**



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PLEASE CONTACT:- 0120-4130964
8527066995

Follow-up Appointment
TO REVIEW IN OPD ON AFTER 3 DAYS DRESSING . REMOVAL OF FOLEYS CATHETER AFTER 3 DAYS , NORMAL DIET, PLENTY OF WATER AVOID STRENUOUS ACTIVITIES.

Dr. VAIBHAV SAXENA
M.B.B.S., M.S.
Director, Urology, Uro-
R.S Maitri Health Care
Reg. No.-
J-278, Patel Nagar 1st Ghaziabad U.P.

Dr. Vaibhav Saxena
UROLOGY

RMO Signature

Note :
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- Ureterovaginal fistula Repair

Pediatric Urology
- Hypospadias
- PUV
- PUJ Obstrution
- Undescended testies

Laser Surgery By Holmium Laser
- For Prostate
- For Stones
- Stricture Urethra

Laparoscopic Surgery
- For Stones
- PUJ Obstruction

Cancer Surgery
- Adrenal
- Kidney
- Ureter
- Urinary Bladder
- Penis
- Prostate
- Testis

Renal Transplant
AV Fistula for hemodialysis
Urinary Infections
Male Infertility
Uroflowmetry

Ref No.. :

Date 12/7/26

Patient Name : MR. VIKAS GANDHI 54y/Male

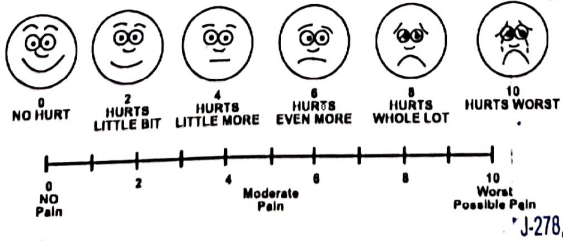
UHID No.

History : TO WHOM IT MAY CONCERN

Investigation :

Treatment Advised : Mr. VIKAS GANDHI, 54yrs Male,
R/A- D-20, DAYANAND NAGAR GHAZIABAD
was suffering from Phimosis, with Balanitis with meatal stenosis & UTI. His Suyeny was must in view of unable to pass urine & UTI. His Preputial Biopsy carry out to be (BXO) which is a premalygnt condition & not any elective or cosmetic suyeny. There is life thretery infection risk & risk of penile cancer

Pain Rating Scale**



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3.3 Risk factors and prevention

Several risk factors for penile cancer have been identified (Table 2) [21] (LE: 2a).

Table 1: Recognised aetiological and epidemiological risk factors for penile cancer

Risk factors	Relevance	Ref
Phimosis	Odds ratio 11-16 vs. no phimosis	[22-24]
Chronic penile inflammation (balanoposthitis related to phimosis), lichen sclerosus	Risk	[25]
Sporalene and ultraviolet A phototherapy for various dermatological conditions such as psoriasis	Incidence rate ratio 9.51 with > 250 treatments	[26]
Smoking	Five-fold increased risk (95% Confidence interval (CI): 2.0-10.1) vs. non-smokers	[22, 23, 27]
HPV infection, condylomata acuminate	22.4% in verrucous squamous cell carcinoma 36-66.3% in basaloid-warty	[13, 28]
Rural areas, low socio-economic status, unmarried		[29-32]
Multiple sexual partners, early age of first intercourse	Three to five-fold increased risk of penile cancer	[21, 23, 33]

Human papilloma virus infection is a risk factor for penile cancer [34]. Human papilloma virus DNA has been identified in 70-100% of intra-epithelial neoplasia and in 30-40% of invasive penile cancer tissue samples (LE: 2a). The HPV virus interacts with oncogenes and tumour suppressor genes (*p16, P53, Rb* genes) [28, 35]. The rate of HPV-positivity differs between different histological subtypes of penile SCC. Human papilloma virus is a cofactor in the carcinogenesis of some variants of penile SCC, while others are not related to HPV. The commonest HPV subtypes in penile cancer are types 16 and 18 [36]. The risk of penile cancer is increased in patients with condyloma acuminata [37] (LE: 2b).

A significantly better five-year disease-specific survival has been reported for HPV-positive vs. HPV-negative cases (93% vs. 78%) in one study [38], while no difference in lymph node metastases and ten-year survival was reported in another study [39] (Table 3). There is no association between the incidence of penile and cervical cancer, although both are linked to HPV [40, 41]. Female sexual partners of patients with penile cancer do not have an increased incidence of cervical cancer [42].

Table 3: Outcomes for HPV and non-HPV penile carcinomas

Non HPV related	Prognosis	HPV related	Prognosis
SCC usual type/NOS	30% DOD	Basaloid SCC	> 50% DOD
Pseudohyperplastic carcinoma	0%	Papillary basaloid carcinoma	
Pseudoglandular carcinoma	> 50%	Warty carcinoma	Mortality low
Verrucous carcinoma	Good	Warty-basaloid carcinoma	30% DOD
Carcinoma cuniculatum	Good	Clear-cell carcinoma	20%
Papillary carcinoma NOS	Good	Lymphoepithelioma-like carcinoma	Not known
Adenosquamous carcinoma	Good		
Sarcomatoid carcinoma	75% DOD		

DOD = died of disease; HPV = human papillomavirus; SCC = squamous cell carcinoma.

At present, except for a few countries, there is no general recommendation for HPV vaccination in males because of the different HPV-associated risk patterns in penile- and cervical cancer. Furthermore, the epidemiological effects of HPV vaccination in girls still have to be assessed [43, 44].

Phimosis is strongly associated with invasive penile cancer [23, 29, 45, 46], due to associated chronic infection. However, smegma is not a carcinogen [45]. The incidence of lichen sclerosus is relatively high in penile cancer but is not associated with adverse histopathological features, including penile intraepithelial neoplasia (PeIN). Other epidemiological risk factors are cigarette smoking, low socioeconomic status and a low level of education [29, 46].

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Neonatal circumcision reduces the incidence of penile cancer; however, it does not seem to reduce the risk of PeIN [23]. The lowest incidence of penile cancer is reported for Israeli Jews (0.3/100,000/year). One matched-pair, case-control study reported that the protective effect of neonatal circumcision against invasive penile cancer (OR 0.41) was much weaker when the analysis was restricted to men without a history of phimosis (OR 0.79, 95% CI: 0.29-2) [23]. Circumcision in adult life does not have any protective effect.

The controversial discussion about neonatal circumcision should take into account that circumcision removes approximately half the tissue that can develop into penile cancer.

3.4 Pathology

Squamous cell carcinoma accounts for over 95% of penile malignancies (see Table 1). It is not known how often SCC is preceded by premalignant lesions (see Table 4) [47-50].

Different histological types of penile SCC with different growth patterns, clinical aggressiveness and HPV associations have been identified (see Table 5). Numerous mixed forms exist such as the warty-basaloid form, with 50-60% the most common mixed form, the usual-verrucous (hybrid), usual-warty, usual-basaloid and the usual-papillary, as well as other rarer combinations.

Other malignant lesions of the penis, all much less common than penile SCC, are melanocytic lesions, mesenchymal tumours, lymphomas and metastases. Penile metastases are frequently of prostatic or colorectal origin. Different types of penile sarcoma have been reported.

Table 4: Premalignant penile lesions (precursor lesions)

Lesions sporadically associated with squamous cell carcinoma (SCC) of the penis: • Bowenoid papulosis of the penis (HPV related) • Lichen sclerosis
Premalignant lesions (up to one-third transform to invasive SCC): • Penile intraepithelial lesions • Giant condylomata (Buschke-Löwenstein) • Bowen's disease • Paget's disease (intradermal ADK)

Table 5: Classification of intra-epithelial neoplasia (PeIN)

• **Non-HPV-related PeIN**
• o Differentiated PeIN
• **HPV-related PeIN**
• o Basaloid PeIN
• **Warty PeIN**
• **Warty-basaloid PeIN**
• **Other rare patterns of PeIN** (pleomorphic, spindle, clear cell, pagetoid)

3.4.1 Gross handling of pathology specimens

Tissue sections determine the accuracy of histological diagnosis. Small lesions should be fully included, bigger lesions should have at least 3-4 blocks. Lymph nodes must be included in their entirety after having been inked, in order to detect metastases. After having been inked, surgical margins have to be completely included [51]. Second-opinion pathology review is highly desirable for this rare tumour entity [52].

3.4.2 Pathology report

The pathology report must include the anatomical site of the primary tumour, the histological type of SCC, grade, perineural invasion, depth of invasion, vascular invasion (venous/lymphatic), irregular growth and front of invasion, urethral invasion, invasion of corpus spongiosum/cavernosum, surgical margins and the *p16*/HPV status (Table 6) [53-56].

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REVIEW ARTICLE

Male Genital Lichen Sclerosus

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Abstract

Male genital lichen sclerosus (MGLSc) is a chronic inflammatory skin disease responsible for male sexual dyspareunia and urological morbidity. An afeared complication is squamous cell carcinoma (SCC) of the penis. The precise etiopathogenesis of MGLSc remains controversial although genetic, autoimmune and infective (such as human papillomavirus (HPV) hepatitis C (HCV), Epstein-Barr virus (EBV) and Borrelia) factors have been implicated: Consideration of all the evidence suggests that chronic exposure of susceptible epithelium to urinary occlusion by the foreskin seems the most likely pathomechanism. The mainstay of treatment is topical ultrapotent corticosteroid therapy. Surgery is indicated for cases unresponsive to topical corticosteroid therapy, phimosis, meatal stenosis, urethral stricture, carcinoma *in situ* (CIS) and squamous cell carcinoma.

Key Words: *Male genital lichen sclerosus, etiology, pathogenesis, management*

What was known?
MGLSc is a cause of male sexual dysfunction and urological morbidity and associated with squamous carcinoma of the penis. It has been held to be an autoimmune disease.

Introduction

MGLSc is an acquired, chronic inflammatory cutaneous disease of disputed etiology. It causes acute and chronic balanoposthitis and scarring that can result in significant sexual and urinary dysfunction as well as conferring a risk of squamous carcinoma of the penis.[1,2]

The exact incidence and prevalence are difficult to ascertain. This could be because it is managed by many specialties (dermatology, genito-urinary medicine, urology and pediatrics), as well as significant under-recognition and hence under-reporting by patients and physicians. The estimated incidence of LSc ranges from 1/300 (0.3%) to 1/1000 (0.1%) in both sexes.[3] A more recent study reported the incidence of MGLSc of 1.4 patients per 100, 000 visit.[4] The epidemiology may vary between countries and racial groups because MGLSc is principally a disease of the uncircumcised although it can persist or recur after circumcision. There is a bimodal age incidence with peaks in young boys and in adult men, late in the fourth decade.[4-8]

MGLSc can be the cause of significant morbidity. The symptomatology is an expression of preputial and urethral dysfunction very often summarized as male dyspareunia.[2,6,9] The spectrum of presentation is wide. It may be asymptomatic. Some patients describe spontaneous itching, burning, bleeding, tearing, splitting (of the prepuce and frenulum), or hemorrhagic blisters or urinary problems such as dysuria and narrowing of the urinary stream, or anatomical changes of the genitalia. Most men (55%) have symptoms associated with sexual activity but dyspareunia may be even more common than this statistic due to failure or difficulty in eliciting the symptomatology.[6] Other presentations are non-retractile foreskin (phimosis), foreskin fixed in retraction (paraphimosis), urethritis, urethral stricture, urinary retention and renal failure. Some cases are diagnosed at the presentation of CIS or frank SCC. The disease and its complications have a significant impact on male sexual health.[6,10]

Signs may be subtle or florid. The organ should be examined systematically and sedulously, including with a hand lens. The classical genital morphologic manifestations of MGLSc are similar to those features of the disease encountered at extragenital sites, and include atrophic ivory-white patches (leukoderma) or plaques, hypertrophic, slightly scaly patches or plaques with telangiectasia and sparse purpura. A constrictive lichenoid posthitis is commonly seen associated with a fibrotic preputial band causing "hourglass" "waisting" of the penile shaft. Variable asymmetric, sometimes florid, Zoonoid inflammation may be present and distract the clinician from the correct diagnosis. The glans may be etiolated, affected by sclerosis especially parameatally and show purpura, ecchymosis or angiokeratomas. Meatal involvement may be subtle with encroachment

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disease might be challenging to diagnose clinically against a background of LSc. A retrospective review by Barbagli *et al.* of 130 patients with MGLSc, revealed 11 men (8.4%) with premalignant or malignant features (7 cases with SCC, 2 cases with verrucous carcinoma, 1 case of SCC associated with verrucous carcinoma and 1 case of erythroplasia of Queyrat).[73] The time interval between diagnosis of LSc and the development of SCC was 14-30 years. Two forms of penile intraepithelial neoplasia (PeIN) have been described (analogy with the vulvar intraepithelial neoplasia classification): Differentiated PeIN which is often associated with MGLSc and undifferentiated or Bowenoid PeIN which is associated with HPV infection.[74]

The association of GLSc and SCC is widely recognized and the risk has been estimated at between 2% and 12.5%.[13,73,75-77] MGLSc is also considered one of the two pathways for the development of CIS and penile cancer, the other being via HPV infection.[2,74] However, in a large series of patients from our clinics not one patient developed SCC, suggesting that accurate diagnosis and definitive curative management are central to arresting carcinogenic drive.

MGLSc and melanocytic lesions

Atypical genital melanocytic naevus has been reported concomitantly in patients with FG- and MGLSc.[78-80] Cases of melanoma complicating FGLSc have appeared in the literature.[81-85] We have seen three in MGLSc.[86] It has been postulated that LSc might be an immune response possibly triggered by the melanocytic lesion, alternatively that the inflammatory environment of LSc provides a microenvironmental niche promoting melanocytic neoplasia.

Diagnosis and Treatment

Most cases of MGLSc can be diagnosed clinically. Under some circumstances a biopsy can be performed. A biopsy is useful to exclude neoplastic transformation and differentiate LSc from erosive lichen planus, scarring cicatricial pemphigoid and lichen simplex.[1,2] The histology of LSc is characterized by flattening of the epidermis with variable hyperkeratosis, hydropic degeneration of basal cells and an inflammatory cell dermal infiltrate. However, biopsy histology of MGLSc may be non-specific, depending on timing, site and severity.[87] A biopsy of the most clinically abnormal area, for example, of inflammation or scarring, may not yield a histological diagnosis. There may be nonspecific inflammatory changes (e.g. zoonoid) leading to an erroneous diagnosis of Zoon's balanitis. In many men the preputial or glanular changes may be secondary and reactive to the balanopreputial dysfunction created by the principal proximate or adjacent or apposed site of the LSc. A negative biopsy does not exclude LSc. A negative biopsy does not exclude neoplasia.[2,9,66,88]

The treatment of MGLSc aims to achieve optimal symptomatic relief, prevention of further deterioration of anatomical architecture changes and a theoretical mitigation of malignant transformation. Our understanding of the role of occlusional contact with urine in the pathogenesis is fundamental to the effective management and the prognosis of MGLSc.[65,89]

Treatment protocols in GLSc are well-established (level evidence III).[2,6,66,90] Avoidance of contact with soap and urine are essential. Regular use of skin barrier emollient should be instituted. Short, trimmed pubic hair is recommended to avoid hair trapping between the foreskin and the glans penis and hence minimize irritation, abrasion and consequent inflammation.[89] Early aggressive treatment may prevent disease progression.[91] An ultrapotent topical corticosteroid (usually clobetasol propionate (level evidence I)) used under supervision for a finite course is effective[2,92-94] and usually problem free, but herpes simplex and wart reactivation do occur.[90,95] prophylactic aciclovir should be prescribed for patients with a history of genital HSV. Most men achieved remission by ultrapotent topical corticosteroid (50-60%).[6] Topical calcineurin inhibitors should not be used in MGLSc because of a theoretical synergistic risk of SCC.[6,61,62,69] If maximal conventional medical treatment is not possible or fails then circumcision is recommended. Removal of the foreskin alters the local environment of glans penis: It removes the occlusive and koebnerizing effects of the foreskin and hence mitigates the consequences of post-micturition microincontinence. Circumcision is curatively effective in the majority (at least > 75% but precise long term follow up data are lacking).[6] Some men might require advanced urethromeatal surgery. Our results show that accurate diagnosis and rational management restores sexual function and mitigates the risk of urinary dysfunction and can conserve the foreskin in about 50%. Furthermore, not one patient of the many hundreds of patients seen in the clinic over 20 years has yet developed penis cancer.[6]

Conclusion

MGLSc is an acquired, chronic, inflammatory and fibrosing cutaneous disease responsible for significant sexual and urological dysfunction as well as creating a risk of squamous carcinoma of the penis. The etiology of MGLSc is now much better understood. It is not primarily an autoimmune condition. It is likely that an interaction between the irritant effects of urine and other pathogenic factors, such as chronicity, occlusion and as yet undetermined differential epithelial susceptibility, or reaction, to injury, are necessary for the development of MGLSc. Better understanding the pathogenesis of MGLSc is important to minimize sexual morbidity, urological dysfunction, mitigate the cancer risk it generates and preserve the foreskin when possible.

23/07/2026

4

EAU Guidelines on
Penile Cancer

O.W. Hakenberg (Chair), E. Compérat, S. Minhas,
A. Necchi, C. Protzel, N. Watkin (Vice-chair)
Guidelines Associate: R. Robinson

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1. INTRODUCTION

1.1 Aim and objectives

The European Association of Urology (EAU) Guidelines on Penile Cancer provides up-to-date information on the diagnosis and management of penile squamous cell carcinoma (SCC).

It must be emphasised that clinical guidelines present the best evidence available to the experts but following guideline recommendations will not necessarily result in the best outcome. Guidelines can never replace clinical expertise when making treatment decisions for individual patients, but rather help to focus decisions - also taking personal values and preferences/individual circumstances of patients into account. Guidelines are not mandates and do not purport to be a legal standard of care.

1.2 Panel composition

The EAU Penile Cancer Guidelines Panel consists of an international multi-disciplinary group of clinicians, including a pathologist and an oncologist. Members of this panel have been selected based on their expertise and to represent the professionals treating patients suspected of having penile cancer. All experts involved in the production of this document have submitted potential conflict of interest statements, which can be viewed on the EAU website Uroweb: http://uroweb.org/guideline/penile-cancer/.

1.3 Available publications

A quick reference document (Pocket guidelines) is available, both in print and as an app for iOS and Android devices. These are abridged versions which may require consultation together with the full text version. Several scientific publications are available, the most recent dating back to 2014 [1], as are a number of translations of all versions of the Penile Cancer Guidelines. All documents are available through the EAU website Uroweb: http://uroweb.org/guideline/penile-cancer/.

1.4 Publication history

The EAU Penile Cancer Guidelines were first published in 2000; the current publication presents a limited update of the 2017 print.

1.5 Summary of changes

Key changes for the 2018 print:

Chapter 3 - Epidemiology, aetiology and pathology. New information has been added on the various histological subtypes of penile carcinomas, risk factors and human papilloma virus (HPV) association.

New and changed recommendations can be found in sections:

3.4.8 Guidelines for the pathological assessment of tumour specimens

Recommendations	Strength rating
The pathological evaluation of penile carcinoma specimens must include an assessment of the human papilloma virus status.	Strong
The pathological evaluation of penile carcinoma specimens must include a diagnosis of the squamous cell carcinoma subtype.	Strong
The pathological evaluation of penile carcinoma surgical specimens must include an assessment of surgical margins including the width of the surgical margin.	Strong

4.2 Guidelines on staging and classification

Recommendation	Strength rating
The pathological evaluation of penile carcinoma specimens must include the pTNM stage and an assessment of tumour grade.	Strong

5.4 Guidelines for the diagnosis and staging of penile cancer

Recommendations	Strength rating
Primary tumour	
Perform a physical examination, record morphology, extent and invasion of penile structures.	Strong

Obtain a penile Doppler ultrasound or MRI with artificial erection in cases with intended organ-sparing surgery.	Weak
Inguinal lymph nodes	
Perform a physical examination of both groins, record the number, laterality and characteristics of inguinal nodes and: • If nodes are not palpable, offer invasive lymph node staging in intermediate- and high-risk patients; • If nodes are palpable, stage with a pelvic computed tomography (CT) or positron emission tomography (PET)/CT.	Strong
Distant metastases	
In N+ patients, obtain an abdominopelvic CT scan and chest X-ray/thoracic CT for systemic staging. Alternatively, stage with a PET/CT scan.	Strong
In patients with systemic disease or with relevant symptoms, obtain a bone scan.	

6.2.6 ***Guidelines for treatment strategies for nodal metastases***

Regional lymph nodes	**Management of regional lymph nodes is fundamental in the treatment of penile cancer**	**Strength rating**
Radiotherapy Radiotherapy	Not recommended for nodal disease except as a palliative option.	Strong
	> T1G2: invasive lymph node staging by either bilateral modified inguinal lymphadenectomy or dynamic sentinel node biopsy.	Strong
Palpable inguinal nodes (cN1/cN2)	Radical inguinal lymphadenectomy.	Strong
Fixed inguinal lymph nodes (cN3)	Neoadjuvant chemotherapy followed by radical inguinal lymphade-nectomy in responders.	Weak
Pelvic Lymph nodes	Ipsilateral pelvic lymphadenectomy if two or more inguinal nodes are involved on one side (pN2) or if extracapsular nodal metastasis (pN3) reported	Strong
Adjuvant chemotherapy	In pN2/pN3 patients after radical lymphadenectomy.	Strong
Radiotherapy	Not recommended for nodal disease except as a palliative option.	Strong

6.3.6 ***Guidelines for chemotherapy***

Recommendations	**Strength rating**
Offer patients with pN2-3 tumours adjuvant chemotherapy after radical lymphadenectomy (three to four cycles of cisplatin, a taxane and 5-fluorouracil or ifosfamide).	Strong
Offer palliative chemotherapy to patients with systemic disease.	Weak

A systematic review (SR) was performed by the Panel on 'Risks and benefits of adjuvant radiotherapy after inguinal lymphadenectomy in node-positive penile cancer' [2]. Even though not fully published, the review findings support the information presented in Section 6.2.2.3 Adjuvant treatment.

This review was performed using standard Cochrane SR methodology: http://www.cochranelibrary.com/about/about-cochrane-systematic-reviews.html

2. METHODS

2.1 Data identification

For the 2018 Penile Cancer Guidelines, new and relevant evidence has been identified, collated and appraised through a structured assessment of the literature. A broad and comprehensive literature search, covering all sections of the Penile Cancer Guidelines, was performed. Databases searched included Medline, EMBASE and the Cochrane Libraries, covering the period between November 1st 2013 and September 20th 2016. All articles relating to penile cancer (n = 838) in the relevant literature databases were reviewed resulting in the inclusion of 29 new publication in this print.

Fully revised Guidelines were produced using the updated research base, together with several national and international guidelines on penile cancer (National Comprehensive Cancer Network [3], French Association of Urology [4] and the European Society of Medical Oncology [5]).

For the 2018 edition of the EAU Guidelines the Guidelines Office have transitioned to a modified GRADE methodology across all 20 guidelines [6, 7]. For each recommendation within the guidelines there is an accompanying online strength rating form which addresses a number of key elements namely:

1. the overall quality of the evidence which exists for the recommendation, references used in this text are graded according to a classification system modified from the Oxford Centre for Evidence-Based Medicine Levels of Evidence [8];
2. the magnitude of the effect (individual or combined effects);
3. the certainty of the results (precision, consistency, heterogeneity and other statistical or study related factors);
4. the balance between desirable and undesirable outcomes;
5. the impact of patient values and preferences on the intervention;
6. the certainty of those patient values and preferences.

These key elements are the basis which panels use to define the strength rating of each recommendation which is represented by the words 'strong' or 'weak' [9]. The strength of each recommendation is determined by the balance between desirable and undesirable consequences of alternative management strategies, the quality of the evidence (including certainty of estimates), and nature and variability of patient values and preferences. The strength rating forms will be available online.

Additional information can be found in the general Methodology section of this print, and online at the EAU website: http://www.uroweb.org/guideline/.

A list of associations endorsing the EAU Guidelines can also be viewed online at the above address.

3. EPIDEMIOLOGY, AETIOLOGY AND PATHOLOGY

3.1 Definition of penile cancer

Penile carcinoma is usually a SCC and there are several recognised subtypes of penile SCC with different clinical features and natural history (see Table 1). Penile SCC usually arises from the epithelium of the inner prepuce or the glans.

Table 1: Histological subtypes of penile carcinomas, their frequency and outcome

Subtype	Frequency (% of cases)	Prognosis
Common squamous cell carcinoma (SCC)	48-65	Depends on location, stage and grade
Basaloid carcinoma	4-10	Poor prognosis, frequently early inguinal nodal metastasis [10]
Warty carcinoma	7-10	Good prognosis, metastasis rare
Verrucous carcinoma	3-8	Good prognosis, no metastasis
Papillary carcinoma	5-15	Good prognosis, metastasis rare
Sarcomatoid carcinoma	1-3	Very poor prognosis, early vascular metastasis
Mixed carcinoma	9-10	Heterogeneous group
Pseudohyperplastic carcinoma	< 1	Foreskin, related to lichen sclerosus, good prognosis, metastasis not reported
Carcinoma cuniculatum	< 1	Variant of verrucous carcinoma, good prognosis, metastasis not reported
Pseudoglandular carcinoma	< 1	High-grade carcinoma, early metastasis, poor prognosis
Warty-basaloid carcinoma	9-14	Poor prognosis, high metastatic potential [11] (higher than in warty, lower than in basaloid SCC)

Adenosquamous carcinoma	< 1	Central and peri-meatal glans, high-grade carcinoma, high metastatic potential but low mortality
Mucoepidermoid carcinoma	< 1	Highly aggressive, poor prognosis
Clear cell variant of penile carcinoma	1-2	Exceedingly rare, associated with human papilloma virus, aggressive, early metastasis, poor prognosis, outcome is lesion-dependent, frequent lymphatic metastasis [12]

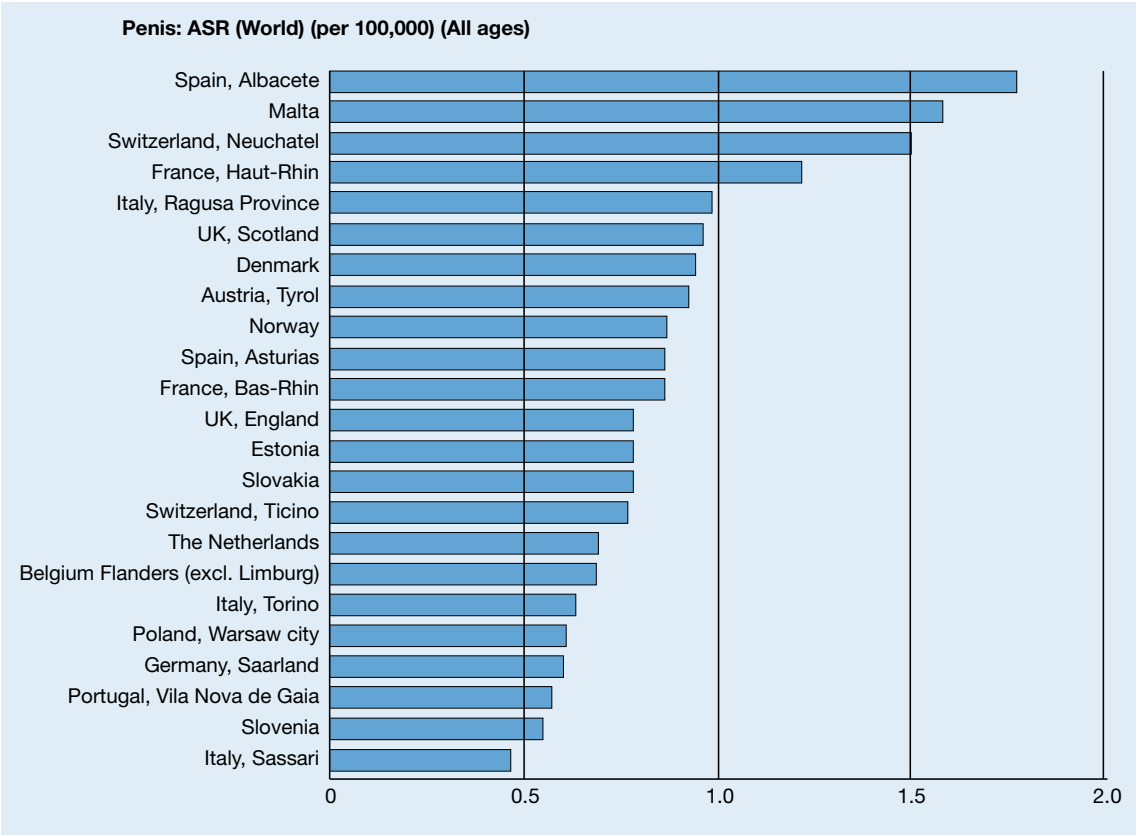
3.2 Epidemiology

In industrialised countries, penile cancer is uncommon, with an overall incidence of around 1/100,000 males in Europe and the USA [13, 14]. There are several areas in Europe with a higher incidence (Figure 1) [15]. Recent data from Scandinavia report an incidence of around 2/100,000 men. In the USA, the incidence of penile cancer is affected by race and ethnicity, with the highest incidence in white Hispanics (1.01), followed by Alaskans and Native American Indians (0.77), African Americans (0.62) and white non-Hispanics (0.51), per 100,000, respectively. In contrast, in some other parts of the world such as South America, South East Asia and parts of Africa the incidence is much higher and can account for 1-2% of malignant diseases in men [15]. The annual age-adjusted incidence is 0.7-3.0 in India, 8.3 in Brazil (per 100,000, respectively) and even higher in Uganda, where it is the most commonly diagnosed male cancer [15, 16].

In the USA, the overall age-adjusted incidence rate decreased from 1973 to 2002 from 0.84 in 1973-1982 to 0.69 in 1983-1992, and to 0.58 in 1993-2002, per 100,000, respectively [13]. In Europe, the overall incidence has been stable from the 1980s until 2013 [14]. An increased incidence was observed in Denmark [17] and the UK (21% between 1979 and 2009) [18].

The incidence of penile cancer increases with age [14], with a peak in the sixth decade but it does occur in younger men [19]. Penile cancer is common in regions with a high prevalence of HPV and this may account for the worldwide variation in incidence [13]. About one third of cases are attributed to HPV-related carcinogenesis [20]. Penile cancer is not linked to HIV or AIDS.

Figure 1: Annual incidence rate (world standardised) by European region/country*



Adapted from [15].

3.3 Risk factors and prevention

Several risk factors for penile cancer have been identified (Table 2) [21] (LE: 2a).

Table 1: Recognised aetiological and epidemiological risk factors for penile cancer

Risk factors	Relevance	Ref
Phimosis	Odds ratio 11-16 vs. no phimosis	[22-24]
Chronic penile inflammation (balanoposthitis related to phimosis), lichen sclerosus	Risk	[25]
Sporalene and ultraviolet A phototherapy for various dermatological conditions such as psoriasis	Incidence rate ratio 9.51 with > 250 treatments	[26]
Smoking	Five-fold increased risk (95% Confidence interval (CI): 2.0-10.1) vs. non-smokers	[22, 23, 27]
HPV infection, condylomata acuminate	22.4% in verrucous squamous cell carcinoma 36-66.3% in basaloid-warty	[13, 28]
Rural areas, low socio-economic status, unmarried		[29-32]
Multiple sexual partners, early age of first intercourse	Three to five-fold increased risk of penile cancer	[21, 23, 33]

Human papilloma virus infection is a risk factor for penile cancer [34]. Human papilloma virus DNA has been identified in 70-100% of intra-epithelial neoplasia and in 30-40% of invasive penile cancer tissue samples (LE: 2a). The HPV virus interacts with oncogenes and tumour suppressor genes (*p16, P53, Rb* genes) [28, 35]. The rate of HPV-positivity differs between different histological subtypes of penile SCC. Human papilloma virus is a cofactor in the carcinogenesis of some variants of penile SCC, while others are not related to HPV. The commonest HPV subtypes in penile cancer are types 16 and 18 [36]. The risk of penile cancer is increased in patients with condyloma acuminata [37] (LE: 2b).

A significantly better five-year disease-specific survival has been reported for HPV-positive vs. HPV-negative cases (93% vs. 78%) in one study [38], while no difference in lymph node metastases and ten-year survival was reported in another study [39] (Table 3). There is no association between the incidence of penile and cervical cancer, although both are linked to HPV [40, 41]. Female sexual partners of patients with penile cancer do not have an increased incidence of cervical cancer [42].

Table 3: Outcomes for HPV and non-HPV penile carcinomas

Non HPV related	Prognosis	HPV related	Prognosis
SCC usual type/NOS	30% DOD	Basaloid SCC	> 50% DOD
Pseudohyperplastic carcinoma	0%	Papillary basaloid carcinoma	
Pseudoglandular carcinoma	> 50%	Warty carcinoma	Mortality low
Verrucous carcinoma	Good	Warty-basaloid carcinoma	30% DOD
Carcinoma cuniculatum	Good	Clear-cell carcinoma	20%
Papillary carcinoma NOS	Good	Lymphoepithelioma-like carcinoma	Not known
Adenosquamous carcinoma	Good		
Sarcomatoid carcinoma	75% DOD		

DOD = died of disease; HPV = human papillomavirus; SCC = squamous cell carcinoma.

At present, except for a few countries, there is no general recommendation for HPV vaccination in males because of the different HPV-associated risk patterns in penile- and cervical cancer. Furthermore, the epidemiological effects of HPV vaccination in girls still have to be assessed [43, 44].

Phimosis is strongly associated with invasive penile cancer [23, 29, 45, 46], due to associated chronic infection. However, smegma is not a carcinogen [45]. The incidence of lichen sclerosus is relatively high in penile cancer but is not associated with adverse histopathological features, including penile intraepithelial neoplasia (PeIN). Other epidemiological risk factors are cigarette smoking, low socioeconomic status and a low level of education [29, 46].

Neonatal circumcision reduces the incidence of penile cancer; however, it does not seem to reduce the risk of PeIN [23]. The lowest incidence of penile cancer is reported for Israeli Jews (0.3/100,000/year). One matched-pair, case-control study reported that the protective effect of neonatal circumcision against invasive penile cancer (OR 0.41) was much weaker when the analysis was restricted to men without a history of phimosis (OR 0.79, 95% CI: 0.29-2) [23]. Circumcision in adult life does not have any protective effect.

The controversial discussion about neonatal circumcision should take into account that circumcision removes approximately half the tissue that can develop into penile cancer.

3.4 Pathology

Squamous cell carcinoma accounts for over 95% of penile malignancies (see Table 1). It is not known how often SCC is preceded by premalignant lesions (see Table 4) [47-50].

Different histological types of penile SCC with different growth patterns, clinical aggressiveness and HPV associations have been identified (see Table 5). Numerous mixed forms exist such as the warty-basaloid form, with 50-60% the most common mixed form, the usual-verrucous (hybrid), usual-warty, usual-basaloid and the usual-papillary, as well as other rarer combinations.

Other malignant lesions of the penis, all much less common than penile SCC, are melanocytic lesions, mesenchymal tumours, lymphomas and metastases. Penile metastases are frequently of prostatic or colorectal origin. Different types of penile sarcoma have been reported.

Table 4: Premalignant penile lesions (precursor lesions)

Lesions sporadically associated with squamous cell carcinoma (SCC) of the penis: • Bowenoid papulosis of the penis (HPV related) • Lichen sclerosis
Premalignant lesions (up to one-third transform to invasive SCC): • Penile intraepithelial lesions • Giant condylomata (Buschke-Löwenstein) • Bowen's disease • Paget's disease (intradermal ADK)

Table 5: Classification of intra-epithelial neoplasia (PeIN)

• **Non-HPV-related PeIN**
• o Differentiated PeIN
• **HPV-related PeIN**
• o Basaloid PeIN
• **Warty PeIN**
• **Warty-basaloid PeIN**
• **Other rare patterns of PeIN** (pleomorphic, spindle, clear cell, pagetoid)

3.4.1 Gross handling of pathology specimens

Tissue sections determine the accuracy of histological diagnosis. Small lesions should be fully included, bigger lesions should have at least 3-4 blocks. Lymph nodes must be included in their entirety after having been inked, in order to detect metastases. After having been inked, surgical margins have to be completely included [51]. Second-opinion pathology review is highly desirable for this rare tumour entity [52].

3.4.2 Pathology report

The pathology report must include the anatomical site of the primary tumour, the histological type of SCC, grade, perineural invasion, depth of invasion, vascular invasion (venous/lymphatic), irregular growth and front of invasion, urethral invasion, invasion of corpus spongiosum/cavernosum, surgical margins and the *p16*/HPV status (Table 6) [53-56].

Table 6: Outcomes for HPV and non-HPV penile carcinomas

Information to include in the pathology report	Recommended	required
Clinical information • Prior treatments (topic, radiotherapy, chemotherapy)	x	
Surgical procedure		x
Tumour localisation	x	
Macroscopic tumour dimension • Depth of invasion • Millimetres from basement membrane to deepest point of invasion • Maximum thickness • Size of tumour		x
Block identification	x	
Histological tumour type		x
Histological grade		x
Microscopic maximum dimensions • Combination of gross and microscopic if large tumours		x
Extent of invasion		x
LVI [58, 59]		x
Perineural invasion		x
Margin status in mm		x
Lymph node (LN) status • Size of largest nodal tumour deposit (not LN size) • Number of LN+, extracapsular spread (ECS), inguinal or pelvic, to be reported in every site separately		x
TNM Stage		x
p16/HPV status	x	

** See also www.ICCR-cancer.org. database.*

3.4.3 Grading

The TNM classification for penile cancer includes tumour grade, due to its prognostic relevance (Table 9). Tumour grading in penile cancer has been shown to be highly observer-dependent and can be problematic, especially in heterogeneous tumours. Grading should use the categories specified by the WHO for penile cancer (Table 7).

Table 7: Grading recommendations for penile SCC

Feature	Grade 1	Grade 2	Grade 3	Sarcomatoid
Cytological atypia	Mild	Moderate	Anaplasia	Sarcomatoid
Keratinisation	Usually abundant	Less prominent	May be present	Absent
Intercellular bridges	Prominent	Occasional	Few	Absent
Mitotic activity	Rare	Increased	Abundant	Abundant
Tumour margin	Pushing/well	Infiltrative/ill defined	Infiltrative/ill defined	Infiltrative/ill defined

3.4.4 Pathological prognostic factors

Pathological subtype, perineural invasion, lymphovascular invasion [58], depth of invasion and grade in the primary tumour are strong predictors of poor prognosis and high cancer-specific mortality [60]. Tumour grade is a predictor of metastatic spread, and lymphatic invasion is a predictor of metastasis. Venous embolism is often seen in advanced stages. The extent of lymph node metastasis and extracapsular spread are also strong predictors of prognosis.

The variants of penile SCC can be divided into three prognostically different groups (Table 8).

Table 8: Prognosis of the variants of penile SCC

Penile SCC	Good prognosis	Intermediate prognosis	Poor prognosis
Local growth	Destructive	Destructive	Destructive
Metastasis	Rare	Intermediate	Common
Risk of cancer-related mortality	Very low	Intermediate	High
SCC variants	• Verrucous • Papillary • Warty Pseudohyperplastic carcinoma cuniculatum	• Usual SCC • Mixed forms • Pleomorphic form of warty carcinoma	• Basaloid, • Sarcomatoid adenosquamous

There is discussion as to whether cases that show invasion of the distal urethra have a worse prognosis; however, there is no evidence to support this [61]. Nevertheless, invasion of the more proximal urethra signifies a highly aggressive SCC with a poor prognosis (see Table 9). pT3 denotes a worse prognosis than pT2 [62, 63] (LE: 2b). Capsular extension in even one single lymph node carries a poor prognosis and is denoted as pN3 [64-66].

Chaux *et al.* suggested a prognostic index which incorporates grade, anatomical level of infiltration and perineural invasion to predict the likelihood of inguinal lymph node metastases and 5-year survival [67].

3.4.5 Penile cancer and HPV

The association between penile cancer and HPV is different for the different variants of penile SCC. A high prevalence of HPV infection is found in basaloid (76%), mixed warty-basaloid (82%) and warty penile (39%) SCCs. Verrucous and papillary penile SCCs are HPV-negative. The commonest HPV-types in penile SCC are HPV-16 (72%), HPV-6 (9%) and HPV-18 (6%). Overall, only one-third of penile SCCs show HPV infection, but those that do are usually infected by several HPV strains.

3.4.6 Penile biopsy

Any doubtful penile lesion should be biopsied and, even in clinically obvious cases, histological verification must be obtained before local treatment. Before definitive surgical treatment, confirmatory frozen section excisional biopsy can be done. Histological confirmation is necessary to guide management when:

- there is doubt about the exact nature of the lesion (e.g. PeIN, metastasis or melanoma);
- treatment is planned with topical agents, radiotherapy or laser surgery.

The size of a biopsy is important. In one study, in biopsies with an average size of 0.1 cm it was difficult to evaluate the depth of invasion in 91% of cases. The grade at biopsy, and in the final specimen, may differ in up to 30% of cases, with failure to detect cancer in 3.5% of cases [47]. Furthermore, vascular and lymphatic tumour emboli were detected in only 9-11% of cases. Although a punch biopsy may be sufficient for superficial lesions, an excisional biopsy which is deep enough to properly assess the degree of invasion and stage is preferable.

3.4.7 Intra-operative frozen sections and surgical margins

Surgical treatment must completely remove the penile carcinoma with negative surgical margins, which may be confirmed by intra-operative frozen section [68]. The width of negative surgical margins should follow a risk-adapted strategy based on tumour grade. Only 3 mm of tumour-free tissue is sufficient to consider the surgical margins to be negative [69].

3.4.8 Guidelines for the pathological assessment of tumour specimens

Recommendations	Strength rating
The pathological evaluation of penile carcinoma specimens must include an assessment of the human papilloma virus status.	Strong
The pathological evaluation of penile carcinoma specimens must include a diagnosis of the squamous cell carcinoma subtype.	Strong
The pathological evaluation of penile carcinoma surgical specimens must include an assessment of surgical margins including the width of the surgical margin.	Strong

4. STAGING AND CLASSIFICATION SYSTEMS

4.1 TNM classification

The 2016 UICC TNM classification for penile cancer [51] introduced some changes in comparison to previous editions. The T1 category is stratified into two prognostically different risk groups, depending on the presence or absence of lymphovascular invasion and grading (Table 9). The classification T2 denotes invasion of the corpus spongiosum, while T3 is defined as invasion of the corpora cavernosa, due to the different prognosis of these two patterns [62, 63]. For penile cancer, unlike in other neoplasms, tumour grade is used for the TNM classification in the subdivision of the T1 stage (Table 9).

The current pN1 group consists of one or two ipsilateral inguinal lymph node metastases, pN2 is defined as more than two uni- or bilateral metastatic nodes and pN3 any pelvic nodes, uni- or bilateral, or any extranodal extension regardless of the number of lymph node metastases [51]. Retroperitoneal lymph node metastases are classified as extra-regional nodal and, therefore, distant metastases.

Table 9: 2016 TNM clinical and pathological classification of penile cancer [51]

Clinical classification	
T - Primary Tumour	
TX	Primary tumour cannot be assessed
T0	No evidence of primary tumour
Tis	Carcinoma *in situ*
Ta	Non-invasive verrucous carcinoma*
T1	Tumour invades subepithelial connective tissue
T1a	Tumour invades subepithelial connective tissue without lymphovascular invasion and is not poorly differentiated
T1b	Tumour invades subepithelial connective tissue with lymphovascular invasion or is poorly differentiated
T2	Tumour invades corpus spongiosum with or without invasion of the urethra
T3	Tumour invades corpus cavernosum with or without invasion of the urethra
T4	Tumour invades other adjacent structures
N - Regional Lymph Nodes	
NX	Regional lymph nodes cannot be assessed
N0	No palpable or visibly enlarged inguinal lymph nodes
N1	Palpable mobile unilateral inguinal lymph node
N2	Palpable mobile multiple or bilateral inguinal lymph nodes
N3	Fixed inguinal nodal mass *or* pelvic lymphadenopathy, unilateral or bilateral
M - Distant Metastasis	
M0	No distant metastasis
M1	Distant metastasis
Pathological classification	
The pT categories correspond to the clinical T categories. The pN categories are based upon biopsy or surgical excision	
pN - Regional Lymph Nodes	
pNX	Regional lymph nodes cannot be assessed
pN0	No regional lymph node metastasis
pN1	Metastasis in one or two inguinal lymph nodes
pN2	Metastasis in more than two unilateral inguinal nodes or bilateral inguinal lymph nodes
pN3	Metastasis in pelvic lymph node(s), unilateral *or* bilateral extranodal or extension of regional lymph node metastasis
pM - Distant Metastasis	
pM1	Distant metastasis microscopically confirmed
G - Histopathological Grading	
GX	Grade of differentiation cannot be assessed
G1	Well differentiated
G2	Moderately differentiated
G3	Poorly differentiated
G4	Undifferentiated

**Verrucous carcinoma not associated with destructive invasion.*

4.2 Guidelines on staging and classification

Recommendation	Strength rating
The pathological evaluation of penile carcinoma specimens must include the pTNM stage and an assessment of tumour grade.	Strong

5. DIAGNOSTIC EVALUATION AND STAGING

Penile cancer can be cured in over 80% of cases if diagnosed early, but is a life-threatening disease when lymphatic metastasis occurs. Local treatment can be mutilating, and devastating for the patient's psychological well-being.

5.1 Primary lesion

Penile carcinoma is usually a clinically obvious lesion but it may be hidden under a phimosis [24]. Physical examination should include palpation of the penis to assess the extent of local invasion and palpation of both groins to assess the lymph node status.

Ultrasound (US) can provide information about infiltration of the corpora [70, 71]. Magnetic resonance imaging (MRI) with an artificially induced erection can be used to exclude corporal invasion but is very unpleasant for the patient [72, 73]. The sensitivity and specificity of MRI in predicting corporal or urethral invasion was reported as 82.1% and 73.6%, and 62.5% and 82.1%, respectively [74]. Penile Doppler US has been reported to have a higher staging accuracy than an MRI in detecting corporal infiltration [75].

5.2 Regional lymph nodes

Careful palpation of both groins for enlarged inguinal lymph nodes must be part of the initial physical examination of patients suspected of having penile cancer.

5.2.1 Non-palpable inguinal nodes

If there are no palpable lymph nodes, the likelihood of micro-metastatic disease is about 25%. Imaging studies are not helpful in staging clinically normal inguinal regions, although may be used in obese patients in whom palpation is unreliable:

- Inguinal US (7.5 MHz) can detect abnormal, enlarged nodes. The longitudinal/transverse diameter ratio and absence of the lymph node hilum are findings with relatively high specificity [76].
- Conventional computed tomography (CT) or MRI cannot detect micro-metastases reliably [77].
- ^{18}FDG-positron emission tomography (PET/CT) does not detect lymph node metastases < 10 mm [78, 79].

Further management of patients with normal inguinal nodes should be guided by pathological risk factors of the primary tumour. Lymphovascular invasion, local stage and grade are predictive of lymphatic metastasis [80, 81]. Existing nomograms are not accurate. Invasive lymph node staging is required in patients at intermediate- or high risk of lymphatic spread (see Section 6.2).

5.2.2 Palpable inguinal nodes

Palpably enlarged lymph nodes are highly indicative of lymph node metastases. Physical examination should note the number of palpable nodes on each side and whether these are fixed or mobile. Additional imaging does not alter management and is not required (see Section 6).

A pelvic CT scan can be used to assess the pelvic lymph nodes. Imaging with ^{18}FDG-PET/CT has shown high sensitivity (88-100%) and specificity (98-100%) for confirming metastatic nodes in patients with palpable inguinal lymph nodes [79, 82].

5.3 Distant metastases

Staging for systemic metastases should be performed in patients with positive inguinal nodes [83-85] (LE: 2b). Abdominal and pelvic CT should be done plus a chest X-ray, although a thoracic CT is more sensitive. PET/CT is an option [81].

There is no tumour marker for penile cancer. The SCC antigen (SCC Ag) is increased in less than 25% of penile cancer patients. One study reported that SCC Ag did not predict occult metastatic disease, but was an indicator of disease-free survival (DFS) in lymph-node-positive patients [86].

5.4 Guidelines for the diagnosis and staging of penile cancer

Recommendations	Strength rating
Primary tumour	
Perform a physical examination, record morphology, extent and invasion of penile structures.	Strong
Obtain a penile Doppler ultrasound or MRI with artificial erection in cases with intended organ-sparing surgery.	Weak
Inguinal lymph nodes	
Perform a physical examination of both groins, record the number, laterality and characteristics of inguinal nodes and: • If nodes are not palpable, offer invasive lymph node staging in intermediate- and high-risk patients; • If nodes are palpable, stage with a pelvic computed tomography (CT) or positron emission tomography (PET)/CT.	Strong
Distant metastases	
In N+ patients, obtain an abdominopelvic CT scan and chest X-ray/thoracic CT for systemic staging. Alternatively, stage with a PET/CT scan.	Strong
In patients with systemic disease or with relevant symptoms, obtain a bone scan.	

6. DISEASE MANAGEMENT

6.1 Treatment of the primary tumour

The aims of the treatment of the primary tumour are complete tumour removal with as much organ preservation as possible, without compromising oncological control. Local recurrence has little influence on long-term survival, so organ preservation strategies are justified [87].

There are no randomised controlled trials (RCTs) or observational comparative studies for any of the treatment options for localised penile cancer. Penile preservation appears to be superior in functional and cosmetic outcomes to partial or total penectomy, and is considered to be the primary treatment method for localised penile cancer. However, there are no RCTs comparing organ-preserving and ablative treatment strategies.

Histological diagnosis with local staging must be obtained before using non-surgical treatments. With surgical treatment, negative surgical margins must be obtained. Treatment of the primary tumour and of the regional nodes can be staged.

Local treatment modalities for small and localised penile cancer include excisional surgery, external beam radiotherapy (EBRT), brachytherapy and laser ablation. Patients should be counselled about all relevant treatment options.

6.1.1 Treatment of superficial non-invasive disease (PeIN)

Topical chemotherapy with imiquimod or 5-fluorouracil (5-FU) is an effective first-line treatment. Circumcision is advisable prior to the use of topical agents. Due to high persistence/recurrence rates, treatment must be assessed by biopsy and long-term surveillance is warranted. An insufficient response may signify underlying invasive disease. Significant inflammatory responses may occur [88, 89]. Complete responses have been reported in up to 57% of PeIN cases [90] and in 74% of cases treated by circumcision and 5-FU without relapse. If topical treatment fails, it should not be repeated.

Laser treatment with a neodymium:yttrium-aluminium-garnet (Nd:YAG) or Carbon dioxide (CO_2) laser is an effective treatment option [91-96]. Visualisation may be improved by photodynamic diagnosis with the CO_2 laser [97]. Rebiopsy for treatment control is mandatory.

Glans resurfacing, total or partial, can be a primary treatment for PeIN or a secondary option in case of failure of topical chemotherapy or laser therapy. Glans resurfacing consists of complete removal of the glandular epithelium followed by reconstruction with a graft (split skin or buccal mucosa). However, in one study in cases of glans resurfacing for presumed PeIN, up to 20% of patients were found to have invasive disease on histopathological examination [88].

6.1.2 *Treatment of invasive disease confined to the glans (category T1/T2)*

Small and localised invasive lesions should receive organ-sparing treatment. Additional circumcision is advisable for glandular tumours. Foreskin tumours are treated by 'radical circumcision'. Local excision, partial glansectomy or total glansectomy with reconstruction are surgical options. External beam radiotherapy or brachytherapy are radiotherapeutic options. Small lesions can also be treated by laser therapy but the risk of more invasive disease must be recognised.

Treatment choice depends on tumour size, histology, stage and grade, localisation (especially relative to the meatus) and patient preference.

6.1.2.1 *Intra-operative frozen section*

Many authors recommend intraoperative frozen sections to assess surgical margins. Others have suggested that frozen sections are only needed if there are definite concerns [98]. For glans resurfacing, some advocate the use of acetic acid staining to delineate abnormal areas [99]. Data from one multi-centre study suggests that differentiated penile intraepithelial neoplasia, squamous hyperplasia and lichen sclerosis present at the surgical margins are frequent findings and are not relevant for cancer-specific survival [65].

6.1.2.2 *Width of negative surgical margins*

There is no clear evidence as to the required width of negative surgical margins. With organ-sparing these can be minimal. For a general recommendation, 3-5 mm can be considered a safe maximum [100, 101]. A grade-based differentiated approach can also be used, with 3 mm for grade one, 5 mm for grade two and 8 mm for grade three. This approach has its limitations due to the difficulties with penile cancer grading.

6.1.3 *Results of different surgical organ-preserving treatments*

6.1.3.1 *Laser therapy*

The results of CO_2 laser treatment have been reported by three retrospective studies from the same institution with a median follow-up of five years and a total of 195 patients [91-93]. Laser treatment was given in combination with radiotherapy or chemotherapy for PeIN or T1 penile cancers. No cancer-specific deaths were reported. Local recurrence ranged from 14% for PeIN [93] to 23% for T1 tumours [92], with an estimated cumulative risk of local recurrence at five years of 10% for PeIN (n = 106) and 16% for T1 (n = 78) tumours [91]. The reported rate of inguinal nodal recurrence was between 0% [93] and 4% [92]. The rate of secondary partial penectomy at ten years was 3% for PeIN and 10% for T1 tumours [91].

Four studies, three from the same institution, reported results of Nd:YAG laser treatment for a total of 150 patients with a follow-up of at least four years [94-96, 102]. Local recurrence rates ranged from 10% to 48% [94, 95]. One study [96] reported recurrence-free survival rates of 100%, 95% and 89% at one, two and five years, respectively. Inguinal nodal recurrence was reported in 21% of patients [94] and cancer-specific mortality was reported as 2% [102] and 9% [95]. The three studies from the same institution reported overall survival (OS) rates of 100% at four years [94] and 85-95% [96, 103] at seven years. The rate of secondary partial penectomy was highly divergent, with 4% in one study [96] and 45% in another [95]. One study reported that no complications and no adverse effects on urinary or sexual function were observed [94].

Other studies have presented data on a variety of laser treatments with either a CO_2 or a Nd:YAG laser, a combination of both, or a potassium titanyl phosphate (KTP) laser [104-107], with a mean follow-up of 32-60 months with stages PeIN to T3 included. These studies reported on a total of 138 patients, with local recurrence rates of 11% [92], 19% [105] and 26% [107]. In one study, recurrence-free survival at five years was 88% [105]. The cancer-specific survival (CSS) probability at five years was 95% in one study [105], and 2% at five years in another [105].

6.1.3.2 *Moh's micrographic surgery*

Moh's micrographic surgery is a historical technique by which histological margins are taken in a geometrical fashion around a conus of excision. The original description [108] consisted of 33 consecutive patients treated between 1936 and 1986 with 79% cured at five years [108]. The second study reported 68% recurrence-free survival at three years, 32% local recurrences and 8% inguinal nodal recurrence [109]. In both studies, one partial amputation and one cancer-specific death occurred. In a contemporary series of 48 cases, there were no recurrences among 10 primary invasive SCCs with a cure rate of 100% (mean follow-up, 161 months, median follow-up, 177 months), but one recurrence in 19 cases of penile intraepithelial neoplasia (cure rate 94.7%) [110].

6.1.3.3 *Glans resurfacing*

Three studies have reported results of glans resurfacing in a total of 71 patients with PeIN or T1 with a median follow-up of 21-30 months [88, 111, 112]. No cancer-specific deaths were reported, the rates of local recurrence were 0% [111] and 6% [112], without reports of nodal recurrence or complications.

6.1.3.4 *Glansectomy*

Results of glansectomy were reported in three studies [100, 113, 114], while a fourth also reported on glans-preserving surgery [114]. One study reported 87 patients with six local (6.9%), eleven regional (12.6%) and two systemic recurrences (2.3%) with a mean follow-up of 42 months [100]. The other two studies reported on a total of 68 patients with a follow-up of 63 [114] and 114 months [113], respectively, in which there was one patient (8%) with local recurrence [113], six (9%) with inguinal nodal recurrence, and no cancer-specific deaths.

6.1.3.5 *Partial penectomy*

Results of partial penectomy were reported in rather heterogeneous studies with a total of 184 patients with T1-T3 tumours and a follow-up of 40-194 months [93, 114-119]. The rate of local recurrence ranged from 4-50% and cancer-specific mortality from 0-27%. The reported five-year OS ranged from 59-89% [117, 119, 120].

6.1.3.6 *Summary of results of surgical techniques*

Although conservative, organ-sparing surgery may improve quality of life (QoL), local recurrence is more likely than after amputation surgery for penile cancer. In one study the local recurrence rate after organ-sparing surgery was 18%, most of these occurred within 36 months [121], and amputation was necessary in 17% of the recurrences. Compared to this, the local recurrence rate after amputation surgery (partial or radical) was lower (4%). Glansectomy with circumcision for the treatment of small penile lesions has a very low rate of local recurrence (2%) [100].

In one large cohort of patients undergoing organ-sparing surgery, isolated local recurrence was 8.9% and five-year disease-specific survival (DSS) 91.7%. Tumour grade, stage and lymphovascular invasion were predictors of local recurrence. In the largest cohort of penile surgery, the 5-year cumulative incidence of local recurrence after organ-sparing (including laser treatment) was 27% while it was only 3.8% in the amputation group [98]. Of the 451 patients treated by organ-sparing surgery, 16% eventually underwent amputation. However, there was no significant difference in survival between the organ-sparing and the amputation groups. These results suggest that the local recurrence rates following penile preserving surgery are higher than with partial penectomy, although survival appears to be unaffected.

6.1.4 *Summary of results of radiotherapy for T1 and T2 disease*

Radiotherapy is an organ-preserving approach with good results in selected patients with T1-2 lesions < 4 cm in diameter [122-127] (LE: 2b). It can be given as external radiotherapy with a minimum dose of 60 Gy combined with a brachytherapy boost or as brachytherapy alone [123, 125]. Reported results are best with brachytherapy with local control rates ranging from 70-90% [123, 125]. The American Brachytherapy Society and the Groupe Européen de Curiethérapie-European Society of Therapeutic Radiation Oncology consensus statement for penile brachytherapy also reported good tumour control rates, acceptable morbidity and functional organ preservation for penile brachytherapy for stages T1 and T2 [128]. Penile preservation rates of 70-88% have been reported [129], with overall penile conservation rates of 87% and 70% at five and ten years. Pulsed-dose-rate brachytherapy has been introduced as a new modality and 15% local recurrences have been reported in one series [130].

In the few comparisons of surgical treatment and radiotherapy, results of surgery were slightly better. In a meta-analysis comparing surgery and brachytherapy, 5-year OS and local control rates with surgery were 76%/84% for surgery and 73%/79% for brachytherapy, respectively [131]. The organ preservation rate for brachytherapy was 74% and there was no difference in survival. Local recurrence after radiotherapy can be salvaged by surgery [132].

Specific complications of radiotherapy for penile cancer are urethral stenosis (20-35%), glans necrosis (10-20%) and late fibrosis of the corpora cavernosa [133] (LE: 3). With brachytherapy, meatal stenosis has been reported to occur in 40% of cases, but was much lower in a contemporary series of 73 patients with only 6.6%. In that series, 2.6% reported pain with sexual intercourse and 5.3% dysuria over a follow-up of 5 years. Penile amputation for necrosis was necessary in 6.8 % of patients [134].

Functional outcome after radiotherapy has not often been reported. In one report, 17/18 patients with normal erections before treatment maintained these after treatment [129].

Table 10 provides an overview of the complications and outcomes of primary local treatments.

Table 10: Summary of reported complications and oncological outcomes of local treatments*

Treatment	Complications	Local recurrence	Nodal recurrence	Cancer-specific deaths	References
ND:YAG laser	n.r.	10-48%	21%	2-9%	[94-96, 102]
C02 laser	Bleeding, meatal stenosis, both < 1%	14-23%	2-4%	n.r.	[91-93]
Lasers (unspecified)	Bleeding 8%, local infection 2%	11-26%	2%	2-3%	[104-107]
Moh's micrographic surgery	Local infection 3%, Meatal stenosis 6%	32%	8%	3-4%	[108-110]
Glans resurfacing	n.r.	4-6%	n.r.	n.r.	[88, 111, 112, 135]
Glansectomy	n.r.	8%	9%	n.r.	[113, 114]
Partial penectomy	n.r.	4-13%	14-19%	11-27%	[93, 117, 119, 120]
Brachytherapy	Meatal stenosis > 40%	10-30%	n.r.	n.r.	[122, 123, 125]
External beam radiotherapy	Urethral stenosis 20-35%, Glans necrosis 10-20%	n.r.	n.r.	n.r.	[123, 127, 128, 132, 133]

**The ranges are the lowest and highest number of occurrences reported in different series.*

6.1.5 ***Treatment recommendations for invasive penile cancer (T2-T4)***

6.1.5.1 *Treatment of invasive disease confined to the glans with or without urethral involvement (T2)*

Total glansectomy, with or without resurfacing of the corporeal heads, is recommended [115] (LE: 3). Radiotherapy is an option (see Section 6.1.6). Partial amputation should be considered in patients unfit for reconstructive surgery [132].

6.1.5.2 *Treatment of disease invading the corpora cavernosa and/or urethra (T3)*

Glansectomy with distal corporectomy and reconstruction or partial amputation with reconstruction are standard [100, 101, 126]. Radiation therapy is an option.

6.1.5.3 *Treatment of locally advanced disease invading adjacent structures (T4)*

Extensive partial amputation or total penectomy with perineal urethrostomy is the standard advisable treatment [101]. For locally advanced and ulcerated cases, neoadjuvant chemotherapy may be an option. Otherwise, adjuvant chemotherapy or palliative radiotherapy are options (see Sections 6.2.4 and 6.1.6).

6.1.5.4 *Local recurrence after organ-conserving surgery*

A second organ-conserving procedure can be performed if there is no corpus cavernosum invasion [97, 101, 121, 126, 136]. For large or high-stage recurrence, partial or total amputation is required [133]. A total phallic reconstruction may be offered to patients undergoing total/subtotal amputation [137, 138].

6.1.6 ***Guidelines for stage-dependent local treatment of penile carcinoma***

Primary tumour	Use organ-preserving treatment whenever possible	Strength rating
Tis	Topical treatment with 5-fluorouracil (5-FU) or imiquimod for superficial lesions with or without photodynamic control.	Strong
	Laser ablation with carbon dioxide (CO_2) or neodymium:yttrium-aluminium-garnet (Nd:YAG) laser.	
	Glans resurfacing.	
Ta, T1a (G1, G2)	Wide local excision with circumcision, CO_2 or Nd:YAG laser with circumcision.	Strong
	Laser ablation with CO_2 or Nd:YAG laser.	
	Glans resurfacing.	
	Glansectomy with reconstruction.	
	Radiotherapy for lesions < 4 cm.	

T1b (G3) and T2	Wide local excision plus reconstruction.	Strong
	Glansectomy with circumcision and reconstruction.	
	Radiotherapy for lesions < 4 cm in diameter.	
T3	Partial amputation with reconstruction or radiotherapy for lesions < 4 cm in diameter.	Strong
T3 with invasion of the urethra	Partial penectomy or total penectomy with perineal urethrostomy.	Strong
T4	Neoadjuvant chemotherapy followed by surgery in responders or palliative radiotherapy.	Weak
Local recurrence	Salvage surgery with penis-sparing in small recurrences or partial amputation.	Weak
	Large or high-stage recurrence: partial or total amputation.	

6.2 Management of regional lymph nodes

The development of lymphatic metastases in penile cancer follows the route of anatomical drainage. The inguinal lymph nodes, followed by the pelvic lymph nodes, provide the regional drainage system of penis. The superficial and deep inguinal lymph nodes are the first regional node group to be affected, which can be uni- or bilateral [87].

The 'sentinel' inguinal nodes, i.e. those first affected by lymphatic spread, appear to be located in the medial superior zone followed by the central inguinal zones [90]. No solitary lymphatic spread has been observed from the penis to the two inferior groin regions and no direct drainage to the pelvic nodes, either [88, 97]. These findings confirm earlier studies.

Pelvic nodal disease does not occur without ipsilateral inguinal lymph node metastasis. Also, crossover metastatic spread, from one groin to the contralateral pelvis, has never been reported. Further lymphatic spread from the pelvic nodes to retroperitoneal nodes (para-aortic, para-caval) is classified as systemic metastatic disease.

The management of regional lymph nodes is decisive for patient survival. Cure can be achieved in limited lymph node disease confined to the regional lymph nodes. Radical lymphadenectomy is the treatment of choice. Multimodal treatment combining surgery and chemotherapy is often indicated.

The management of regional lymph nodes is dependent on the clinical inguinal lymph node status. There are three possible scenarios. First, the clinical lymph nodes appear normal on palpation and are not enlarged. Secondly, the inguinal lymph nodes are palpably enlarged, either uni- or bilaterally. Thirdly, there are grossly enlarged and sometimes ulcerated inguinal lymph nodes, uni- or bilaterally.

In clinically node-negative patients (cN0), micro-metastatic disease occurs in up to 25% of cases and invasive lymph node staging is required since no imaging technique can reliably detect or exclude micro-metastatic disease. In clinically positive lymph nodes (cN1/cN2), metastatic disease is highly likely and lymph node surgery with histology is required. Enlarged fixed inguinal lymph nodes (cN3) require multimodal treatment by (neoadjuvant) chemotherapy and surgery. Even if present in only one node, capsular penetration/extra-nodal extension in lymph node metastasis carries a high risk of progression and is classified as pN3, which also requires multimodal treatment.

6.2.1 Management of patients with clinically normal inguinal lymph nodes (cN0)

Risk stratification for the micro-metastatic inguinal lymph node disease depends on stage, grade and the presence/absence of lymphovascular invasion in the primary tumour [100]. pTa/pTis tumours and those with low grade have a comparatively low risk of lymphatic spread. Well-differentiated G1 pT1 tumours are considered low risk, pT1G2 intermediate risk and pT1G3 and all higher stage tumours are considered high risk for lymphatic spread [101].

For these patients, three management strategies are possible: surveillance, invasive nodal staging or radical lymphadenectomy. Early inguinal lymphadenectomy in clinically node-negative patients is superior for long-term patient survival compared to later lymphadenectomy with regional nodal recurrence [91, 92]. One prospective study comparing bilateral lymphadenectomy, radiotherapy and surveillance in such patients reported significantly better five-year OS lymphadenectomy vs. inguinal radiotherapy or surveillance (74% vs. 66% and 63%, respectively) [93].

6.2.1.1 Surveillance

Surveillance of regional lymph nodes carries the risk of regional recurrence arising later from existing micro-metastatic disease. Patient survival is over 90% with early lymphadenectomy and below 40% with lymphadenectomy for regional recurrence [94, 95]. Patients considering surveillance must be informed about this risk. Surveillance is only recommended in patients with pTis/pTa tumours and with the appropriate caveats in low risk G1 pT1 tumours [94-96]. Compliance is required for surveillance.

6.2.1.2 Invasive nodal staging

Since no imaging technique can detect micro-metastatic disease, invasive lymph node staging is recommended for pT1 tumours of intermediate and high risk, as well as for T2-T4 tumours [92, 105] (LE: 2b). Fine-needle aspiration cytology also does not reliably exclude micro-metastatic disease and is not recommended.

Invasive nodal staging can be done by either dynamic sentinel-node biopsy (DSNB) or by modified inguinal lymphadenectomy (mILND), both of which are standard techniques [139]. Dynamic sentinel-node biopsy aims to detect affected sentinel nodes in both groins. Technetium-99m (^{99m}Tc) nanocolloid is injected around the penile cancer site on the day before surgery often combined with patent blue. A gamma-ray probe is used intra-operatively to detect the sentinel nodes, which is possible in 97% of cases. The protocol has been standardised for routine use [107]. Dynamic sentinel-node biopsy has a reported high sensitivity in some centres (90-94%) [107, 108] (LE: 2b). In a meta-analysis of eighteen studies, the pooled sensitivity was 88%, which improved to 90% with the addition of patent blue [109].

Modified ILND is an alternative option, whereby the medial superficial inguinal lymph nodes and those from the central zone are removed bilaterally [87, 106] (LE: 3), leaving the greater saphenous vein untouched.

Both methods of invasive lymph node staging may miss micro-metastatic disease leading to regional recurrence [91]. The false-negative rate may be as high as 12-15% for DSNB, even in experienced centres [95, 96]. The false-negative rate of mILND is unknown. If lymph node metastasis is found, ipsilateral radical inguinal lymphadenectomy is indicated.

6.2.2 ***Management of patients with palpable inguinal nodes (cN1/cN2)***

With uni- or bilateral palpable inguinal lymph nodes (cN1/cN2), metastatic lymph node disease is highly likely. The notion that these may be inflammatory and that antibiotic treatment should first be used is unfounded and dangerous as it delays curative treatment.

Palpably enlarged groin lymph nodes should be surgically removed, pathologically assessed (by frozen section) and, if positive, a radical inguinal lymphadenectomy should be performed. In clinically doubtful cases, US-guided fine needle aspiration cytology is an option [140].

In such cases, CT or MRI can provide staging information about the pelvic nodal status and ^{18}F-FDGPET/CT can identify additional metastases [141]. Dynamic sentinel-node biopsy is not indicated in patients with palpably enlarged lymph nodes [142] (LE: 3).

6.2.2.1 Radical inguinal lymphadenectomy

Radical inguinal lymphadenectomy carries a significant morbidity due to impaired lymph drainage from the legs and scrotum. Morbidity can be as high as 50% [143] in the presence of significant risk factors such as increased body mass index (BMI). Recent series have reported lower morbidity in about 25% of cases [144, 145] (LE: 2b). Therapeutic radical inguinal lymphadenectomy can be life-saving and should not be underused for fear of associated morbidity [146].

Tissue handling must be meticulous in order to minimise post-operative morbidity. Lymphatic vessel walls do not contain smooth muscle and are therefore not reliably closed by electrocautery. Numerous metal clips may also cause post-operative problems so that ligation of all lymphatic vessels is advisable [147, 148]. Post-operative morbidity may be reduced by preserving the saphenous vein and post-operative measures to improve drainage, such as stockings, bandaging, inguinal pressure dressings or vacuum suction and prophylactic antibiotics [149]. Transposition of the Sartorius muscle is not recommended. There is no benefit from using fibrin glue intraoperatively [150]. Advanced cases may require reconstructive surgery for wound closure. The most commonly reported complications in recent series were wound infections (1.2-1.4%), skin necrosis (0.6-4.7%), lymphoedema (5-13.9%) and lymphocele formation (2.1-4%) [144, 145].

Minimally-invasive surgical techniques (laparoscopic, robot-assisted) for inguinal lymphadenectomy are technically feasible and, in small series, have been reported to significantly reduce post-operative morbidity except for the rate of lymphoceles [144, 150-153].

6.2.2.2 Pelvic lymphadenectomy

Patients with two or more inguinal lymph node metastases on one side and/or extracapsular lymph node extension need to undergo ipsilateral pelvic lymphadenectomy. This recommendation is based on a study in which the rate of positive pelvic nodes was found to be 23% in cases with more than two positive inguinal nodes and 56% in those with more than three positive inguinal nodes or extracapsular extension [101, 154] (LE: 2b).

Positive pelvic nodes carry a worse prognosis than only inguinal nodal metastasis (five-year CSS 71.0% vs. 33.2%) [155]. In a study of 142 groin node-positive patients, significant risk factors for pelvic nodal metastasis were the number of positive inguinal nodes (cut-off three), the diameter of inguinal metastatic nodes (cut-off 30 mm) and extra-nodal extension. The percentage of pelvic nodal metastases was 0% without any of these risk factors and 57.1% with all three risk factors present [155].

Pelvic lymphadenectomy may be performed simultaneously with inguinal lymphadenectomy or as a secondary procedure. If bilateral pelvic dissection is indicated, it can be performed through a midline suprapubic extraperitoneal incision. It is important to avoid unnecessary delay if these procedures are indicated [156].

6.2.2.3 Adjuvant treatment

In patients with pN2/pN3 disease, adjuvant chemotherapy is recommended after lymphadenectomy [157] (see Section 6.3.1). One retrospective study reported long-term DFS of 84% in node-positive patients with adjuvant chemotherapy after radical lymph node surgery vs. 39% in historical controls without adjuvant chemotherapy after lymphadenectomy [157]. More recent studies have confirmed the survival benefit of adjuvant chemotherapy after radical inguinal lymphadenectomy [158-160].

Although adjuvant radiotherapy has been used after inguinal lymphadenectomy, there are no data showing definite patient benefit. Adjuvant radiotherapy after inguinal lymphadenectomy should not be administered outside of clinical studies.

*6.2.3 **Management of patients with fixed inguinal nodes (cN3)***

Patients with large and bulky, sometimes ulcerated, inguinal lymph nodes require staging by thoracic, abdominal and pelvic CT for pelvic nodes and systemic disease. In clinically unequivocal cases, histological verification by biopsy is not required.

These patients have a poor prognosis. Multimodal treatment with neoadjuvant chemotherapy followed by radical lymphadenectomy in responders is recommended [161-163]. Responders to neoadjuvant chemotherapy with post-chemotherapy surgery have been reported to achieve long-term survival in 37% of cases [161]. Contemporary studies have confirmed this patient benefit [162, 164, 165].

*6.2.4 **Management of lymph node recurrence***

Patients with regional recurrence should be treated in the same way as patients with primary cN1/cN2 disease. However, patients with regional lymph node recurrence after DSNB or modified inguinal lymphadenectomy already have disordered inguinal lymphatic drainage and are at a high risk of irregular metastatic progression. Inguinal nodal recurrence after radical inguinal lymphadenectomy has a five-year CSS rate of 16% [166].

There is no evidence for the best management in such cases. Multimodal treatment with neoadjuvant and/or adjuvant chemotherapy after radical lymph node surgery is recommended.

*6.2.5 **The role of radiotherapy in lymph node disease***

Radiotherapy is used in some institutions for the treatment of inguinal lymph nodes. However, this is not evidence-based. One of the rare prospective trials in penile cancer found that inguinal radical lymphadenectomy is superior to inguinal radiotherapy for lymph-node positive penile cancer patients [167].

There is no evidence that adjuvant radiotherapy after radical inguinal lymphadenectomy improves oncological outcome [168]. One study reported poor long-term survival in patients with adjuvant inguinal and pelvic radiotherapy [169]. Other studies have likewise not demonstrated a patient benefit [168-174].

In one comparative retrospective study, adjuvant chemotherapy was far superior to adjuvant radiotherapy after radical inguinal lymphadenectomy in node-positive patients [157]. A large retrospective analysis of the SEER database (National Cancer Institute Surveillance, Epidemiology and End Results Program) of 2,458 penile cancer patients treated with either surgery alone or surgery plus EBRT concluded that the addition of adjuvant EBRT 'had neither a harmful nor a beneficial effect on CSS' [175].

Due to this lack of positive evidence, radiotherapy cannot be recommended outside of controlled trials for the treatment of lymph node disease in penile cancer. Prophylactic radiotherapy for cN0 disease is not indicated. Radiotherapy for advanced lymph node disease remains a palliative option.

*6.2.6 **Guidelines for treatment strategies for nodal metastases***

Regional lymph nodes	**Management of regional lymph nodes is fundamental in the treatment of penile cancer**	**Strength rating**
No palpable inguinal nodes (cN0)	Tis, Ta G1, T1G1: surveillance.	Strong
	> T1G2: invasive lymph node staging by either bilateral modified inguinal lymphadenectomy or dynamic sentinel node biopsy.	Strong
Palpable inguinal nodes (cN1/cN2)	Radical inguinal lymphadenectomy.	Strong
Fixed inguinal lymph nodes (cN3)	Neoadjuvant chemotherapy followed by radical inguinal lymphadenectomy in responders.	Weak

Pelvic lymph nodes	Ipsilateral pelvic lymphadenectomy if two or more inguinal nodes are involved on one side (pN2) or if extracapsular nodal metastasis (pN3) reported.	Strong
Adjuvant chemotherapy	In pN2/pN3 patients after radical lymphadenectomy.	Strong
Radiotherapy	Not recommended for nodal disease except as a palliative option.	Strong

6.3 Chemotherapy

6.3.1 Adjuvant chemotherapy in node-positive patients after radical inguinal lymphadenectomy

Multimodal treatment can improve patient outcome. Adjuvant chemotherapy after radical lymphadenectomy in node-positive patients has been reported in a few small and heterogeneous series [158-160]. Comparing different small-scale clinical studies is fraught with difficulty.

The value of adjuvant chemotherapy after radical inguinal lymphadenectomy in node-positive penile cancer was first demonstrated by a study which reported long-term (DFS) of 84% in 25 consecutive patients treated with twelve adjuvant weekly courses of vincristine, bleomycin, and methotrexate (VBM) during 1979-1990 and compared this to a historical control group of 38 consecutive node-positive patients with radical lymphadenectomy (with- or without adjuvant inguinal radiotherapy) who had achieved a DFS rate of only 39% [161].

The same group also published results of an adjuvant chemotherapy regimen with three courses of cisplatin and 5-FU with lower toxicity and even better results compared to VBM [176] (LE: 2b). The same group has published results of adjuvant chemotherapy with cisplatin, 5-FU plus paclitaxel or docetaxel (TPF), with three to four cycles after resection of pN2-3 disease [177]. Of 19 patients, 52.6% were disease-free after a median follow up of 42 months and tolerability was good. Results of adjuvant treatment with paclitaxel and cisplatin also improved outcome [178].

Therefore, the use of adjuvant chemotherapy is recommended, in particular when the administration of the triple combination chemotherapy is feasible and there is curative intent (LE: 2b). There are no data concerning adjuvant chemotherapy in stage pN1 patients. Adjuvant chemotherapy in pN1 disease is, therefore, recommended only in clinical trials.

6.3.2 Neoadjuvant chemotherapy in patients with fixed or relapsed inguinal nodes

Bulky inguinal lymph node enlargement (cN3) indicates extensive lymphatic metastatic disease. Primary lymph node surgery is not generally recommended since complete surgical resection is unlikely and only a few patients will benefit from surgery alone.

Limited data is available on neoadjuvant chemotherapy before inguinal lymph node surgery. However, it allows for early treatment of systemic disease and down-sizing of the inguinal lymph node metastases. In responders, complete surgical treatment is possible with a good clinical response.

Results of neoadjuvant chemotherapy for bulky inguinal lymph node metastases were modest in retrospective studies including five to twenty patients treated with bleomycin-vincristine-methotrexate (BVM) or bleomycin-methotrexate-cisplatin (BMP) regimens [162, 163, 179], as well as in the confirmatory BMP trial of the Southwest Oncology Group [180]. However, treatment-related toxicity was unacceptable due to bleomycin-related mortality.

Cisplatin/5-FU (PF) chemotherapy achieved a response rate of 25-50% with more acceptable toxicity [181, 182]. Over a period of 30 years, five different neoadjuvant chemotherapy regimens were used in twenty patients [87], with long-term survival in 37% of responders who underwent radical lymph node surgery after neoadjuvant chemotherapy. In the EORTC cancer study 30992, 26 patients with locally advanced or metastatic disease received irinotecan and cisplatin chemotherapy. Although the study did not meet its primary endpoint (response rate), there were three cases of pathologically complete remissions [183].

A phase II trial evaluated treatment with four cycles of neoadjuvant paclitaxel, cisplatin, and ifosfamide (TIP). An objective response was reported in 15/30 patients, including three pathologically complete remissions (pCRs). The estimated median time to progression (TTP) was 8.1 months and the median OS was 17.1 months [164] (LE: 2a).

Hypothetical similarities between penile SCC and head and neck SCC led to the evaluation, in penile cancer, of chemotherapy regimens with an efficacy in head and neck SCC, including taxanes. The combination of cisplatin and 5-FU plus a taxane has been used in neoadjuvant and adjuvant settings [177]. An overall objective response rate of 44% was reported in 28 patients treated neoadjuvantly, including 14% pCR (LE: 2b). Similarly, a phase II trial with TPF using docetaxel instead of paclitaxel reported an objective response of 38.5% in 29 locally advanced or metastatic patients, although the study did not meet its primary endpoint. However, there was significant toxicity [184] (LE: 2a). Further evidence of the benefit of neoadjuvant chemotherapy was published recently [165].

Overall, these results support the recommendation that neoadjuvant chemotherapy using a cisplatin- and taxane-based triple combination should be used in patients with fixed, unresectable, nodal disease (LE: 2a).

There are hardly any data concerning the potential benefit of radiochemotherapy together with lymph node surgery in penile cancer. It should therefore only be used in controlled clinical trials [185].

*6.3.3 **Palliative chemotherapy in advanced and relapsed disease***

A recent retrospective study of 140 patients with advanced penile SCC reported that visceral metastases and an > 1 ECOG-performance status were independent prognostic factors, and that cisplatin-based regimens had better outcomes than non-cisplatin-based regimens after adjusting for prognostic factors [186] (LE: 3).

Before taxanes were introduced, chemotherapy data in penile cancer were limited by small numbers, patient heterogeneity and retrospective design (except for the EORTC trial [183]). Initial response rates ranged from 25% to 100%, with very few sustained responses and very few long-term survivors. The introduction of taxanes into penile cancer chemotherapy has enhanced the activity and efficacy of the regimens used [87, 162-164, 178-184, 187].

There are virtually no data on second-line chemotherapy in penile cancer. One report using second-line paclitaxel monotherapy reported a response rate of < 30% and no patient survived [188] (LE: 2a). Anecdotally, a benefit of second-line cisplatin with gemcitabine has been observed [189] (LE: 4).

*6.3.4 **Intra-arterial chemotherapy***

Intra-arterial chemotherapy which refers to intra-aortic application has been trialled in locally advanced cases, especially of cisplatin and gemcitabine in small case series [190-193]. Apart from a limited clinical response, the outcome was not significantly improved.

*6.3.5 **Targeted therapy***

Targeted drugs have been used as second-line treatment and they could be considered as single-agent treatment in refractory cases. Anti-epidermal growth factor receptor (EGFR) targeted monotherapy has been trialled [194], as EGFR is expressed in penile SCC [190, 191] and there are assumed similarities with head and neck SCC [190, 191]. There have been other studies, particularly with the anti-EGFR monoclonal antibodies panitumumab and cetuximab, without long-term response, however [195]. Some activity of tyrosine kinase inhibitors has been reported as well [193]. Further clinical studies are needed (LE: 4).

*6.3.6 **Guidelines for chemotherapy***

Recommendations	Strength rating
Offer patients with pN2-3 tumours adjuvant chemotherapy after radical lymphadenectomy (three to four cycles of cisplatin, a taxane and 5-fluorouracil or ifosfamide).	Strong
Offer patients with non-resectable or recurrent lymph node metastases neoadjuvant chemotherapy (four cycles of a cisplatin- and taxane-based regimen) followed by radical surgery.	Weak
Offer palliative chemotherapy to patients with systemic disease.	Weak

7. FOLLOW-UP

7.1 Rationale for follow-up

Early detection of recurrence increases the likelihood of curative treatment since local recurrence does not significantly reduce long-term survival if successfully treated [87, 196]. In contrast, disease that has spread to the inguinal lymph nodes greatly reduces the rate of long-term DSS. Follow-up is also important in the detection and management of treatment-related complications.

Local or regional nodal recurrences usually occur within two years of primary treatment [87]. After five years, all recurrences were either local or new primary lesions [87]. This supports an intensive follow-up regimen during the first two years, with a less intensive follow up later for a total of at least five years. Follow-up after five years may be omitted in motivated patients who will undertake regular self-examination reliably [87].

7.1.1 *When and how to follow-up*

After local treatment with negative inguinal nodes, follow-up should include physical examination of the penis and groins for local and/or regional recurrence. Additional imaging has no proven benefit. Follow-up also depends on the primary treatment modality. Histology from the glans should be obtained to confirm disease-free status following laser ablation or topical chemotherapy.

After potentially curative treatment for inguinal nodal metastases, CT or MRI imaging for the detection of systemic disease should be performed at three-monthly intervals for the first two years.

Although rare, late local recurrence may occur, with life-threatening metastases becoming very unusual after five years. Therefore, regular follow up can be stopped after five years, provided the patient understands the need to report any local changes immediately [197]. In patients unlikely to self-examine, long-term follow up may be necessary.

7.1.2 *Recurrence of the primary tumour*

Local recurrence is more likely with all types of local organ-sparing treatment but does not influence the rate of cancer-specific survival in contrast to regional lymph node recurrence [87, 196]. Local recurrence occurred during the first two years in up to 27% of patients treated with penis-preserving modalities [98]. After partial penectomy, the risk of local recurrence is about 4-5% [87, 98, 196].

Local recurrence is easily detected by physical examination, by the patient himself or his physician. Patient education is an essential part of follow-up and the patient should be urged to visit a specialist if any changes are seen.

7.1.3 *Regional recurrence*

Most regional recurrences occur during the first two years after treatment, irrespective of whether surveillance or invasive nodal staging were used. Although unlikely, regional recurrence can occur later than two years after treatment. It is therefore advisable to continue follow up in these patients [197]. The highest rate of regional recurrence (9%) occurs in patients managed by surveillance, while the lowest is in patients who have undergone invasive nodal staging by modified inguinal lymphadenectomy or DSNB and whose lymph nodes were negative (2.3%).

The use of US and fine needle aspiration cytology (FNAC) in suspicious cases has improved the early detection rate of regional recurrence [76, 198, 199]. There are no data to support the routine use of CT or MRI for the follow-up of inguinal nodes.

Patients who have had surgery for lymph node metastases without adjuvant treatment have an increased risk of regional recurrence of 19% [87]. Regional recurrence requires timely treatment by radical inguinal lymphadenectomy and adjuvant chemotherapy (see Section 6).

7.1.4 *Guidelines for follow-up in penile cancer*

	Interval of follow-up		**Examinations and investigations**	**Minimum duration of follow-up**	**Strength rating**
	Years one to two	Years three to five			
Recommendations for follow-up of the primary tumour					
Penile-preserving treatment	Three months	Six months	Regular physician or self-examination. Repeat biopsy after topical or laser treatment for penile intraepithelial neoplasia.	Five years	Strong
Amputation	Three months	One year	Regular physician or self-examination.	Five years	Strong
Recommendations for follow-up of the inguinal lymph nodes					
Surveillance	Three months	Six months	Regular physician or self-examination.	Five years	Strong
pN0 at initial treatment	Three months	One year	Regular physician or self-examination. Ultrasound with fine-needle aspiration biopsy optional.	Five years	Strong

pN+ at initial treatment	Three months	Six months	Regular physician or self-examination. Ultrasound with fine-needle aspiration cytology optional, computed tomography/ magnetic resonance imaging optional.	Five years	Strong

7.2 Quality of life

7.2.1 Consequences after penile cancer treatment

In patients with long-term survival after penile cancer treatment, sexual dysfunction, voiding problems and cosmetic penile appearance may adversely affect the patient's QoL [200]. However, there is very little data on sexual function and QoL after treatment for penile cancer. In particular, there is heterogeneity of the psychometric tools used to assess QoL outcomes and further research is needed to develop disease-specific patient reported outcome measures for penile cancer.

Comparative studies

There are only two comparative studies in the literature reporting on the health-related quality of life (HRQoL) outcomes following surgery for localised penile cancer. One study compared wide local excision with glansectomy [201]. Among 41 patients there was reduction in post-operative International Index of Erectile Function (IIEF) and the authors concluded that local excision led to better sexual outcomes than glansectomy. In another study of 147 patients, the IIEF-15, the SF36 Health Survey and the Impact of Cancer questionnaire were used [202].

Compared to an age-matched population sample, men after partial penectomy reported significantly more problems with orgasm, cosmesis, life interference and urinary function than those who had undergone penile-sparing surgery (83% vs. 43%, $p < 0.0001$). Interestingly, there were no differences in erectile function, sexual desire, intercourse satisfaction or overall sexual satisfaction.

7.2.2 Sexual activity and quality of life after laser treatment

A retrospective interview-based Swedish study after laser treatment for penile PeIN [104] in 58 out of 67 surviving patients with a mean age of 63 years, of whom 46 participated, reported a marked decrease in some sexual practices, such as manual stimulation, caressing and fellatio, but a general satisfaction rate with life overall and sexuality which was similar to that of the general Swedish population.

A large study on CO_2 laser treatment of penile cancer in 224 patients reported no problems with erectile or sexual function following treatment [91]. In another study [107], no sexual dysfunction occurred in nineteen patients treated.

7.2.3 Sexual activity after glans resurfacing

In one study with ten patients [111], 7/10 completed questionnaires (IIEF-5 and a non-validated 9-item questionnaire) at six months. The median IIEF-5 score was 24 (no erectile dysfunction). All patients who were sexually active before treatment were active after three to five months, 7/7 stated that the sensation at the tip of their penis was either no different or better after surgery, and 5/7 patients felt that their sex life had improved. Overall patient satisfaction with glans resurfacing was high.

7.2.4 Sexual activity after glansectomy

Two studies reported sexual function after glansectomy [112, 113]. In one (n = 68) with unclear methodology [113], 79% did not report any decline in spontaneous erection, rigidity or penetrative capacity after surgery, and 75% reported recovery of orgasm. In the other study [114], all twelve patients had returned to 'normal' sexual activity one month after surgery.

7.2.5 Sexual function after partial penectomy

Sexual function after partial penectomy was reported by three studies [203-205]. In one with 18 patients with a mean age of 52 years, the IIEF scores were significantly worse for all domains of sexual function after surgery [203] and 55.6% of patients had erectile function that allowed sexual intercourse. In patients who did not resume sexual activity, 50% were ashamed of their small penis and missing glans, while another third blamed surgical complications. Of those who had resumed sexual intercourse, 66.7% reported the same frequency and level of sexual activity as before surgery, while 72.2% continued to have ejaculation and orgasm every time with sexual activity. Overall, only 33.3% maintained their pre-operative frequency of sexual intercourse and were satisfied with their sex life.

In another study, an 'Overall Sexual Functioning Questionnaire' was used in 14 patients with a median time of 11.5 months after surgery (range 6-72) [204]. Prior to surgery, all patients had had normal erectile function and intercourse at least once a month. In 9/14 patients, sexual function was 'normal' or 'slightly decreased', while 3/14 had had no sexual intercourse since surgery. Alei *et al.* reported an improvement in erectile function over time [205]. In a report of 25 patients after partial penectomy and neoglans formation, the IIEF-5, Quality of Erection Questionnaire (QEQ), Erectile Dysfunction Inventory of Treatment Satisfaction (EDITS) and Self-Esteem and Relationship (SEAR) were used. This study reported a high percentage of patient and partner satisfaction with surgical treatment and recovery of sexual function, self-esteem, and overall relationship satisfaction [206].

*7.2.6 **Quality of life and sexual function after total penectomy***

In ten patients with penile cancer evaluated after total amputation of the penis, there were significant effects on sexual life and overall QoL, although there were no negative implications in terms of partner relationships, self-assessment or the evaluation of masculinity [207].

*7.2.7 **Quality of life after partial penectomy***

Several qualitative and quantitative instruments have been used to assess 'psychological behaviour and adjustment' and 'social activity' as QoL indicators [204, 208]. Patient-reported fears were those of mutilation, loss of sexual pleasure and of cancer death and what this would mean for their families. The study reported no significant levels of anxiety and depression on the General Health Questionnaire-12 and the Hospital Anxiety and Depression Scale. 'Social activity' remained the same after surgery in terms of living conditions, family life and social interactions.

7.3 Total phallic reconstruction

There is very limited data about total phallic reconstruction following full or near-total penile amputation [137, 209, 210]. Although it is not possible to restore function without a penile prosthesis, cosmetically acceptable results can be obtained.

7.4 Specialised care

Since penile cancer is rare, patients should be referred to a centre with experience and expertise in local treatment, pathological diagnosis, chemotherapy and psychological support for penile cancer patients. Some countries have centralised the care of penile cancer patients (Sweden, Denmark, the Netherlands, the UK).

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9. CONFLICT OF INTEREST

All members of the Penile Cancer Guidelines working group have provided disclosure statements of all relationships that they have that might be perceived as a potential source of a conflict of interest. This information is publicly accessible through the European Association of Urology website: http://uroweb.org/guideline/penile-cancer/.

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10. CITATION INFORMATION

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REVIEW ARTICLE

Male Genital Lichen Sclerosus

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Abstract

Male genital lichen sclerosus (MGLSc) is a chronic inflammatory skin disease responsible for male sexual dyspareunia and urological morbidity. An afeared complication is squamous cell carcinoma (SCC) of the penis. The precise etiopathogenesis of MGLSc remains controversial although genetic, autoimmune and infective (such as human papillomavirus (HPV) hepatitis C (HCV), Epstein-Barr virus (EBV) and Borrelia) factors have been implicated: Consideration of all the evidence suggests that chronic exposure of susceptible epithelium to urinary occlusion by the foreskin seems the most likely pathomechanism. The mainstay of treatment is topical ultrapotent corticosteroid therapy. Surgery is indicated for cases unresponsive to topical corticosteroid therapy, phimosis, meatal stenosis, urethral stricture, carcinoma *in situ* (CIS) and squamous cell carcinoma.

Key Words: *Male genital lichen sclerosus, etiology, pathogenesis, management*

What was known?
MGLSc is a cause of male sexual dysfunction and urological morbidity and associated with squamous carcinoma of the penis. It has been held to be an autoimmune disease.

Introduction

MGLSc is an acquired, chronic inflammatory cutaneous disease of disputed etiology. It causes acute and chronic balanoposthitis and scarring that can result in significant sexual and urinary dysfunction as well as conferring a risk of squamous carcinoma of the penis.[1,2]

The exact incidence and prevalence are difficult to ascertain. This could be because it is managed by many specialties (dermatology, genito-urinary medicine, urology and pediatrics), as well as significant under-recognition and hence under-reporting by patients and physicians. The estimated incidence of LSc ranges from 1/300 (0.3%) to 1/1000 (0.1%) in both sexes.[3] A more recent study reported the incidence of MGLSc of 1.4 patients per 100, 000 visit.[4] The epidemiology may vary between countries and racial groups because MGLSc is principally a disease of the uncircumcised although it can persist or recur after circumcision. There is a bimodal age incidence with peaks in young boys and in adult men, late in the fourth decade.[4-8]

MGLSc can be the cause of significant morbidity. The symptomatology is an expression of preputial and urethral dysfunction very often summarized as male dyspareunia.[2,6,9] The spectrum of presentation is wide. It may be asymptomatic. Some patients describe spontaneous itching, burning, bleeding, tearing, splitting (of the prepuce and frenulum), or hemorrhagic blisters or urinary problems such as dysuria and narrowing of the urinary stream, or anatomical changes of the genitalia. Most men (55%) have symptoms associated with sexual activity but dyspareunia may be even more common than this statistic due to failure or difficulty in eliciting the symptomatology.[6] Other presentations are non-retractile foreskin (phimosis), foreskin fixed in retraction (paraphimosis), urethritis, urethral stricture, urinary retention and renal failure. Some cases are diagnosed at the presentation of CIS or frank SCC. The disease and its complications have a significant impact on male sexual health.[6,10]

Signs may be subtle or florid. The organ should be examined systematically and sedulously, including with a hand lens. The classical genital morphologic manifestations of MGLSc are similar to those features of the disease encountered at extragenital sites, and include atrophic ivory-white patches (leukoderma) or plaques, hypertrophic, slightly scaly patches or plaques with telangiectasia and sparse purpura. A constrictive lichenoid posthitis is commonly seen associated with a fibrotic preputial band causing "hourglass" "waisting" of the penile shaft. Variable asymmetric, sometimes florid, Zoonoid inflammation may be present and distract the clinician from the correct diagnosis. The glans may be etiolated, affected by sclerosis especially parameatally and show purpura, ecchymosis or angiokeratomas. Meatal involvement may be subtle with encroachment

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and blunting of the architecture or more overt with "pin hole" narrowing of the meatus, but may be more overt and extensive. Adhesions occur and can be subcoronal or transcoronal. Anatomical definition is lost with effacement of the coronal sulcus and early penile papules. The frenulum is a particular target manifesting sclerosis or complete obliteration of the frenulum.[1,2,6] Perianal involvement with MGLSc in men is very rare.[1,2] Concomitant extragenital LSc is also extremely rare (<0.1%).[6,10]

Etiology

Genetic factors

Genetic factors have been proposed on the basis on reports of familial cases and association with HLA antigens. Familial LSc has been observed in identical and non-identical twins, sisters, mothers and daughters.[11,12] Family history was found in 12% of female genital (FG) LSc.[11] However, there is little evidence of a familial predisposition to LSc in boys.[11,13] The human leukocyte antigen (HLA) complex is known to determine an individual's susceptibility to inflammatory diseases by influencing both cellular and humoral immunity. Majority of this work involved women. Several studies have found HLA DQ7 and, to a lesser extent, HLA DQ8 and DQ9 to be more common in FG LSc than in controls,[14] especially if the onset of LSc is in childhood.[15] HLA-B*08, B*18, -B*15, B*57, -CW*03, -CW*07, CW*18, -DQ3, -DQ7, -DRB1*04, -DRB*4, -DRB1*07, -DRB1*12 and the haplotypes HLA-DRB1*12, -DQB1*03:01/04/09/010 were found to increase susceptibility to FG LSc, whereas HLA-DR17, -DRB1*03:01/04 and the haplotypes HLA-DRB1*03, -DQB1*02, -DRB1*03:01, DQB1*02:01/02/03 appear to protect against (FG) LSc.[12,16,17] Increased frequencies of HLA-DR11, -DR12 and DQ7 have been reported in MGLSc.[18]

GLSc has been associated with atopic eczema in boys.[19] Filaggrin (FLG) is a key protein that facilitates terminal differentiation of the epidermis and formation of the epithelial barrier, and mutations have been found to be present in atopic dermatitis. A barrier defect due to *FLG* mutations might contribute to the susceptibility of genital epithelium to urinary irritation in MGLSc.[20] However, no significant differences were found in a study of 93 MGLSc with the most prevalent filaggrin loss-of-function mutations (R501X, 2282del4, R2447X and S3247X).[21]

Autoimmunity

It remains debatable whether autoimmunity underlies this condition. A personal and/or family history of autoimmunity has been shown to be associated with LSc. Organ-specific antibodies such as anti-thyroid, gastric (parietal-cell) tissue, intrinsic factor, antinuclear and anti-smooth muscle autoantibodies have been found in patients with LSc. Autoimmune conditions such as thyroid disease, diabetes, vitiligo, alopecia areata, pernicious anemia, scleroderma and rheumatoid arthritis have been reported in patients with LSc.[12,15,22-25] The similarities of some clinical and histopathological features in LSc and lipoid proteinosis, a rare autosomal recessive genodermatosis associated with pathogenic loss-of-function mutations in the extracellular matrix protein 1 (ECM1) gene, has led to the hypothesis that ECM1 autoimmunity might contribute to the etiopathogenesis of LSc.[26,27] Further study suggested although autoantibodies to ECM1 have been found in both FG- and MGLSc, but this may be an epiphenomenon.[27,28] Similar conclusion has also been drawn for the role of circulating anti 180NC 16a and anti-BP230 autoantibodies because the percentage of patients with positive circulating anti-BP 180 antibodies appears not to exceed the number found in general population.[29,30] It has been proposed there might be different pathogenetic background in male and female patients with preponderance of the role of autoimmunity in (FG) LSc.[31] Furthermore, different percentages of CD4+ and FOXP3+ lymphocytes were observed in (FG)- and MGLSc suggesting a different pathogenetic pathway.[32]

Infection

(i) Acid-fast bacilli and spirochates
Previous studies have linked several infective agents with LSc. These include acid-fast bacilli and spirochetes.[33-35]

(ii) *Borrelia burgdorferi*
Acrodermatitis chronica atrophicans caused by *Borrelia burgdorferi* has some clinical and histological similarities with LSc. Many studies to date have produced conflicting results, which could be due to regional variations in *Borrelia burgdorferi* and different experimental technique used.[36,37] Such an association has been negated.[38]

(iii) HPV
Human papillomavirus (HPV) infection has been implicated as a causative agent. A variety of HPV sub-types such as HPV 16, 18, 33 and 51 have been reported in MGLSc.[39] It remains debatable whether some of these HPV are merely skin surface "passenger HPV" or "driver HPV" that are implicated in the pathogenesis of MGLSc. $p16^{INK4a}$ can be used as a surrogate biomarker to confirm the diagnosis of HPV-infected penile lesions.[39-42] Recent evidence has shown that there is a lack of clinical correlation of MGLSc with HPV and MGLSc has an HPV-unrelated transcriptosome,[6,43] endorsing the view that HPV might be an innocent bystander.[43] Another study has shown that there was no difference in prevalence of MGLSc between HPV-positive and -negative samples.[44] HPV has also shown to

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be absent from the foreskin of boys circumcised for persistent phimosis after their first year of life and that topical glucorcorticoid treatment failure is not associated with HPV or any specific histopathological changes.[45] However, strong immunostaining for $p16^{INK4a}$ correlated with HPV16/18 infection in 6 out of 18 cases of MGLSc has been reported.[39] A larger study is required to examine this association. Recent work has shown that there is no correlation of MGLSc (n = 62; HPV positivity = 37.9%) with $p16^{INK4a}$ immunostaining (P = 0.53) suggesting the detection of skin surface "passenger HPV" and it is unlikely to play a role in the etiopathogensis of MGLSc.[46]

(iv) HCV

HCV has been implicated by case reports[47,48] but a recent study from our group (n = 61) showed that not one patient with MGLSc had HCV suggesting that it is highly unlikely to play a pathogenic role in MGLSc.[49]

(v) EBV

Epstein–Barr virus (EBV) DNA has been found in 26.5% of 34 vulvar biopsies with FGLS compared with 0% in controls[50] but its role in MGLSc is not known.

Environmental factors

LSc demonstrates the Koebner phenomenon. It arises from areas that have undergone trauma, in old scars (e.g. after vulvectomy and circumcision), on skin grafts, at sites prone to constant friction, and after sunburn or radiation treatment.[51-56]

Exposure to urine

MGLSc is unequivocally a disease of the uncircumcised male.[6] MGLSc is exceedingly rare in the male circumcised at birth indicating that the foreskin must play an obligate role in the etiopathogenesis of MGLSc. However, MGLSc does occur in the circumcised male: with hypospadias, with genital jewelry (piercing), after surgery, trauma and instrumentation and in the bariatric patient. It does recur in grafts especially skin grafts more than mucosal grafts.[6,57-59]

Post-micturition dribbling or microincontinence has been proposed to play a central role in the etiopathogenesis of MGLSc.[60-64] Many men presenting with MGLSc confess to dribbling post-micturition. A recent study by our group has shown that 91-100% men with MGLSc have dribbling compared to 14% controls implicating the role of post-micturition microincontinence in MGLSc.[65] Furthermore, MGLSc patients are often found to have an abnormal urethral meatus or navicular fossa on meticulous examination. In the circumcised male, a tiny drop of urine appearing at the meatus post-micturition will have negligible contact with a keratinized glans before being absorbed by undergarments. In an uncircumcised male, with a similarly dysfunctional terminal urethral apparatus, the situation is very different. In men, dribbled urine, consequent upon post micturition microincontinence, is likely to pool and become occluded between the inner prepuce and distal penis/glans, affecting the frenulum, perimeatal glans and visceral prepuce. Occlusion and the phenomenon of koebnerization precipitate inflammation; inflammation progresses to sclerosis.[6,61,63,64,66] The urinary hypothesis is also supported by the observation of LSc occurring around perineal urethrostomies in patients who undergone perineal urethroplasty for long urethral strictures or severely scarred urethral plate. Chronic intermittent leakage of urine from the perineal urethrostomy following voiding is inevitably occluded between juxtaposed perineal folds with identical pathological consequences in the affected skin to the penile situation described above.[67] This hypothesis is strengthened by the occurrence of extragenital LSc in the peristomal skin in urostomy patients likely due to occlusion and urine contact.[68]

Whether MGLSc is a non-specific pathological response to urinary exposure or there is some specific mechanism or culpable constituent of urine remains unknown. Magnetic resonance spectroscopy study suggests that there is not one single indictable component of urine.[69]

Other factors

(i) Drugs

A small study has shown an inverse relationship between the presence of vulval LS and use of beta-blockers and ACE inhibitors. ACE inhibitors may diminish the LSc inflammatory process and therefore protect from LSc. Beta-blockers block cyclic AMP level resulting in upregulation of keratinocyte proliferation, reduced differentiation and increased motility of lymphocytes, which may influence the clinical expression of patients with LSc.[70] The role of drugs in the causation of MGLSc is not known.

(ii) Metabolic disease

A recent case-controlled study suggested that there might be a metabolic or lifestyle component in the etiology of MGLSc. Increased rates of elevated body mass index, diabetes mellitus, coronary artery disease (CAD) and smoking were observed in MGLSc compared to control group.[71,72] Diabetes mellitus and CAD are known to be associated with microvascular disease systematically, and the authors suggested an association of LSc with direct vascular compromise.[71] However, obese men are frequently seen with GLSc, including those where suprapubic obesity and retraction of the penis (the "vanishing penis") into the pubic mound create a pseudo or neo foreskin in males circumcised at birth or later.

MGLSc and SCC

The risk of transformation of MGLSc to carcinoma *in situ* (CIS) is difficult to quantify. Early microinvasive

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disease might be challenging to diagnose clinically against a background of LSc. A retrospective review by Barbagli *et al*. of 130 patients with MGLSc, revealed 11 men (8.4%) with premalignant or malignant features (7 cases with SCC, 2 cases with verrucous carcinoma, 1 case of SCC associated with verrucous carcinoma and 1 case of erythroplasia of Queyrat).[73] The time interval between diagnosis of LSc and the development of SCC was 14-30 years. Two forms of penile intraepithelial neoplasia (PeIN) have been described (analogy with the vulvar intraepithelial neoplasia classification): Differentiated PeIN which is often associated with MGLSc and undifferentiated or Bowenoid PeIN which is associated with HPV infection.[74]

The association of GLSc and SCC is widely recognized and the risk has been estimated at between 2% and 12.5%.[13,73,75-77] MGLSc is also considered one of the two pathways for the development of CIS and penile cancer, the other being via HPV infection.[2,74] However, in a large series of patients from our clinics not one patient developed SCC, suggesting that accurate diagnosis and definitive curative management are central to arresting carcinogenic drive.

MGLSc and melanocytic lesions

Atypical genital melanocytic naevus has been reported concomitantly in patients with FG- and MGLSc.[78-80] Cases of melanoma complicating FGLSc have appeared in the literature.[81-85] We have seen three in MGLSc.[86] It has been postulated that LSc might be an immune response possibly triggered by the melanocytic lesion, alternatively that the inflammatory environment of LSc provides a microenvironmental niche promoting melanocytic neoplasia.

Diagnosis and Treatment

Most cases of MGLSc can be diagnosed clinically. Under some circumstances a biopsy can be performed. A biopsy is useful to exclude neoplastic transformation and differentiate LSc from erosive lichen planus, scarring cicatricial pemphigoid and lichen simplex.[1,2] The histology of LSc is characterized by flattening of the epidermis with variable hyperkeratosis, hydropic degeneration of basal cells and an inflammatory cell dermal infiltrate. However, biopsy histology of MGLSc may be non-specific, depending on timing, site and severity.[87] A biopsy of the most clinically abnormal area, for example, of inflammation or scarring, may not yield a histological diagnosis. There may be nonspecific inflammatory changes (e.g. zoonoid) leading to an erroneous diagnosis of Zoon's balanitis. In many men the preputial or glanular changes may be secondary and reactive to the balanopreputial dysfunction created by the principal proximate or adjacent or apposed site of the LSc. A negative biopsy does not exclude LSc. A negative biopsy does not exclude neoplasia.[2,9,66,88]

The treatment of MGLSc aims to achieve optimal symptomatic relief, prevention of further deterioration of anatomical architecture changes and a theoretical mitigation of malignant transformation. Our understanding of the role of occlusional contact with urine in the pathogenesis is fundamental to the effective management and the prognosis of MGLSc.[65,89]

Treatment protocols in GLSc are well-established (level evidence III).[2,6,66,90] Avoidance of contact with soap and urine are essential. Regular use of skin barrier emollient should be instituted. Short, trimmed pubic hair is recommended to avoid hair trapping between the foreskin and the glans penis and hence minimize irritation, abrasion and consequent inflammation.[89] Early aggressive treatment may prevent disease progression.[91] An ultrapotent topical corticosteroid (usually clobetasol propionate (level evidence I)) used under supervision for a finite course is effective[2,92-94] and usually problem free, but herpes simplex and wart reactivation do occur.[90,95] prophylactic aciclovir should be prescribed for patients with a history of genital HSV. Most men achieved remission by ultrapotent topical corticosteroid (50-60%).[6] Topical calcineurin inhibitors should not be used in MGLSc because of a theoretical synergistic risk of SCC.[6,61,62,69] If maximal conventional medical treatment is not possible or fails then circumcision is recommended. Removal of the foreskin alters the local environment of glans penis: It removes the occlusive and koebnerizing effects of the foreskin and hence mitigates the consequences of post-micturition microincontinence. Circumcision is curatively effective in the majority (at least > 75% but precise long term follow up data are lacking).[6] Some men might require advanced urethromeatal surgery. Our results show that accurate diagnosis and rational management restores sexual function and mitigates the risk of urinary dysfunction and can conserve the foreskin in about 50%. Furthermore, not one patient of the many hundreds of patients seen in the clinic over 20 years has yet developed penis cancer.[6]

Conclusion

MGLSc is an acquired, chronic, inflammatory and fibrosing cutaneous disease responsible for significant sexual and urological dysfunction as well as creating a risk of squamous carcinoma of the penis. The etiology of MGLSc is now much better understood. It is not primarily an autoimmune condition. It is likely that an interaction between the irritant effects of urine and other pathogenic factors, such as chronicity, occlusion and as yet undetermined differential epithelial susceptibility, or reaction, to injury, are necessary for the development of MGLSc. Better understanding the pathogenesis of MGLSc is important to minimize sexual morbidity, urological dysfunction, mitigate the cancer risk it generates and preserve the foreskin when possible.

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What is new?
MGLSc is due to chronic urinary micro incontinence and occlusion. It is treated with either a short course of ultrapotent topical steroid or circumcision.

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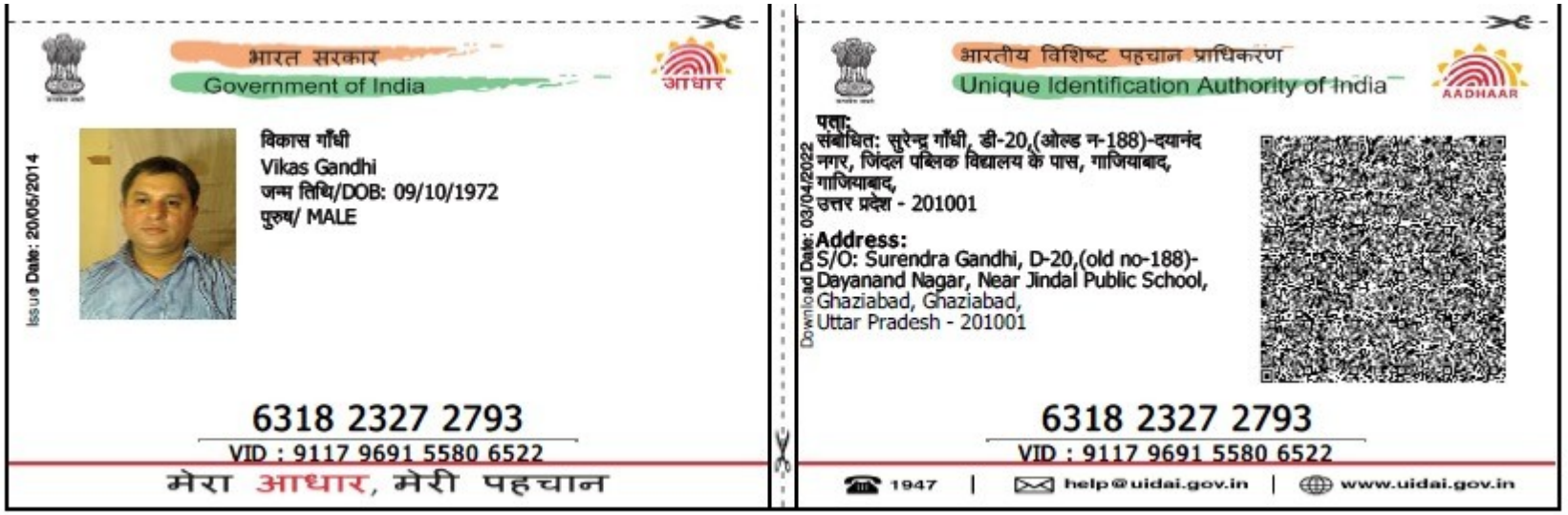

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